HEARING THE UNSPOKEN



STORIES OF HEALTH, ILLNESS AND THE HUMAN CONDITION

Vijayaprasad Gopichandran



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Hearing the Unspoken: Stories of Health, Illness, and the Human Condition

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This is a work of narrative non-fiction and clinical fiction. While based on true clinical encounters from twenty-five years of medical practice, all names, identifying characteristics, and specific circumstances have been altered to preserve patient confidentiality and medical privacy.

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Preface

Right from the days of my early training in medical college, more than all the complex pathology, molecular mechanisms, clinical procedures, surgeries and treatments, stories of patients have fascinated me. My specialization in community health and subsequently work as a primary health care provider has only strengthened this interest in knowing and understanding patients through their stories. As a primary health care provider, I see common health problems like fever, cough, cold, body aches and pains daily. The total number of drugs I prescribe are limited. I don't use cutting edge technology or do lifesaving procedures. Despite doing the same thing day in and day out what makes primary care exciting is the variety of stories that I get to hear and be part of. Stories of victories, failures; heart breaks, joy; resilience, courage; tenacity, weakness. I realized that I don't remember patients by their diagnoses, or by their clinical complexity, but by the stories that they told me. The patient with primary hyperhidrosis (excessive sweating) was 'the boy who was sent away from school because his sweat soiled and damaged the desk on which he was sitting'. The next time they came to the clinic, I had sensitive and personal inquiries to make, that instantly established the rapport between us. I started a blog in which I wrote and reflected on these stories. People who read these blogs could connect with these stories. I realized that each patient story teaches me something important. Each story is unique; I may not be able to apply the learning from one story into the life of another patient. But the constant reflection and learning from these stories sharpens my understanding of people and their lives.

I sat down to collate some of the transformative stories of my patients way back in 2018. I first wrote the narratives of patients from my days as a medical student between 1996-2001. Then again during the COVID 19 lock down, amidst busy COVID 19 duties, I needed a creative break and so reflected on some stories from my post graduate training between 2007-2010 and wrote them down. In 2024, I resigned my job as a full-time medical teacher and started full time practice of rural primary care. I started writing stories of patients I saw during this work. Earlier this year, inspired by some exceptional writing by a few of my medical colleagues, I opened my computer and dug out all the narratives I had written so far. Some were in bullet points; some were just crude summaries, and some were part of blogs. Some were written in English and some in Tamil. I brought them all together and organized 22 of those stories in this book.

This is partly a memoir of my clinical journey from MBBS days to recent times and partly fiction. In some the human story is real and the clinical vehicle on which the story travels is fictional and, in some others, vice versa. I made sure that the patients' identity is not revealed at any point in the narratives. I have attempted to be true to the emotions, and sentiments that the stories evoked, even though I have altered the facts and clinical details. I have fought a constant inner conflict throughout the process of writing these stories. Are these my stories to tell? After a lot of introspection, I concluded that I am narrating these stories from my point of view, as I was an integral part of the stories that I have written about. I have tried to stay true to the clinical setting and the encounter itself, without dramatizing events, reducing patients to caricatures, or turning care providers into heroes.

I have grouped the stories into five thematic baskets.

1. Stories of the novice clinician
2. Marginal voices
3. Social complexity, conflict and communication
4. Isolated in illness
5. Mysteries in medicine

Stories of the novice clinician has four patient stories from my MBBS, internship and junior residency days. These stories capture the naïve and idealistic response of a young clinician in training to complex patient situations. These are instances etched in my memory from stumbling upon intense lessons on the human endeavour called clinical medicine, extending far beyond the technological aspects that we are taught in medical college. The young doctor in training has a lot of ideals and moral values, but the practice environment in the medical colleges throw them into the deep end of real life, where deviations from these ideals are constantly negotiated. The young doctor struggles to retain their moral sensitivity, at the same time learning the ropes of the realities of medical practice. They sometimes drown in an abyss of moral distress and sometimes subvert the conflicts to retain a semblance of idealism. The stories left a deep impact on me and so I could vividly recollect them beyond the decades that have passed from the time they happened.

Marginal voices are painful stories of people living in the fringes of the society. Some of these stories are from my first exposure to these marginal voices, strongly imprinted in my memory by the absolute shock that they evoked when I encountered them. These stories shift the lens from the illness towards individuals

navigating immense structural barriers of gender marginalization, social ostracization, familial estrangement, and profound tragedy. They are portraits of people who refuse to be defined merely as victims of their circumstances, carving out dignity, parenthood, and purpose through deliberate acts of courage. They sensitized me to silence all the noise within and without, so that I can hear those soft and disenfranchised voices. These stories inspired me.

Social complexity, conflict and communication contains five very powerful stories. All these stories are from my days as a primary care provider in recent times. After more than 25 years of clinical practice, I have started seeing and understanding the social complexities that influence health, health seeking behaviour, and health care decisions. Why does a patient decide not to take their diabetes medications despite deteriorating health? Why does a male find it difficult to express their vulnerability to the doctor? The answers to these questions are far more complex than what we think. Evidence based medicine, and standard treatment guidelines provide step by step procedures for treatment. But real-life clinical scenarios require improvisation of these guidelines. These are the most frustrating situations for which the clinician is never prepared. In all these situations communication is the only key to a successful clinical encounter.

Isolated in illness contains four intense stories of how people belonging to different age, gender, class, castes, religions get seriously isolated by illness. Illness is a lonely experience. It is a lonely experience for the patient as well as for the care giver. The good clinician understands this loneliness and tries to be there with the patient and the care giver. The primary care clinic is the safe space where the loneliness is recognized, acknowledged and occasionally comforted. The primary care physician, the nurse, and the staff in the primary care clinic are the main people who provide care to alleviate loneliness. I understood this from these stories.

The last basket Mysteries in Medicine contains four stories of mysteries and miracles. The practice of medicine is full of mysterious events. Vanishing tumours, extra sensory perceptions, and spontaneously healing conditions are common. These stories contain such 'miraculous events' that I encountered in my work. The one message that I took home from these stories is that there is so much that we do not understand about the human body and therefore it helps to maintain a sense of humility in the practice of medicine.

Patient stories are powerful tools to learn the art of medicine. While clinical guidelines informed by rigorous research evidence can provide procedures to diagnose and treat patients, when a patient comes and sits in front of the

doctor, they do not just bring their illness with them, but also multiple social and cultural realities. The art of medicine involves the skill of carefully listening to patient stories, interpreting them, reading between the lines and using them to inform clinical decisions. Many of these stories capture my personal insecurities, dilemmas, mistakes, and moments of helplessness in clinical situations. All clinicians have these moments of vulnerability. Reading and understanding these narratives re-emphasize that the clinical encounter is an exchange between two vulnerable individuals and not a transfer of expertise from the powerful doctor to the weak patient. This helps humanize the health care provider for the patients and provide humility for the provider. Patients who read these stories can draw inspirations from the courage, grit and resilience that the people show in the face of adversity.

The stories in this book form a mosaic of clinical medicine not as an infallible science, but as a deeply human art, one where listening is an act of courage, and every patient's life is a sacred text waiting to be read.

Wishing you a meaningful reading experience!

Vijayaprasad Gopichandran

Making of a clinician



The idealistic responses of a young doctor in training, to the complexities of patients' lives.

Friendship forged by pain

It was 2001, I was an intern in the Government General Hospital, Chennai posted in the department of General Medicine. I was in the seventh unit, whose admitting day is on Sunday. The outpatient department is closed on Sundays and so all our admissions came in through the Casualty department. All interns are required to report at 7 AM in the ward, quickly do rounds of all the patients in the ward, complete activities left over from the previous day and go to the Casualty to see patients who come there for general complaints. Three of us were posted in the unit and one was on night duty on Saturday. The two of us entered the ward and relieved the intern who was on night duty. The first thing I noticed when I walked into the ward was the dry hacking cough coming from the extreme right corner of the ward. There was no time to stand and examine the patient. I just asked my colleague who was leaving after the night duty, "Who is that, coughing so badly? New admission?"

"Yes, he was sent in from the casualty early this morning. Ideally must have gone to 6th unit. But their ward is full and so they asked us to take over. I have not yet sent any investigations. You better do an evaluation and write his case sheet. 75 year old thatha with cough and breathlessness for evaluation", saying this she hurried out. She had to be back after a brief break of about an hour to continue the admission day duties.

I was not very comfortable evaluating patients with respiratory problems. Respiratory System was my Achille's heel even in the final year MBBS clinical examination. I pulled out my handbook in which I usually maintained a 'to-do' list and noted down "75 year old man with cough and breathlessness – evaluate and make case sheet". Before I realized it, the whirlwind of activity in the ward caught up with me. It was way past lunch time, that I stepped back into the ward. As I entered the same dry hacking cough welcomed me and sent me a harsh reminder of the work that was pending.

Picking up a new case sheet, my pen, some empty glass test tubes, syringes, needles and cotton swabs I walked to the last bed towards the right corner of the ward. A frail old man, thin built, wearing a dark blue checked lungi and an old white sleeveless vest which was almost brown in color with some holes on the back was lying on the bed. The standard cot and coir bed looked too big for him because of his diminutive frame. His eyes were closed. Every time he let out that hacking cough, his face would go into a frown and all his neck and upper chest muscles would go into a contraction. The neck veins would bulge like

huge snakes. He was in a lot of discomfort. I went close to him and gently called out, "Thatha… shall we talk now? I need to ask you some questions. Then I need to do a check up" The old man opened his eyes, took a few deep breaths and whispered, "Ok thambi shall I sit up?" I helped him sit up and helped him lean on the back rest that was attached to the upper end of the cot. The back rest was made of metal and there was just a thin dark green cloth draped on it. So when the old man leaned on it, the cold metallic surface made him shiver and he went into another bout of cough provoked by contact with the cold surface. "Who has come with you to the hospital? Do you need some help?" I asked the old man, looking at the obvious discomfort and suffering that he was in. "My son came to admit me, and he must have left last night. I am alone. Don't worry, I will be fine. You ask me your questions…go on…" the old man said amidst his bouts of cough.

I went through the routine history and physical examination that is required to fill out the case sheet. As I went through the routine, the old man seemed to be relaxed and did not get any further bouts of cough. He was eager to participate and actively contribute to the history and physical examination. As I was finishing up, it was the evening snack time. The hospital dietary department staff had rolled in his large cart with a huge drum of hot milk and a large vessel full of boiled chickpeas sundal. The attenders of all the other patients queued up in front of the cart with their boxes and cups to get the sundal and milk for the patients. I asked the old man, "thatha… do you have a box and cup? Can I help you get the snack?" The old man was clueless. He was just looking here and there helpless. Meanwhile a middle aged woman, attender of the patient in the next bed, took out a plate and a tumbler and she got the old man some sundal and milk and gave it to him. The old man took it from her with immense gratitude in his eyes. That is the beauty of a government hospital ecosystem. Attenders and patients care for one another in their time of vulnerability. I walked out to the nursing station to hand over the blood samples that I had collected from the old man.

"Mr. Mahalingam, 75 year old man, not a known diabetic or hypertensive. History of smoking 1 pack of beedis daily for the past 20 years. Rickshaw puller by occupation for more than 40 years, now retired since 10 years. Presented to the casualty last evening with severe acute breathlessness and chest pain. He has been having a bad cough since a month. On examination, the breath sounds are absent on the right side of the chest. Left lung shows crepitations and mild wheeze. His blood tests are all unremarkable" I presented the case of the old man briefly during the post-admission day rounds with the head of the unit the next morning. Usually the post graduate residents present the case to the head and

carry out the treatment instructions. But I had become a senior intern and had proven myself to be efficient. So the head of the unit had instructed that cases be assigned to me independently and I can present during the rounds. The head of the unit walked to the cot, where the old man was lying. Immediately the senior nurse accompanying the rounds rushed to the side of the cot and propped up the old man and made him lean on the back rest. She placed a soft comfortable pillow against the back rest. This time as the old man leaned back there was a look of calmness and relief on his face. The head of the unit put his stethoscope on the chest of the old man and listened intently. He then looked at me, "Have you taken an x ray? Put it up on the panel" I rushed to the foot end of the old man's cot, the x ray was lying in the box at the foot end. I took it out and placed it on the X ray panel. The X ray showed a dense white opacity filling the entire right side of the chest. "Looks like a dense pleural effusion (collection of liquid around the lungs). Why don't u tap it and send the fluid for testing?" the Unit head said and moved on to the next patient. I noted it down in my handbook and moved ahead with the entourage. The post graduate residents who had to now present their cases pushed me to the back and took their positions and the rounds continued.

I had performed several pleural fluid tapping procedures before and so it was not new or even exciting anymore. The feeling of the huge 20 ml syringe with the thick 20 gauge needle piercing the pleural layer and entering the pleural cavity and the yellow straw-colored fluish gushing into the syringe as we exert the negative pressure gives a high to the intern when he/she does it for the first few times. But then after a few times, the procedure just becomes a chore to be completed. The process of stabbing a large needle with a syringe on the chest is a scary procedure for many. Fortunately, the patient is in a seated posture and the needle is inserted on the back of the chest. So the patient does not get to see it as the large needle is plunged into the chest. Mostly the intern gives local anesthetic drug in the site of the injection. But in the government hospital the supply of local anesthetic is usually scanty and so very little drug is given, producing only partial numbing of the pain, or sometimes no numbing at all.

I explained the procedure to the old man. When I showed him the needle and the syringe, his eyes widened with fear. He was too scared and started crying. "Thambi….leave this. I don't want to undergo this test. Let me go home. Send me home…" he said and wept inconsolably. The old man was all alone in the hospital. He was seeing caregivers attending to all other patients. Some wives were feeding their husbands and some mothers cleaning and washing their sons. Every minute in the ward must have been a constant reminder of how alone he

was. I decided that I will ask my colleague to do the tapping while I stood by his side as his attender, holding his hand and supporting him. I explained this to him and told him that I will be with him all the way and he need not worry about the pain. The old man got some courage when I said this and he agreed for the procedure. As my friend performed the tapping, I stood by his side, holding his hand. The old man held my hand with a firm grip. My friend kept explaining the steps of the procedure and as soon as my friend said, "it is all over, now I will send the fluid for testing" the old man's grip relaxed. His entire body was shaking with the fear and exhaustion of all the anxiety and stress. I brought him some tender coconut water from outside the hospital and poured it in a glass and gave it to him. He drank it and went to sleep. It was not my duty that day, but I told my friend that I will take the night duty in the ward that day and stay over, just to be a moral support to the old man. I stayed over and escorted him to the toilet whenever he wanted to go. I also brought him his dinner and helped him have it. After dinner, we sat there on his cot and were having a light moment.

"Thambi…why don't you go and freshen up and come. You young doctors are here all the time. You work so hard. You don't eat, don't sleep. I am watching you people."

"that's ok thatha…this is part of our routine work. Over the months we have learned our own methods to find rest and breaks in the middle of all this work. Don't worry about me. I went and grabbed a quick lunch when you were sleeping. I also had coffee several times."

"I have worked like you all my life. Pulling the rickshaw without food, water, rest for hours together makes me weak. I know how it must feel. Don't tell me all these excuses. Go and rest for a bit."

He was silent for a bit and engrossed in deep thought. After that he said,

"I don't have any place to go to if you discharge me from here. Can you keep me here a little longer?"

"Why do you say that thatha? Can't you go to your son's place? You said your son came and admitted you"

"It is a long story. I will tell you some time when you have the time."

"thatha…my duty for today is over. I have all the time. Tell me…"

"My wife passed away 10 years ago in a road accident. She was crossing the road, and a lorry hit her, and she died on the spot. It was a rude shock for all of us. Because of my constant smoking and alcohol use, my relationship with my two sons has always been strained. My wife was the only bonding agent between me and the children. After her death my life has become very difficult. My sons don't care for me. They are both married and have their own families to take care of. That is why I think now neither of them will take me if I go home. I must find some arrangement to live on my own…"

I sat quietly by him listening to him as he explained the strained relationship with his sons and how he regretted his smoking and alcoholism. He also lamented how he wasted time not forming healthy relationship with his sons. He shed tears remembering how lonely he was in his life since the past 10 years.

"You go and have dinner and sleep for some time. I know you have to come back to work here again in the morning" the old man said and shooed me out of the ward in a friendly gesture.

As I was walking to the hostel to catch some sleep, some of my co-interns were rushing towards the hospital. I stopped one of them and asked what the matter was.

"There is a mass casualty. A bus toppled in the high way and about 70 people are seriously injured and brought to the casualty. They have asked all interns who are available to report to the casualty department to help. Come….lets go…"

From then till the next afternoon for almost 20 hours, everything was a haze. I was in the casualty starting intravenous lines, infusing fluids, suturing lacerations, fixing broken bones, rushing patients to the operation theatre and running here and there. I did not go to the hostel room, and did not go to the ward the next whole day. After the casualty settled a bit, I was dead beat. There was an absolute exhaustion that made my legs tremble with each step. There was a dull ache all over the body, especially the shoulders, elbows and knees. I was hungry, irritable, angry, sad and tired. I just pulled myself to my hostel room and crashed into a deep sleep. I woke up the next morning and got ready and went to the ward. A good night's sleep after complete exhaustion gives a high. I was humming my favourite song as I stepped into the ward. My first instinct was to search for my friend, the old man. His bed was empty. I asked my colleague about him. My colleague said, "Yesterday his pleural fluid report came and it showed tuberculosis. So we started him on anti tuberculosis treatment and discharged him."

All that I could think of was “thatha did not have a place to go to after discharge”. I was wondering where he would have gone. I was disappointed that I couldn’t speak to him or even say a proper goodbye. I did not have his contact details, no phone number, no address. I hadn’t even gathered enough information from him to guess where he could be living.

Yesterday, almost 20 years since this episode, I saw an elderly man in the clinic with a dry cough. The cough sounded so loud and painful. The thin old man was bent over with the spasms of the cough that had taken over his entire body. I was immediately transported back to my internship days. I was reminded of Mr. Mahalingam’s tobacco stained toothy smile and the bond that we had forged in that short time, over pain, misery and some straw coloured pleural fluid.

A bee in the ear; a break in the heart

It was the last rotation of my internship. I was posted in the Government Hospital in Triplicane. Triplicane is one of the localities in Chennai where the Ramzan month fasting is observed with great fervor. The streets were buzzing with food trucks and stalls selling Haleem early in the morning between 3.30 – 5.00 AM. Those who observed the day long fast during the month of Ramzan would have their breakfast early and then would fast the entire day to have Iftaar in the evening. Those of us who were doing night duty in the GH would walk out around 3.30 AM to see and sometimes participate in the early morning eating ritual. I was fortunate to be posted in GH during the Ramzan month and going out for observing the early morning buzz became part of my breaktime routine during the night shift. It was a Sunday evening and I was posted in the GH casualty department for the night shift. GH is a maternity centre and so the casualty is not at all busy. Most of the activity in the hospital at night time happens in the labour room. My labour room postings had completed the previous week and I was posted in the casualty to cover for someone else who was supposed to be there, but took leave for some personal reason.

The casualty room in the GH was a small room, all of 500-600 square feet floor area. The room had a table and a chair for the duty doctor to sit and a small wooden stool for the patient. There was a small examination couch on the right side against the wall. Just behind the table and chair was a curtain which separated the casualty from the doctor's duty area behind it. The duty area had a small cot with mattress for the doctor to lie down in case there are no patients. There was a single 40 watts light bulb for the entire space. There was a small window on the left wall through which there was a view of the open space adjacent to the casualty with the labor room and the wards on the other side of the open space. Most people in the local area did not know that such a overnight casualty service existed in the GH. It was in an area inside the hospital campus that was quite difficult to access in case of an urgency. Moreover, other busy hospitals and nighttime emergency services were available not far away. So, the GH casualty was only a formality and patients rarely came there. The intern posted there usually took their books and studied all night, or slept in that small cot in the duty area peacefully. I was familiar with all this because I had already done one night shift in the casualty a few days ago. I went for the duty well prepared with some books to read. The movie Lagaan had released and the music album of the movie was a hit. I had a small walkman cassette player

with earphones and I had carried that with me to listen to the songs from the album.

"Hey, what are you studying man…just come and hang out with us. We are going out for dinner" some of the other interns posted in the gynecology wards were going out for dinner and they called me to come. I was not very close to this batch of interns and was not comfortable leaving the duty station to go out for dinner. I politely refused and stayed back in the casualty. I was used to the absolute silence and loneliness of the casualty room in the GH. The lady housekeeping staff came around 9 PM to check on me and asked me if I needed anything.

"Doctor, do you want some hot tea? I have a flask. I can get some for you.." she offered.

I thanked her and declined her offer. I was planning to sleep that night and so wanted to avoid coffee or tea. After reading for a bit, I went to the duty area behind the curtain and lay down on the cot to sleep. The fan in the duty area was old and kept creaking. The creaking of the fan and the sounds of crickets in the distance were the only sounds in that area. Occasionally the sound of dragging slippers as someone walked by interrupted the symphony of the creaky fan and the crickets. It was the security guard who was making his rounds across the hospital. I don't know when I dozed off.

There was suddenly a sound of metal clashing against metal and heavy footsteps. It was too loud in that quiet space, and I woke up and sat up with a startle. My heart was pacing fast, and I was sweating. I got up and put on my slippers and came to the casualty duty area to check on what was happening. As I pulled the curtain and peeped into the casualty area, I saw a middle-aged man standing in front of the casualty table. He had a key chain in his hand and was clanking the key against the bowl of water that was kept near the sink to raise some noise. He seemed agitated and was moving shifting his body weight from one leg to the other. He was a tall man with a receding hairline and heavy bags under his eyes. He had a big paunch and was wearing a loose old white t shirt and dark blue track pants. His face was contorted in a painful frown. I moved into the casualty area and offered him a seat and asked him, "What happened? Tell me…"

"A bee has entered my ear. It is paining like anything. I need help." he shouted.

"Which ear…show me. Let me see."

He tilted his head to the left, looked down so that his right ear would face upwards and pointed to his right ear.

"Please sit down here first. Only if you sit, I can take a look…"

"I can't sit. If I sit the bee starts moving and the pain worsens…"

I did not want to argue with him because he was in obvious distress. So, I took my torchlight from the table and went near him and held his right ear pulled the pinna slightly upward and outward and shined the torch into his ear canal. I peeped into the ear. It looked perfectly clean. I could clearly see the ear drum. There was no foreign object, no bee, not even wax in the ear canal.

"I am not able to see any bee in your ear. I need to take a closer look. Can you please sit down here?" I asked him.

He showed obvious anger and irritation on his face and with that he sat down on the stool. I sat in the chair and positioned his head comfortably and took another peek into his right ear canal. Still nothing, the ear canal was clean and there was nothing in it.

"There is no bee in your ear."

The man got irritated. He let out a low pitched growl and said, "see…it is there… now it is fluttering its wings…It is hurting…it is hurting…"

I had seen a similar patient when I was posted in the ENT department. There the senior ENT surgeon had told me that sometimes when the tube connecting the middle ear with the throat is blocked, it can give rise to the sensation of some foreign object inside the ear or like a bee flying. Many patients confuse the tubal block to a foreign object in the ear. I remembered this and started explaining it to the man.

"Your ear is perfectly clear. There is nothing in your ear. Sometimes even if you have a cough or cold, the tube between your ear canal and your throat can get blocked and it can give rise to a similar sensation. Don't worry, we can make that alright by medicines."

The man got agitated and started shouting at me, "I don't know why they allow such inexperienced people to do duty in the hospital at nighttime. Call your seniors. You obviously don't know what you are doing here. I am saying there is

a bee in my ear. I have seen it. Do I know what is inside my ear or do you know it? I did not come here to get a lecture from you. Call your senior doctor now."

It was 2 AM in the morning. The only other senior doctors in the hospital would be the post graduate residents in the labor room. The labor room in the GH is usually extremely busy at night times and nobody would come to this casualty to settle the anger and frustration of this man. I was in a fix and did not know what to do. As I sat there thinking and strategizing, time was running, and the man got increasingly agitated.

I told the man that I will get some senior doctor and left the room. I ran to the labor room. The labor room was buzzing with activity. All the labor tables were full and deliveries were happening. One of my friends is a tall and hefty built guy. He was there in the labor room. I went to him and called him, "Bharat, I need a favor. I've got a guy in casualty convinced there's a bee in his ear. It's just a tubal block, but he won't listen to me. Come play 'senior doctor' for two minutes?"

Bharat knew the drill. He was the go-to guy for all of us whenever we were in a fix. We would use him as our 'human shield' as he could easily pose for a senior doctor.

"Mister, you have a tubal block and that is causing this sensation. There is no bee in your ear." Bharat announced to the man with an authoritative voice.

The man quietened down. He sat calmly with his head bowed down quietly.

Bharat excused himself and walked back to the labor room. I sat down and sorted through the medicine box and took out a pain killer tablet and a tablet that would relieve the eustachian tubal block. I handed it over to him and said, "take these tablets now. You will feel better soon…"

He reached out and took the tablets. He looked here and there for some water. I walked to the back of the curtain brought my water bottle and gave him the water. He drank the water and popped in both the tablets. We both sat there quietly.

"What is your name anna? And Age?" I asked, as I had to note down the casualty visits in the census register.

"Karunakaran…my family and friends call me Karuna. Age is 38 years." as he said this, the agitation and anger in his voice had substantially calmed down.

I scribbled down what he told me and put the pen down. We both sat there quietly.

"Thambi, what is your name?" Karuna asked me.

"Vijay…my name is Vijay anna…" I replied. I was surprised that he asked me my name.

"Thambi, you must forgive me for shouting at you earlier. Sorry pa…I was very much agitated. You must be a good doctor too….I just could not tolerate the pain in my ear."

"Don't worry about it Anna. It is normal to be angry and agitated when we are in pain. I understand."

He smiled and then there was some silence. Neither of us spoke anything.

"Where do you live Anna? What work do you do?" I initiated small conversation as the silence was getting awkward for me.

"I live nearby. I work in Chennai Corporation as a sanitary worker. My duty hours are from 6 AM in the morning till 2 PM in the afternoon. I have been alotted 10 streets in this area. I must go and clean up all the streets and collect the garbage from households."

"Why don't you go and sleep for some time. It is already 3 AM…and you have to start your work in another 3 hours" I said.

"That is ok thambi. I can't sleep now. Do you want to go and continue sleeping?"

"No Anna, it is ok. You tell me." I realized he wanted to stay and talk some more. I could easily sleep another hour or two, as I had to return to duty at 7 AM in the gynecology ward. But the look on Karuna's face and the sadness in his voice felt like he was imploring for company and conversation.

"I messed up my life and I am regretting the things I did" said Karuna and started tearing up. It was like all he needed was a kind word and gesture for him to open up.

I was taken by surprise by the tears and the emotional breakdown. I did not know what to do and sat silently. What started as small choking of voice and tears in his eyes, became loud sobs and Karuna was crying with his entire body shaking with the intensity of his emotions. The silence of the casualty room was

broken by his sobs. The emotional outburst touched me deeply and I placed my hand supportively on his shoulder. We sat there like that for a few minutes. He slowly calmed down.

"My wife took my 5 year old son and left me 4 months ago. I used to drink heavily. She kept asking me to stop drinking. By the time I decided to stop, it was too late and she left me…" by now all this tears and sobs had calmed down. He said this and stared emptily at the curtain behind me.

"I haven't touched alcohol since the past 4 months. But she is refusing to come back. She is not seeing me if I go to their home. She is not even showing me my son."

A fresh burst of tears were flowing down his cheeks. Karuna was extremely lonely and depressed.

For almost another 30 minutes Karuna and I were talking about his happy life with his wife and son and how everything changed after they left. There were no elders in the family to make peace and reconcile their differences. Karuna's life had become colorless, lonely and miserable. He just wished that some miracle happens and brings back his wife and son to him.

"Anna, I don't have enough life experience to tell you what to do. But I think you should keep trying to talk to your wife and bring her back. Try to show her that you love her and your son. When the anger passes, she will understand and come back to you." I said, without even fully understanding what I was saying.

Karunakaran patted me on the back and said, "Thambi, thank you so much. I felt like I have just discussed with my own brother. I have a fresh feeling. I will remember this always." He got up and walked away. Was it a 'bee in the ear' or was it the buzzing of grief and loneliness in his heart? I never got to know. But I felt happy and satisfied that I had established a deep human connection and calmed a lonely man's hurt heart in the process.

Doctor becomes the patient's attender

She was a petite young woman, with a beautiful garland of jasmine flowers on her plaited hair. If not for her old cotton saree draped in such a seasoned fashion, one would have mistaken her for a young girl. She walked into the outpatient department with her head bowed and sat in front of me. She was accompanied by a man almost twice her height, muscular built with a huge handlebar moustache and a large white streak of Vibhuti on his forehead. He stood there by her side as she sat on the wooden stool in front of me. Wednesdays were antenatal clinic in the hospital. They were the busiest days of the week in the outpatient department. More than 500 pregnant women from all over 80 villages in the surrounding areas would throng the clinic. Some 10-15 of us doctors would be in the clinic at any time. From 8 AM in the morning to 3-4 PM, each of us would see about 40-50 pregnant women, speak to them, examine them, and ensure a safe and healthy pregnancy. It was one such day and this lady was one of the last few women in the queue that afternoon. It takes a huge effort to be polite on a Wednesday afternoon, when the stomach is grumbling in hunger and almost every other doctor has left to have lunch but you. I gathered as much strength I had and asked her,

"What is your name?"

She was silent and he spoke on her behalf. He said, "Muthamma…Muthu"

I looked up at him with a little disinterest. Why do these men always have to fill in the answers for their wives? Was it because they thought these women were stupid? Was it their social role as protectors of their women folk? It was irritating. I ignored his reply and repeated the question, "What is your name?" but just to be polite I added another one, "Where are you from?"

She slowly looked up at me and whispered, "Muthamma…we are from Arasampattu"

"Which month of pregnancy is this?"

"This is the third month…" this time it was the man.

"Ayya, could you allow Muthu to speak? If I need any information from you, I will ask you specifically. Is that ok with you?" I told the man in as polite a tone as I could muster.

"It's not that. Muthamma is very nervous when she comes to the hospital. That is why I was answering. Sorry doctor…" the man was gentle. It was strange. Seeing his bulky frame and huge moustache, I had stereotyped him to be an all-brawn, 'macho' man from the village.

"Have you had children before?" I asked Muthamma, to further my line of enquiry.

Muthamma's head bowed low, and I noticed that she had started crying quietly. Her shoulders were shaking gently, and soft sobs were coming out. The man put his hand gently on her shoulder.

"Doctor, this is the third pregnancy. The first two she carried till term, and we lost both the babies just before delivery. They were both delivered still born"

There were tears welling up in the man's eyes too. I could sense that this hospital visit was very much nerve racking and traumatic for both. What Muthamma had was a Bad Obstetric History. This pregnancy is precious and high risk. I was a junior resident in the hospital at that time. So, we do not get to manage such high risk pregnancies on our own. I spent more time, got a detailed history, examined her and took her with me to see the consultant obstetrics specialist to discuss with her.

"Ma'am, this 26-year-old lady, is pregnant for the third time and she has no living children. Previous two pregnancies resulted in still births at term. She is now 14 weeks pregnant. How shall we work her up ma'am?" I asked my senior consultant.

"This seems quite high risk. Let us do routine blood investigations and get an ultrasound scan. It is already too late in the evening now. They will not get a bus to go back to their village. So why don't you admit them for a day? Let us evaluate them in detail and then send them home by tomorrow. Ask them if they have come prepared for admission? Admit her in the antenatal (pregnant women) ward. Let us take it from there." my consultant said this and left. I came back to Muthamma and Pandi, short for Pandiyan, that was the name of the husband, which I came to know while taking the history.

"My senior doctor madam, is asking Muthamma to get admitted for one day." I said.

Muthamma's face which was already nervous, went into a serious frown. Pandi was also becoming agitated.

“I don’t understand why we should get admitted. We are not prepared for admission. We don’t even have change of clothes for Muthamma. Can we go now and come back tomorrow for admission?” Pandi asked.

“It is already 6.00 PM and all buses must have left by now. How will you go back to the village? That is why we are asking you to stay. Nothing serious. Muthamma’s pregnancy seems to be ok for now. But since you have lost two previous babies, we must be very careful this time. That is why we want to get some blood tests and scans and then send you after confirming everything is ok.”

Pandi stood there thinking. He was looking down at Muthamma as she sat in the bench outside the outpatient department. Muthamma was looking up at him with worry written all over her face. Pandi held her hand and reassured her without any words.

“OK doctor. Go ahead and admit her. Can I go and get some change of clothes and inform my relatives and come? I will hitch a ride with any lorries or trucks going that side and I will be back early in the morning.”

Muthamma held Pandi’s hand desperately as though telling him “don’t leave me here alone”. In the hospital, the nurses do not allow any patient to be admitted alone. They insist that some attender should always be around, just in case there is some emergency or some event that takes place during their stay in the hospital. The only way I could get Muthamma to stay back was to take responsibility for her during her ward stay. I very cautiously walked up to the staff nurse in charge of the ward and said,

“Sister, this lady will be admitted overnight in the antenatal ward. She doesn’t have any attenders for tonight. I will take responsibility. I will be in my room in the hostel. You can call me any time if you have anything to say. Will that work?”

“Doctor, you people will say like this to manipulate the situation. But when it comes to turning up during need, none of you will come. And I cannot take the risk of admitting her based on what you say. It will only result in me being reprimanded by the boss. Impossible. Ask the husband to stay back. Otherwise, I am not letting her inside this ward.” Sister Sylvia always had a very professional and strict tone when it comes to matters of ward administration.

“Sister, how about, I stay in the ward tonight? That way, you can be sure that I am around if any need arises.”

Sylvia sister's stern look loosened up a bit. "OK, only if you are in the duty room. And I will ask the night duty staff not to hesitate to call you for anything regarding this patient."

"Done sister. I will just go to the hostel room freshen up and come. Meanwhile please proceed with the admission formalities" I said this to her and walked out towards Muthamma and Pandi.

"Pandi, you go ahead and go home. I will take care of Muthamma for tonight. But please come back and relieve me early in the morning."

There was a look of gratitude on Pandi's face. He took leave from Muthamma and rushed out to go to their village. I left Muthamma in the ward and went to the hostel room to freshen up. When I got back, it was about 7.30 PM and dinner time. All the women in the antenatal ward were being served dinner by their attenders. Muthamma was sitting alone and consciously turning her gaze away from the other patients in the ward who were eating. I quickly walked back to my hostel room, picked up my tiffin box from the room and rushed to the canteen. That night the dinner menu was Aappam, a rice based bread, with stew made of mixed vegetables in a coconut milk base. I packed 4 aappams and the box full of stew and walked back to the ward. I took it to Muthamma.

"Muthamma, have this. This is your dinner for tonight."

For the first time I saw a gentle smile crack across the corner of her mouth. It was a crooked smile and quite hesitant, as though she was not sure if she was allowed to smile like that at a doctor.

"No sir, no sir…it is ok. You have the dinner. I will be fine…" Muthamma was shaking her hands and head in a gesture that she did not want the food.

"You must eat something. When did you eat last?"

With a broad grin on her face she said, "Morning, I had 3 idlis here in the shop outside."

"What about lunch?"

"There was no time for lunch…." By now she had reached out and took the packet of aapams and the tiffen box.

"Carry on and eat. I will get you some water"

I went to the nursing station and started filling my water bottle with the water in the Reverse Osmosis filter.

“Doctor, are you on duty tonight? I must ask some questions about patients.” Pramila sister was on duty that night in the antenatal ward.

“No sister. I am not on duty. I think Mark is on duty. Why don’t you call him?”

“I will call Mark. But why are you here if you are not on duty?”

“I am the attender for Muthamma”

“Ahh….yes yes….Sylvia sister told me that you are the attender. Is she related to you?”

“No sister. She is a patient I saw this afternoon. They were hesitating to get admitted. I just convinced her husband that I will take care till he comes back tomorrow morning.”

Pramila sister gave me a nod of approval and moved on.

By the time I took the water to Muthamma, she was almost done with her dinner. I gave her the water. She drank half the bottle. She then took my tiffen box and started walking towards the bathroom to wash her hands and the box. I reached out and asked her to give me the box. She took the box away, turned the other way and rushed towards the bathroom. When she returned the tiffen box, she had a full smile on her face.

Mark, my colleague who was on duty that night in the wards, ran across the antenatal ward at that moment. I rushed out of the ward and shouted out, “Mark, what happened? Need any help?”

“Ohh…, you are here. This is great. Come, help us. There is an emergency cesarean section posted in the labour room and we are short of hands.” Mark shouted back as he ran towards the labour room.

Emergency Cesarean sections are high tension events in the labour room. The junior resident on duty identifies the problem and organizes the surgery and transfers the woman to the operation theatre. The senior doctor on call is alerted and they rush to the operation theatre. Usually there is another junior resident on standby for assisting in the cesarean section. While the duty resident monitors the woman, receives the newborn baby, resuscitates the baby and documents

everything, the stand by resident scrubs in and assists the surgeon. Mark was the resident on duty and he summoned me to be the standby resident for assisting the surgery. I ran to the labour room, saw the patient who had to undergo the emergency cesarean section, wheeled her to the theatre and went on to scrub and wash my hands in preparation to assist the surgery. The next one hour and 10 mins went off in a haze. The cesarean section was completed. Mother and baby were well. As I came out of the theatre, my low back started cramping and aching. The head of the baby was stuck, and the surgeon had asked me to apply pressure to maneuver it out. All that pushing standing in an odd position had led to sprain of my low back. I sat down to rest my back against the soft pillow on the bed in the duty room. As my back touched the soft pillow, it felt like every part of my body was letting out a groan of relief. I checked the time, and it was 10.30 PM. I must check on Muthu. If I don't show up in the ward, Pramila sister may alert Sylvia sister that I was not around. I needed to remain in their good books, if I needed to manouver the system for other patients like Muthamma. I got up and walked towards the antenatal ward. The lights were out and just one small zero watts bulb was on in the nursing station. Pramila sister was sitting there and writing something. All women in the ward except Muthamma were sleeping. Muthamma was sitting on her bed and staring at the wall.

"Muthamma, why aren't you asleep?" I went and stood next to her and asked.

"I am thinking about my husband. He also cannot sleep if I am not around."

"Why don't you lie down? You have travelled all the way from your village and spent the entire day in the hospital. You must be very tired. Please lie down. Even if you can't sleep its ok. Just lie down and rest."

She looked at me with an innocent pair of eyes.

Reluctantly Muthamma lay down on her bed on her left side. Within a few seconds she started letting out a gentle snoring sound. I walked away and went and sat in the nursing station. After a while, I went and lay down in the duty room and was deep asleep too.

When I got up the next morning around 5 AM, the entire ward was abuzz with activity. All women who are scheduled for induction of labour pains, are required to get ready and go to the labour room by 6 AM. Muthamma must have been woken up by all the sound and activity. She was sitting up in her cot. As soon as she saw me, she lifted her hand to her head and placed a salute.

“Good morning Muthamma. Did you sleep well?”

“Aah doctor. I did.”

“Why don’t you go wash yourself up a bit. I will get you some coffee to drink.”

“No… no… doctor. That is ok. You carry on. I will wait for my husband.”

As I prepared to leave Pandi came back.

“Pandi, good morning. I must leave for duty in another half an hour. If you had come late, I wouldn’t have been able to go.”

Pandi turned as he heard me and came close to me and reached out as though trying to hold my hand. But he did not reach out all the way. He was hesitating unclear whether he could hold the hand of a doctor. I stretched my hand out and held his. He grasped my hand vigorously and kept nodding his head. There were no words. He looked me up and down, at my disheveled hair, wrinkled clothes, and his eyes widened in respect and admiration. Meanwhile Muthamma went to the bathroom and returned.

“Come, let us go and have coffee” I said and did not wait for them to say anything. The three of us walked out to the tea stall just outside the hospital.

The cool breeze of December morning 6.30 AM, the fresh aroma of coffee decoction, the sight of people doing their morning walk on the roads, it was all so refreshing. As the three of us sat there sipping our piping hot coffee, a strong bond had started forming between us. Six months later, Pandi and Muthamma welcomed a healthy baby boy—and named him Vijay, binding our lives together far beyond that small hospital ward.

The emotional weight of understanding

That day the labour room was particularly calm. Maybe because it was a Sunday morning. I was on duty in the hospital that day. Sunday duties are quite lean for the entire hospital, but quite hectic for the junior resident doctor on duty. One or two doctors do the rounds for the entire 100 bedded hospital on Sundays. I had finished rounds of all the wards and had just taken a small break to have a cup of refreshing tea. I was sitting in the labour room, because that is where most of the action is on Sundays.

"Doctor, while it is still calm, why don't you go and rest in your duty room. I will give you a call if needed." This was the kind Kalaiarasi sister, the staff nurse who was on duty in the labour room along with me.

I gave her a nod of gratitude and walked to the doctor's duty room. The duty room was small with two cots, two wooden chairs and a big table. There was an attached bathroom and toilet for the doctor to freshen up. The room had a fluorescent tube light, a small 40 watts bulb and an old fan, which made periodic creaking sounds when it was switched on. The tube light was old and flickered frequently. So we never switched it on. Most of the time it is just the 40 watts bulb. The room had only one window and that too faced the adjacent outpatient block. Most of the time, we kept it closed, other than when the housekeeping staff comes to clean the duty room and air it for a bit. The cots have old cotton stuffed mattresses with hospital sheets spread on them. There was a small torn paper lying in one corner of the room and it was flipping in the fan draft. On days when I am exhausted from running around in the hospital all day long, I barely notice anything in the duty room. But on days like this, every micro detail is noted. I sat down on the chair, opened my laptop to prepare for a seminar that I was supposed to present the next morning in the departmental meeting.

"Sir, patient in the labour room…" Murthy, the housekeeping man on duty that day called out in the duty room.

"Coming…" I said and closed my laptop and went to the labour room.

It was still calm. The clock showed 10.45 AM. Kalaiarasi sister was sitting in the nursing station and a tall young woman in a dark red pyjama kurta was standing in front of her with her back towards me. Kalaiarasi sister was getting her information from her and registering her into the labour room. As I moved

closer to the nursing station, I could see the young woman. She had a full term pregnancy.

"Sathya. 26 years old. From Singagiri…" she was giving her details to sister.

I pushed a plastic chair close to her and asked her to sit down. Sathya turned towards me and then adjusted the position of the chair and sat down.

"My due date is only next week. But pain started from early this morning. My mother thinks this could be labour pains. So we came here…"

"It could be labour pain, yes. Let us examine you and find out. Have you undergone all your pregnancy check up here?" I asked her.

"Yes, all checkup I have regularly undergone here doctor."

By then Murthy had pulled out her out patient record and handed it to the staff nurse on duty in the ward. She brought it and gave it to Kalaiarasi sister. Sister handed it over to me.

I saw the outpatient record and it showed that this was the first pregnancy. Sathya had been married only for 1 year now. There was nothing in the outpatient record to indicate any complications.

"Sister, let us examine her and see." I said. Kalaiarasi sister escorted Sathya to the examination room, helped her change into a hospital gown and positioned her on the examination table. I still remember the hesitation and doubts that I had when I performed my first examination of a woman in labour in the hospital. I had so many doubts about whether it was respectful of the woman, whether she would feel violated and so on. As the months of performing the duties moved on, I observed one thing. Most women react strongly when they see a man entering the examining room. But once I started talking to them and treating them like fellow human beings whom I am going to help through a biological event in their life, the gender difference goes to the background. But it is still likely that some of them go through the examination and the delivery supported by a male, only because they do not have a choice. One thing I ensure is that I enter the examination room with enough announcement and never sneak up on the women when they are positioned in an extremely uncomfortable and vulnerable manner. I walked in with a conversation, "Sathya, I will first examine your abdomen to see if everything is normal and check the baby's position and

heartbeat. Then I will have to perform an examination of your birth passage to see if the uterus has opened and approximately how much longer it will take for delivery of the baby."

Sathya was not shocked, or surprised. She seemed calm. She replied, "Yes, doctor. I am aware of the internal examination. Please go ahead."

Sathya was in active labour. Her uterus was contracting, but Sathya was quiet during the contractions. Was it because she did not feel the labour pains, or was it because her pain tolerance was so high? I couldn't say. When I examined her birth passage, she did not even flinch. The mouth of her uterus was open to about 2 cm diameter. But her water had already broken, and she had not realized it. There was no fluid leaking actively from the open uterus.

"You seem to be in active labour. This being your first childbirth, delivery may take up to 12-16 hours from now. Are you feeling any pains?" I asked her.

"Doctor, I am feeling very mild pains. It is not severe."

"Ok…let me consult my senior doctor and we will see how to proceed. The main thing is even before your uterus has opened enough to allow the birth of the baby, the water has broken. This may not be good for the baby. So I will see if we can speed up the process of delivery."

"Thank you doctor."

Sathya seemed to be a very brave young woman. The expression of fear and anticipation of impending labour pains that would torment her body, which is very common, especially among women who were in labour for the first time, was completely lacking on her face. She was calm and quiet and seemed to be mentally prepared for the labour pains.

I went and discussed on the phone with my senior professor about further course of action. She advised me to instill a medication in Sathya's birth passage that would speed up the softening of the mouth of the uterus and its opening. This way, the baby could be delivered sooner than normal. This was important because the water had broken already and if we allowed natural labour to progress, then it may take too long and that may cause infections from outside to track into the uterus through the opening and harm the baby. I explained the whole process to Sathya and she agreed and I instilled the drug in her birth passage.

“Sister, let us keep her in the labour room for monitoring. I will be in the duty room. Please check her contractions and baby’s heart rate every 30 mins and maintain a chart” I gave these instructions to the nurse and went back to the duty room.

The clock showed 12.15 in the afternoon. I started studying for my seminar in the duty room. The duty room gets quite warm in the middle of the day. The fan is not efficient in cooling and making the room comfortable. It is never a good experience staying in the duty room during daytime. But I had to be around because there was Sathya in active labour and I also had to work on the seminar presentation. I have had days when I have crashed in this same duty room unmindful of the heat in the middle of the day, after a particularly tiring session in the operation theatre all morning. But when we are not busy, the environmental discomfort becomes obvious and disturbing. I packed up my laptop and walked out into the labour room. Things were still quiet. I walked into the cubicle where Sathya was admitted. Kalaiarasi sister was listening to the heartbeat of Sathya’s baby using her stethoscope. Sathya was lying on the labour cot quietly and cooperating with sister for her examination. Kalaiarasi sister noticed me and in a low voice declared, “everything is fine doctor. The baby’s heart rate seems to be ok. And contractions are good and strong.”

Strong contractions were racking Sathya’s body, but she was still quiet. There was an occasional flinch on her face due to the pain.

“Sister, I will be in my hostel room. Please call me if there is anything you need. I will send the intern here to keep close watch. Looks like it is a peaceful duty today.” I said this and as Kalaiarasi sister nodded in agreement, I took the laptop and walked to my hostel room. The hostel room is inside the hospital campus and if I ran, I could reach the hospital in less than 2 minutes.

Back in my room, the rare luxury of a hot home-cooked meal and a quiet afternoon drifted me straight into a deep sleep—unaware that the shift was about to collapse.

I suddenly woke up with my mobile ringing. I was confused for a few seconds as I had lapsed into a deep sleep. I searched around for the phone and found it on the table near my cot. It was from the labour room landline. My heart was already beating fast with the abrupt waking up.

“Doctor, something is not ok. I am unable to hear Sathya’s baby’s heart beat. Can you come…” It was Kalaiarasi sister. Her voice was frantic. I have never

heard Kalaiarasi sister lose her calm. She is one of the senior staff nurses of the hospital with more than 20 years experience. It is a blessing for us junior residents when she is on duty with us. She has a calming presence and can make even the most tough duties in the labour room seem like an easy game. If Kalaiarasi sister is panicking, then something must be seriously wrong. I noticed, I was drenched in sweat. I wiped myself with a cloth towel, put on my shirt and ran out of the hostel room towards the labour room.

Even as I neared the labour room, I could hear screaming sounds and lot of loud voices from within. It sounded like the usual busy labour room. But this was not how I had left the room. As I opened the door of the labour room, it was absolute chaos there. Two interns were running helter skelter. Kalaiarasi sister was standing inside Sathya's cubicle. Two other staff nurses had been summoned to help in the busy labour room and they were doing something. All the 7 cubicles in the labour room were full. At least 3 of them were emitting high pitched screaming sounds of women in labour. I cannot even begin to explain the intense guilt that coursed through my whole body as I stood there for a full minute trying to understand the chaos. One of the interns Velmurugan walked up to me and said,

"Hi, I was going to call you. At the same time 5 women in labour landed up in the labour room. So Vinitha and I assessed all the women and have admitted them in the cubicles. You must examine them and make plans."

I felt a dash of anger and irritation. "Why didn't you call me the second they walked in? We've lost critical time!" I yelled at Velmurugan. I now had to evaluate 5 women in labour and make plans for them. It suddenly dawned on me that it was Kalaiarasi sister who had called me and that too for an emergency. I ran to Sathya's cubicle. Sathya was still quiet. But her face was screwed up and distorted in an expression of pain. Kalaiarasi sister had started an intravenous fluid infusion and was standing holding Sathya's hands. As I walked in, she handed me her stethoscope. I placed the stethoscope on Sathya's abdomen and immediately bang-bang-bang-bang I heard the baby's heart beat go strong. I rolled my eyes at Kalaiarasi sister and said, "What sister? The heart rate is so strongly heard. Why did you scare me like that?"

Kalaiarasi sister did not show any expression. She quietly gestured me to walk out of the cubicle and when outside she said, "Don't panic! The baby's heart beat is very variable. When she gets a strong contraction it is disappearing. This is not a good thing. I am telling you from my experience, please do something. I have an instinct that something is not alright."

The cacophony in the labour room was building up to a crescendo. I was starting to have a splitting headache. In as much as Kalaiarasi sister is a senior and experienced person, she is also known to be very superstitious and emotional. This intuition of hers must be one of those superstitious moments. I had examined the fetal heart rate for a whole 3 minutes and it was fine. Sathya's was an uncomplicated pregnancy. There was no chance of anything abnormal happening. I did not have the mind space to engage with her intuition at that time. "Sister, I think you are unnecessarily worried about nothing. Let us settle the other patients. I don't think anything is wrong with Sathya" When I said this Kalaiarasi sister's expression turned sour. But I was not ready for an argument with her. "Vinitha…Vinitha…." I yelled and called out for the other intern.

"Stay here with this woman and keep monitoring the baby's heart beat closely. Call me out if you need to…but remember the labour room is full. So don't call me unless it is absolutely important…" I said. I turned and saw Sathya looking at me intently. In a weak and weatherbeaten voice she asked, "Doctor…. What is happening? Is everything alright?" The look of fear had now appeared on her face. When I saw the fear and heard her quivering voice, I was moved. "Sathya, don't worry. Everything seems to be fine. I have examined you and you are proceeding correctly as expected. This doctor will stay with you and monitor you closely. I will come as soon as I finish assessing the other women. Is that ok?" I asked her with as polite and soft a tone that I could put on in that situation. Sathya looked at me and with a lot of difficulty put up a brave smile on her face. I turned and rushed to the other cubicles. It was almost 4 PM when I was finished examining and planning the further treatment for all the 5 women who were admitted and one woman who had gone into labour in the ward and transferred to the labour room. My legs and low back were begging for some rest. I sat down in the plastic chair in the nursing station. The nursing station is positioned in the labour room in such a manner that all 7 cubicles are visible from there. I looked up and saw Vinitha was still standing next to Sathya. Sathya was now writhing on the labour cot. There was a look of absolute fear and pain on her face. I could hear that she was letting out cries. That is when it suddenly occurred to me that the medicine had been instilled around 12 noon. It was already 4 PM and time for an internal examination to see if the mouth of the uterus was open. I got up and went to Sathya's room. Sathya was not her same calm self anymore. The pain of labour had beaten and defeated the strength of her calm personality, and she was moaning, crying and writhing in pain. I went on to her right side and took out the stethoscope and tried to listen to the heart beat of her baby. Nothing….no sound. I searched more…still nothing. My heart started beating frantically. I called out "Velmurugan…bring the doppler

probe…" We used the doppler probe to locate the baby's heart beat when we could not find it using the stethoscope. I desperately searched for the heart beat. I couldn't find it. By now Kalaiarasi sister had walked into Sathya's cubicle. She also started searching. We both locked eyes and there was a look of sad resignation in Kalaiarasi sister's eyes. "let us do an internal examination and see if the uterus is opened up.." I said and dashed for the gloves.

"Doctor, what happened? Why are you are running around?" Sathya yelled in a very shaky voice in between bouts of high intensity pain.

I did not answer. I did not know what to say. I put on the gloves and when I positioned her for the examination, I noticed that the baby's head was already crowning. It was all set to be born. Usually the labour in a first pregnancy never progresses so fast. Within four hours, she had already progressed into the delivery stage.

"Sister, prepare for forceps delivery. The head is crowning…" I screamed at the top of my voice. Everyone in the labour room rushed towards Sathya's cubicle. Like a clockwork things proceeded and within a few minutes using the instrument we had delivered Sathya's baby. When the delivery was over and I looked up, I noticed that Sathya had passed out. I heaved a sigh of relief and looked at the side where Kalaiarasi sister was holding the baby. She turned and looked at me and summoned me with her eyes. I handed over the cleaning up of the afterbirth and other procedures to Vinitha and Velmurugan and rushed to Kalaiarasi sister who was holding the baby. The baby was blue, limp and not breathing. I went into panic mode, took a dry cloth and dried and warmed the baby desperately. My mind had already started chanting prayers. Still no breath. I sucked out all fluids from the mouth and nose of the baby and kept wiping and warming it. Still no breath. I started doing chest thumps and started pushing air into the baby's lungs using a bag and mask. The colour was still blue and baby still limp. I kept giving chest thumps and bag and mask breaths. Half an hour went by like this, still no response. Kalaiarasi sister touched my shoulder. I understood and called out, "Time of death 5.14 pm.."

Velmurugan and Vinitha had stiched up Sathya's birth injuries, cleaned her up and positioned her back on the labour cot. The housekeeping staff had cleaned up the entire room by then. Strangely the labour room had fallen extremely quiet. I went and sat down on the nursing station. The tall young woman in red kurta pyjama was just a few hours ago standing right in this nursing station with absolute calm and poise. She had just lost her baby and she was not even aware of this. I had not time to feel the emotions now. I started writing detailed notes

for how this whole thing happened. Then I called my senior professor and gave her the update. By then the other women had all started going into childbirth one after another. By the time I finished everything and wrote all my notes, the clock showed 8.45 PM. I dropped the pen and phone on the table and closed my face with my palms. It was a long day and I was on duty all night too. As all the events of the day ran past my mind, I realized I had to check on Sathya. Kalaiarasi sister's shift had ended and she had left. I asked the other staff nurse who had come in her place where Sathya was. "Doctor, she is in the general women's ward. We did not keep her in the routine post-delivery ward what with all other women there with crying babies!"

I rehearsed the clinical facts repeatedly as I walked down the dimly lit corridor toward the general women's ward. I braced myself for the wall of grief, the screaming, the wild accusations, the chaotic outrage that I felt I deserved. Outside the ward, Sathya's husband, mother-in-law, and father stood in a tight, suffocating knot of silent doom. When I gave them the news, they asked for no explanations, no post-mortems only their daughter-in-law. Every step I took into that ward felt like walking toward an executioner.

But when I stepped past the curtain, there was no anger waiting for me.

Sathya was sitting propped up against the cold metal backrest of her cot. I stopped in my tracks. In that quiet space, my mind suddenly flashed back to 10:45 that morning, when she had first walked into the nursing station in her dark red kurta. While other women in labor were consumed by panic and pain, Sathya had carried an extraordinary, dignified calm. Now, hours later, in the hollow shadow of the worst tragedy of her life, that same profound calm remained.

She looked at me. There was an immense, devastating sadness in her eyes, but no malice, no fury, and no blame. I stood frozen, armed with all my rehearsed medical explanations, my defenses, and my clinical justifications. But Sathya didn't ask for explanations. As a single tear traced down her left cheek, she slowly lifted both her hands, brought her palms together, and placed a gentle namaste toward me—expressing a quiet, heartbreaking gratitude for a struggle she knew I had fought alongside her. That simple gesture of forgiveness undid me completely. My professional armor shattered in an instant. The woman who had lost everything stood serene in her grace, while I the doctor who was supposed to be her protector collapsed under the crushing weight of my own guilt, weeping uncontrollably in the quiet ward.

Marginal Voices



Encounters with patients living in the margins, oppressed and vulnerable.

The eternal honor

Getting up from sleep in the mornings had become a major task for Parimala. She slept in the cowshed, where her son and daughter in law had allocated space for her. She just spread an old torn saree on the floor, as far away from the cows as was possible. With a lot of struggle, she spread herself on the cloth. The hard cement floor poked on all her ribs and the hip bones. She went to sleep on her left side. The initial few minutes were good. But just as she started to sleep, her body ached and she must turn. Involuntarily as she turned on the floor, the floor to bone contact sent across several waves of pain all over her body and she woke up immediately. She grunted, groaned and creaked as she moved her frail body to the right. She was always scared when she will get the next urge to move. As she lay there unable to sleep, and the hard floor poked her back a mosquito buzzed in her ear. She knew that the next few hours were going to be the field day for the mosquitoes who had just realized the presence of her warm body on the floor of the cowshed. The torture of the summer heat, the pangs as her frail body rolled on the hard cement floor and the constant puncture of millions of mosquitoes was all it took to make her stay wide awake all night. She sometimes talked to herself "…it is ok Parimala. Just a few more days. Then it will all be over. Death is not too far away" she said to calm her 83-year-old body in that deep dark shed. As she lay there unable to sleep, her mind wandered to the past, which is where she often delved.

"Shut up Parimala! All that you can do is sit around and cry. Will you stop crying now?" bellowed her father.

Parimala hated crying. But crying was like the skin tag that hung out from her neck. She knew if she pulled it hard, it would break loose and it would start bleeding from there. But still, she could not stop herself from tugging on the skin tags. Crying for her was like that. She had to cry for almost everything. Last week she cried because she was hungry and there was no food in the house. Yesterday she cried because she had been locked out of the house and the only key was with her dad, who was at work. That day it was a totally different reason.

"…will you stop crying or what?" dad's voice struck like a thunder. Dad's anger had a calming effect on her, and she slowly stopped crying.

"I have never had any happiness because of you. First, you killed your mother during your birth. Then look at you… darker than an ugly crow and now a miserable failure in school. This is your fourth attempt at 10th standard and

even now you have failed. What am I going to do with you?" Her father was a very good man. He never re-married after her mother died while giving birth to Parimala. But that was just one side of her father. The other side was that he hated her from his core. While he sincerely provided food, shelter, clothing, and necessities for her, he never spent time with her and avoided her like the plague. For the outside world he was a dedicated and devoted father. But only the two of them knew the true extent of the rot in their relationship.

"…enough is enough. I cannot continue your schooling anymore. Kicha, my cousin's son is working as a cook for marriage ceremonies. You will marry him and that is it", he yelled and stormed out. Parimala was only 15. Parimala was scared thinking about marriage. But she did not have a choice and did not know whom to talk to.

As Parimala lay in the cow shed, she felt some warm fluid running by her side. The cow had passed urine, and it was flowing by her side. She was unable to get up fast enough to avoid being soiled. The saree she was wearing and the saree on which she lay, both got wet. She slowly dragged herself to her right, away from the urine. By then she was fully wet. "Maybe the mosquitoes will go away! Maybe I can get some sleep now!" She decided not to move any further. Sleeping in wet clothes was not new to her. As she rolled over to her left, her thoughts started flying back to the past.

Parimala closed the bedroom door and hid under the bed. Kicha was standing outside yelling profanities. Her heart was racing like a horse. Kicha pushed the door open loudly, entered with a stagger in his step.

"Yeh!!! Parimala…. you worthless dog… How many times have I told you to never look me in my eyes. How dare you even raise your head to look at me? Where are you hiding you nasty pig?"

As his voice boomed in the room, Parimala crouched deeper under the bed to hide from him. He found her, dragged her from under the bed, pulled her hair and beat her blue and black. 10 years of constant abuse and one would think things get predictable and people get used to it. But not in Parimala's case. Even though she knew exactly when Kicha would pull her, which hand he would use to punch her face, and the exact amount of pressure he would apply, it hurt every single time like it was the first time.

"You look worse than a dirty beggar. Look at your face. You are so ugly, and down there you are always dry as the desert. I wish I could kill you. You are useless, worthless piece of shit…" Kicha yelled.

Kicha forced his bulky torso reeking of sweat and alcohol upon Parimala right there on the floor, pushed and pulled her in all directions, slapped and hit her and had his way with her like he always did.

When she was newly married, she used to try and wash off the strong stench off her body after he was done. But she didn't try anymore. The odor was less on her body and more deeply embedded in her mind. No amount of soap would wash it away.

Fortunately, day broke and the sun came up. Parimala got up slowly. The first half an hour after getting up was the worst struggle. She could not move even one muscle in her body. She walked out of the shed, washed her face and mouth and came and sat down to warm herself in the sun. Her daughter in law came and thrust one small pot of thin rice gruel in front of her and stormed off. As she sat outside the cowshed, drowning her hand into the small pot of thin rice gruel she watched the happenings on the street.

Two young boys, 18-19 years old, were going door to door on the opposite side of the road. One was wearing a blue colored shirt and had a funny hair cut. They were very enthusiastic and were jumping about knocking doors and speaking something to the people. The other one wore a gray shirt and had long hair that flowed down to his shoulder level. He was the more mature and mellowed one. He was patiently writing something down as the blue shirt boy kept talking to the people who opened the door. It went on for a long time. Parimala sat there squinting against the bright sunlight and watching. Suddenly the two boys were standing in front of her.

"Paatti, we are here to enroll people to donate their bodies and their organs. Shall I explain what body donation is?" asked the blue shirt boy.

"Why do you even ask? Does it look like I am going anywhere? Come, sit down and talk to me."

"Dinesh…this Paatti has a mean sense of humor. Come and sit down. Let us talk to this Paatti" again this was the blue shirt calling out to the grey shirt.

Grey and blue shirt came and sat down just outside the cow shed next to Parimala.

"Tell me, what do you want to say?"

"No Paatti, we are here to…."

"Hey….you have to raise your voice a bit. I cannot hear if u speak so softly."

"...ok..let me start again. We are from the medical college. We are medical students. We are here to ask people to sign up for donating their bodies after their death. This will help medical students to use the body and learn about the human body. This will help us become better doctors."

"What will you do with it in case I donate my body"

"Our mortuary service will come and collect the body from your relatives. We will then clean up the body, embalm it and then keep it in our college for medical students to cut open and study it."

"That does not sound nice to me. What I have to offer are my stories, my experiences and my advice. How can u see all that if you cut me open?"

"Paatti….it is not like that…if you are alive we will get your advice and stories from you. But after you are dead, we cannot get those things. So instead of burning or burying your body, we get to use it to learn medicine. Do you understand?"

Parimala nodded with a smile. As she did not say anything further the boys got up and left. As the boys were leaving Parimala called out to them, "Here…come back…give me the card…" She took the body donation pledge card that the boys gave, kept it in her safe box and smiled at them. Right across the road, her young neighbor Parvathy stood watching this and smiled at Parimala. The boys reminded Parimala of her young son. Her mind drifted back to her past.

"Amma!!! Damn it! I am so ashamed of you. When they invite parents for school meetings, can't you at least wear a proper saree like all the other mothers? You look ugly, you wear dirty torn sarees, and you don't even know to speak properly. I wish I had a different mother"

Parimala's son was embarrassed by her. When Parimala got pregnant, she was so excited. She felt like there was some meaning to her miserable existence. Despite Kicha's beatings and abuses, seeing her son was the single most consoling part of her life. But her son grew up fast and learned to dislike her, just like Kicha.

"Chee…go away. I don't feel like coming near you. Don't touch me, your ugliness will stick on to me."

As Parimala watched him run out of the home and join his friends on the street to laugh and play, it felt like he had trampled on her heart. Parimala felt guilty that she had brought her son into this world full of ugliness and hatred. She was guilty that she had brought shame and sadness to her innocent son, by her ugliness and lack of class.

Every time she stood in front of the mirror, molten lava started erupting inside her filled with acidic bitterness of self-loathing and shame.

Her father hated her, Kicha disliked her and now her own son was embarrassed by her.

She beat her face in hatred, "Why am I born this way? Why am I so worthless? Why? Why?" She desperately looked for an answer as tears mixed with the small streak of blood that started flowing from her broken lip.

The bitter taste of blood in that memory faded, returning her to the dim, cold cowshed in the dark hours before dawn. Parimala took one last, slow breath against the hard floor. Suddenly all the pain had disappeared. Parimala was lying on the same cowshed, but now she was feeling free. She was watching her body spread eagled on the same old tattered saree on the floor, but she was not inside the body. She looked around and understood she was dead. She was liberated. Death was indeed blissful. As she watched, her daughter in law came in, saw her body lying there and called out to her son. Her son came in, and with a disgusting look on his face put his fingers under his mother's nose. He must have felt no air moving past and he declared "The old woman is gone. Let us prepare for the funeral. Let us finish it off today"

As he started preparing for the funeral, Parvathy, the friendly neighbour called out,

"Don't be in a hurry…I think Parimala amma has donated her body to the medical college",

"What do you mean donated the body?"

"Wait, I kept that phone number somewhere safe." Said Parvathy and rushed into her home and brought out a visiting card.

"Take this card. Call this number. Parimala amma has donated her body to the medical college. If you call them, they will come and collect the body. They will use it for teaching medical students."

Parvathy was always a kind and intelligent lady. As Parimala heard her speak, she wanted to give her one last hug.

As Parimala watched, her son made the call. They wrapped her body in an old saree and kept it on the bench just outside the house. She looked very much relieved.

"Thank God the old woman spared us of all the funeral expenses" said the daughter in law.

These words would have hurt Parimala if she had been alive. Parimala did not feel the weight of gloom descending on her whole body and crushing her whole system under its pressure. Death was indeed blissful. Parimala wondered if it was possible to stay unaffected even when alive. She wished she could go back to the young Parimala and give her some of this feeling as gifts at various moments in her life.

Parimala saw her father standing there towering over her diminutive figure and yelling at her. Little Parimala was crying, as she always did those days. Parimala touched and caressed little Parimala's head gently. Suddenly little Parimala stopped crying and fell asleep. Parimala had just transferred the gift of equanimity briefly to the little one. Then Parimala moved forward some more years. Kicha was on top of young Parimala, violating her with all his might. Young Parimala was lying there like a log of wood, her face flinching in pain with the grossly abnormal position in which her legs were twisted by Kicha's violence. Parimala moved close to young Parimala and caressed her eyebrows. For a moment the creases on young Parimala's forehead disappeared and she lapsed into blissful equanimity. Then Parimala moved on a few more years and there older Parimala was standing receiving the verbal blows of her son's harsh words. Parimala rushed in and gently rubbed older Parimala's shoulder, and suddenly her tears stopped and there was a gentle look of peace on her face. Some more equanimity to the older Parimala ever thirsting for love, compassion, kindness, and respect.

Parimala had just a few more moments left before she vanished into eternity. In the quiet, brightly lit dissection hall of the medical college, twenty young

students gathered around the stainless-steel table. Parimala watched as they stood in a moment of solemn silence before beginning their lesson.

With careful hands, they held their scalpels and forceps, moving over her worn skin not to strike, push, or discard, but to learn. Every incision was guided by focus and quiet reverence. In life, her body had known only burden, neglect, and violence. Here, in death, twenty pairs of gentle, earnest hands treated her remains with solemn honor. For the first time in eighty-three years, Parimala had found her purpose—and with it, her quiet eternity.

Double burden

Sukanya stepped into the clinic with three other family members accompanying her. They were her father, mother and elder sister. She was wearing a dark brown night gown. Sukanya was a tall woman with dark brown hair. Her slender eyebrows were pitch black and looked like they were drawn with a pencil. There was a dark green shadow of recently shaved facial hair on her chin and upper lip. As she sat down in the chair beside me, she was looking beyond me through the window. There was an air of discomfort about her.

Sukanya's sister was the one who started speaking, "She is unwell since 2 weeks doctor. She keeps saying she has severe abdominal pain."

I looked at Sukanya and asked her, "Can you tell me more about this pain? Where is the pain in your abdomen. Can you pinpoint the area?"

She looked at me and pointed to her whole abdomen with the palm of her hand. "This whole place is paining." There was a quiet distress in her voice. This distress was mixed with frustration and disappointment. I got a feeling that she has been brought to the clinic against her will.

"Doctor, I came to see you two months back. You found that the blood level in my body is very low. You gave me some injections and I became well. You are very lucky for our family. That is why we brought her here" Sukanya's mother seemed worried and angry at the same time.

"We took him to the big hospital last week. They admitted him there and did a lot of tests. They asked him to stay back. But he disappeared from the hospital. We were searching for him everywhere and then we came back home. Yesterday suddenly he shows up at home." Sukanya's father was furious and his voice was raised and tense. The father's reference to Sukanya in the male gender was not lost on me.

"Let me examine her and see what is wrong." I said this and guided Sukanya to the privacy of the examination room.

"Would you like a lady attendant to be with you while I am examining you?"

"Yes, my sister can come."

Sukanya and her sister came into the examination room. Sukanya got on to the examination couch. There she lifted her night gown. Underneath she was wearing a tight pants. She exposed her abdomen and lay down on the couch.

"I am going to touch some areas of your abdomen to see what is wrong. Let me know if it hurts"

As I started palpating, I could feel the abdomen was hard and my fingers could feel multiple rounded swellings inside the abdomen. As part of clinical examination we are taught to feel around with the fingers to identify important structures in the abdomen. A skilled clinician can draw a detailed mental image of what is inside by just touching the various parts of the abdomen. I couldn't exactly understand what was going on. It felt like there was a huge mass in the abdomen with many rounded projections on its surface. This did not fit in with anything I have seen before. As I was feeling the abdomen Sukanya did not wince in pain, she was lying there calmly, but with a very apprehensive look on her face.

By now I was quite sure that Sukanya was a transgender woman. But I did not want to slam the question on her face. I wanted her to feel comfortable, understood and respected in the clinical space.

"When did you undergo the surgery?"

When I asked this question with a lot of inner hesitation, Sukanya paused for a moment, and locked eyes with me. There was this look in her eyes that showed me the important questions has been asked and answered. Something vaguely resembling a smile appeared on her face. "Four years ago. I got it done in Puducherry in a private hospital"

Now that I got my confirmation I confidently asked her "Are you taking hormones now?"

"No, I am not getting hormones. I could not afford to continue treatment with that private clinic in Puducherry. So I am not on any hormones now"

I completed the examination and asked her to get up and come to the consultation room.

As I started compiling my clinical notes I faced Sukanya and said, "This is not a simple abdominal pain. While examining your abdomen, I could feel a swelling inside. It seems quite big. We may need to do scans to understand what it is."

"When I was admitted in the big hospital, they did all scans. They told me they want to put a tube and camera through my mouth and visualize my stomach. But I did not want that test and so I came away."

Sukanya's father and mother were standing and watching this interaction. Her father was furious. "He is always doing things in haste. Who asked him to run away from the hospital. He is always like this"

I could see a wave of sadness spread all across Sukanya's face. She did not say anything, but slouched further into her chair and closed her eyes. A tear dropped down from her eye.

"Have you brought all the scan reports? May I see them?"

Sukanya opened her eyes and there was absolute dismay on her face. "Oh! God! I completely forgot…" she whispered.

Sukanya's father started yelling at her mother, "See what he has done. He is a fool. Doesn't he know that he must bring all reports when he comes to see a doctor. He is a totally useless fellow."

Sukanya's mother gave him a harsh look, silenced him and said, "Doctor, we will go and bring the reports. Please help us."

"Sukanya's blood pressure is very high. I will give her some tablets and start her on an intravenous fluid. Let her rest here. Can someone go and get all the reports?"

Sukanya's mother and sister ushered her father out of the room. Their village was some 6 Km from our clinic. The father had a small scooter in which he had come. So they managed to convince the father to go and get the reports. "The reports are kept on top of the steel cupboard" Sukanya shouted from the treatment room.

"Who asked him to keep it there? Who is going to steal the reports?" Sukanya's father kept mumbling as he went out to get the reports.

After starting Sukanya on the intravenous fluids, it was time for a short tea break. I walked to the staff resting area behind the clinic and picked up my cup of cool buttermilk. Sukanya's mother came and sat next to me on the bench just outside the clinic. We sat there silently for a minute. "You know doctor, he was a very good boy. Suddenly when he turned 10 years he started behaving very strangely.

He ran away from home 10 years ago." Saying this she started sobbing.

"I understand amma! It can be very hard to lose a child"

"We faced so many insults and humiliations after he ran away. One of our relatives saw him in Chennai, begging on the streets wearing a skirt and top like a girl. Since then we have not been able to walk with our head held high. We haven't attended any family occasions, festivals or celebrations. We are living like outcasts."

"You must be happy that she came back now"

"She came once last year. Her father beat her up and chased her away. For him the humiliation of having such an oddity of a son is too much. We are poor people and if the society isolates us, where will we go?"

I did not know what to say. I have met and interacted with some transgender women who have never gone back to their families. Here at least Sukanya has a family that came with her to the hospital and cares for her.

"She came back this time because she is very sick doctor. Otherwise she wouldn't come here. There is nobody to take care of her in her world. So she has come back to us." I couldn't understand what exactly Sukanya's mother was feeling. It seemed like a mix of happiness that her 'son' came back, sadness that she came back only because of the sickness, and the looming fear of what could be wrong with her.

My break time was over and I excused myself and walked back to the clinic. As I was seeing patients, within the next 20 minutes Sukanya's father was back with the reports. He walked into the consultation room and handed over the file to me. I offered him a seat.

The reports were not reassuring. All blood tests were normal. But her abdominal scan had shown a large mass in her abdomen. The scan was non conclusive. The doctor who had reported the scan had specified that there is a high likelihood that it could be cancer of the stomach that has spread into the abdominal space. The report also recommended an endoscopy of the stomach to visualize the cancer and take some bits of the tissue to test and confirm whether it is cancer. From whatever Sukanya told me and from the interactions with the family it felt like they were not aware of the seriousness of her condition.

I went into the treatment room. Sukanya was lying down with her eyes closed. The intravenous fluid was just about to finish. Her sister was sitting by her side holding her hand and caressing it gently. The love between the siblings, the concern of the sister for her brother's welfare and the warmth that was exchanged transcending barriers of gender made my heart heavy. I pulled a chair and sat in front of them. The father and mother came and joined us from outside the room.

"Sukanya, how are you feeling now"

"Better doctor. The stomach pain has reduced"

"Good, I just now saw all the reports that your father brought from home."

"Doctor, can we talk alone? I want to hear from you first and then talk to the family"

I developed a deep respect for Sukanya's wishes and requested the parents and sister to wait outside.

"Sukanya, looks like the swelling you have in your abdomen is not a simple swelling. The scan report says that it could be cancer. We may have to do some additional tests to confirm that it is cancer. We then have to plan the treatment."

Sukanya's face twisted in pain. The pain of hearing the bad news was evident on her face. Within a few seconds she broke down and started weeping. I sat there silently by her side waiting for the emotions to drain and pass.

"Doctor, do you think I can handle this on my own? Will I need my family support?"

"I do not think anyone should handle cancer alone. You will need all the support you can get"

"Not like that doctor. I am asking whether I will be able to walk, talk, move around, cook for myself, clean myself and such things. If I cannot even do these things then I do not want to be a burden on my family. I will go away."

I did not know how to answer that question.

"It is too early to make such decisions. First, we need to confirm the diagnosis. Then we need to plan the treatment. Once we plan the treatment, we can understand how dependent you will be."

Sukanya was crying profusely now.

“I have already caused so much heart break and trouble for my parents and my sister. Because of me, my sister is not even getting married. I wish I can go away and just die” Sukanya was uncontrollable by now.

I put a hand on her shoulder and calmed her. I had no words to make her feel better. I could understand what she was thinking and feeling.

“I don’t want to be that selfish person who left the family during good times and came back to haunt them during sick times.”

“Do you have a social support system in Chennai? What about your foster family? Do you live with them in Chennai?”

“Yes, I have an amma and akka in Chennai. Both are transgender women like me. We work together and live together. But they are also struggling to make ends meet. They also beg for a livelihood. They do not have jobs. How can I go back and burden them?”

I had no idea what to tell her or how to help her. After a few minutes of sitting there in silence I said, “Let us take one thing at a time. We have to tell your family that you need further tests. I will refer you to the government hospital where you will get all your tests done. Then once the results are out, we can sit together again and plan how to move forward with treatment.”

Sukanya had quietened by then. She did not object to this plan, neither did she agree to it.

As I got the family together and started explaining everything, I could see that Sukanya was keenly watching each of them. Her gaze turned from her sister to her mother, her father and back to her sister. The family did not have any reaction to this information.

“We will take him to the government hospital tomorrow and get him admitted there” said Sukanya’s father as a matter of fact. I wrote the letter and gave it to Sukanya. She got up and sat on the cot. One by one her family walked out of the treatment room and went outside. Sukanya lingered. I understood she wanted to say something. I looked at her.

"I don't want to be a burden on them doctor. I will go back to Chennai tonight. I will take the treatment in Chennai GH. Please don't tell them about this. If I make it out of this illness, I will come and meet you some time."

As Sukanya walked out, there was a certain determination in her step.

Touched by brilliance

The oppressive afternoon heat inside the clinic was thick with the scent of floor cleaning liquid and surgical spirit. Overhead, the ceiling fan rotated with an unhurried, rhythmic drone that did little more than stir the warm, heavy air. It was not a busy clinic. On days like this, reading a book is the best way to pass the lull between patients. It was almost noon, when I heard the ringing of anklets announcing the visit of a young woman. I looked up from the book to see a young girl in a yellow and black checked cotton chudidar. Walking just behind her was a taller girl, about the same age as her. From the look on their face, it was clear the shorter girl was the leader of this duo. They walked in and sat down in front of me.

"Doctor, I have been suffering from a viral fever for the past 1 week. I am better now. But I need a certificate from a medical officer stating this fact and declaring me fit to join college."

"What is your name? Which college do you go to?"

"I am Abhirami. I am studying third year MBBS in the government medical college in the city."

I gave Abhirami her medical certificate and fitness certificate. I learned that she and her friend Sandhya live in the neighbourhood. They started coming to the clinic regularly, not as patients, but as earnest young trainee doctors eager to discuss with me.

"Tell me, Abirami, what is the longest muscle in the human body?"

"The sartorius muscle, sir. Arising from the anterior superior iliac spine and inserting into the superior medial aspect of the tibia via the pes anserinus."

"Which cranial nerve innervates the muscles of facial expression?"

"The seventh cranial nerve, the facial nerve, sir."

"And precisely where is its motor nucleus located?"

"In the caudal part of the pontine tegmentum, situated deep within the reticular formation, ventrolateral to the abducens nucleus."

“First-line regimen for intensive phase pulmonary tuberculosis?”

“Isoniazid, Rifampicin, Pyrazinamide, and Ethambutol for two months, followed by a continuation phase of Isoniazid, Rifampicin, and Ethambutol sir.”

“Classification of leprosy according to the Ridley-Jopling scale?”

“Tuberculoid, Borderline Tuberculoid, Mid-Borderline, Borderline Lepromatous, and Lepromatous leprosy, sir.”

“Treatment of choice for severe Leptospirosis?”

“Intravenous crystalline penicillin G, six million units daily in divided doses, or parenteral ceftriaxone, with oral doxycycline for milder presentations, sir.”

I would quiz her every time she visited. Abhirami always had all the answers. Not a flicker of pride showed on her face; her expression remained as unreadable and still as calm water.

Sandhya who accompanied her every time, sat with her chin propped firmly on her palms, utterly spellbound. Sandhya looked at Abhirami with pure, unadulterated awe.

“Sir, I have no clue how on earth she does it? Most of us spent the last three nights before exams barely sleeping, surviving on canteen tea and half-memorized mnemonics just to avoid getting humiliated in viva rounds. Yet she recites entire medical volumes as if they were written in her own handwriting. It isn’t just brilliance. She works harder than all of us combined.”

Sandhya was not exaggerating. In their entire MBBS batch, I heard that Abirami was a living legend, an enigma who lived entirely on the margins of student life. From the very first week of their anatomy dissections, she had followed an unbroken, monastic regimen. Long before the hostel wardens unlocked the mess gates at dawn, Abirami was already seated in the dim corner of the central library, her head buried in thick, cloth-bound volumes of Robbins Pathology or Guyton’s Physiology. Sandhya told me that she never lingered at the college canteen, never participated in inter-collegiate cultural festivals, and never took part in the casual banter that filled the hospital corridors between clinical postings.

Her appearance was as unvarying as her discipline: simple, starched, faded cotton salwar suits, hair pulled back into an oiled, uncompromising braid, and worn-out flat footwear that bore the scuffs of endless walking. Some days Sandhya would come alone to the clinic. She would say that in every terminal assessment and professional exam, the distance between Abirami's top score and the second rank was a vast chasm—regularly fifteen to twenty marks. To the rest of the batch, who dubbed her behind her back as "Queen of Answers," "Steno," (stenographer who makes meticulous notes of the lectures) and "Mugma," (one who memorizes all facts) she seemed almost robotic. But Sandhya, watching her day after day, saw something entirely different. She saw a girl fighting a silent, desperate war against unseen circumstances, using knowledge as her only weapon and armor. As Sandhya said this, my respect for Abhirami grew.

The clinical years had intensified the demands on their time. Their visits to my clinic became less frequent. During their pediatric posting, Abirami had been assigned to take the complete clinical history of an eight-year-old boy admitted with chronic glomerulonephritis. She had spent hours at the bedside, gently asking the mother about every detail—the onset of facial puffiness, the color of the urine, previous streptococcal throat infections, their socio-economic background, and the family's water source.

When she presented the case to the Head of Pediatrics, her presentation was flawless, her diagnostic reasoning watertight, and her proposed treatment plan textbook-perfect. The professor had publicly lauded her, declaring it one of the finest student presentations he had heard all year.

As they walked away from the bedside, the child's mother, a frail woman in a faded synthetic saree who had watched Abirami's gentle examination of her son, caught Abirami's sleeve. "Hello, you spoke so kindly to my boy. You answered every question the big doctor asked. You are going to be a wonderful doctor. God bless you, child."

When Sandhya narrated this whole incident to me, all Abhirami had to tell me was, "What does that poor mother know? A patient's blessing doesn't pay the rent. It doesn't put food on the table, and it won't buy the expensive textbooks I need. Only marks and degrees can change reality."

The turning point arrived on a blistering Tuesday afternoon, just four weeks before the commencement of their final-year MBBS university examinations.

Abirami had spent the morning in the general medicine wards, examining cases of valvular heart disease. During the midday seminar, sudden, sharp menstrual cramps seized her lower abdomen with incapacitating intensity. Pale, sweating, and lacking any pain medication, she decided to take the rare step of leaving campus early to rest for a few hours in her home before the evening study marathon.

The house was located in a narrow, congested lane, a dark, single-room portion with damp walls and a rusted corrugated roof. As Abirami turned into the alleyway, her heart gave an unexpected jolt. The front wooden door of the house was unlatched and standing half-open.

Under normal circumstances, the house remained padlocked throughout the daylight hours. Her mother had passed away of severe pulmonary illness when Abirami was just twelve years old, leaving her to shoulder the entire domestic burden. Her father was a chronic alcoholic whose daily life revolved around the local liquor shop; he only stumbled home late at night to collapse into an intoxicated stupor, contributing nothing beyond an occasional, begrudging handout for rent and basic provisions. Her younger brother, Bala, now thirteen, was an eighth-standard student at a nearby government corporation school who was never home before five in the evening.

A cold dread knotted in Abirami's stomach. She pushed the creaking wooden door open and stepped inside.

An overpowering, sweetish, acrid smoke hung thick in the dim, unventilated room. On the bare cement floor sat Bala, still wearing his blue school uniform trousers, his shirt unbuttoned and drenched in sweat. His head was slumped sideways against the wall, his eyelids fluttering, his pupils dilated and glassy. Sitting around him were two older boys from the street corner, passing a rolled paper between them with low, mocking laughter.

The shock was so sudden, so violent, that the world seemed to lose its sound.

"Bala!"

The cloth bag in Abirami's hand crashed to the floor, her heavy medicine textbook tumbling out onto the dust.

The two older boys jumped to their feet, startled. Before they could move, Abirami surged forward with a terrifying, primal fury. She seized the nearest boy by his collar, dragging him toward the door with strength drawn from

sheer adrenaline. “Get out! Get out of my house before I call the police!” she screamed, shoving them out into the street and slamming the heavy wooden door shut, shooting the iron bolt into place with a deafening clang.

She turned back to the room, her chest heaving. Bala remained where he was, his head lolling backward, a pathetic, vacant grin spreading across his slack lips.

“Bala! Look at me!” Abirami dropped to her knees, seizing his shoulders and shaking him violently. “What have you done? What did they give you?”

“Akka... leave it na...” Bala mumbled, his tongue heavy and thick, his words slurring into a sickening drawl. “It feels nice... everything is light... don’t shout, Akka... just one puff... just wanted to feel light like Appa...”

The mention of their father struck Abirami like an electric shock. Tears of pure rage and terror sprang to her eyes. “Light? You want to be light like that animal?”

She raised her hand and struck him across the face—a sharp, stinging slap that echoed in the tiny room. Bala groaned, his eyes rolling back. Desperate, Abirami rushed to the kitchen corner, seized a plastic tumbler of cold water, and hurled it directly into his face.

Bala gasped, sputtering, but the drug had taken too deep a hold. His limbs went limp, and he collapsed sideways onto the frayed reed mat, sinking into a heavy, unresponsive stupor.

Abirami stood over him, trembling from head to foot. The silence that filled the room was suffocating. She walked to the small table where her final-year textbooks—heavy, authoritative tomes of Harrison’s Internal Medicine, Bailey *&* Love’s Surgery, and Dutta’s Obstetrics—sat stacked in neat, reverent piles.

Mechanically, she prepared a few dry rotis, forcing herself through the motions of an elder sister, and tried to feed Bala when he stirred briefly hours later. He swallowed two bites without opening his eyes and fell back asleep.

Night fell like a shroud. At eleven-thirty, the front door rattled violently under heavy, erratic thuds.

Abirami unlatched the door. Her father barged in, reeking of cheap country liquor, his clothes disheveled, mud caked on his trousers. He did not even look at his children. He stumbled straight toward the tiny, attached bathroom, groaning

and clutching his abdomen. Moments later, the violent sounds of retching tore through the silence, followed by a heavy, wet thud.

Abirami walked slowly to the bathroom door and pushed it open.

There lay her father, sprawled face-down on the damp cement floor in a pool of his own vomit and bile, utterly helpless, breathing in ragged, stertorous gasps.

Abirami looked down at him. Then, she turned her head and looked at the reed mat where thirteen-year-old Bala lay sleeping, his face young, vulnerable, and already marked by the shadow of the exact same poison.

She sat down beside her open textbook under the flickering yellow light of the single bulb. The printed chapter on cardiovascular therapeutics stared back at her. Tomorrow morning, her entire batch would assemble in the examination hall for the decisive first paper of General Medicine. If she walked into that hall, she knew with absolute certainty that she would emerge with the highest distinction in the university. She would wear the white coat, hang the stethoscope around her neck, and earn the title she had sacrificed her entire youth to achieve.

And then what?

If she went to college tomorrow, she will write the exam. She will pass. She will begin her internship. She will spend thirty-six hours at a stretch in the hospital wards, checking blood pressures, inserting intravenous lines, and treating the fathers and sons of strangers. And while she is away, what happens to this room?

She looked at her father lying in the bathroom dirt.

He will not live another year. His liver is failing; she can see the jaundice in his sclera, the spider angiomas on his chest. And when he dies, who will guide Bala? Those boys will come back tomorrow. They will put ganja in his hands again. In two years, her little brother, the only soul in this world who shares her mother's blood will be lying in his own vomit on this very floor, destroyed beyond repair.

What is the meaning of being a doctor if she cannot save the only person who depends on her for his very life? What will she do with a gold medal around her neck when she has to visit her brother in a mental asylum or identify his body in a mortuary?

Medicine taught Abhirami anatomy, pathology, pharmacology. But it never taught her how to resurrect a lost soul. If she stayed there, chasing her dream, she

would be choosing her degree over her brother's life. She can study medicine at twenty-two, or thirty-two, or never. But Bala only has one childhood. If she does not pull him out of this abyss tonight, he is gone forever.

For years, Abirami had believed that her tears had run dry on the day her mother was cremated. But in that dark room, an immense, crushing weight pressed against her chest. She did not weep. The crisis demanded not sorrow, but absolute, ruthless action.

Her mind, honed by years of rapid diagnostic decision-making, locked onto a singular treatment plan.

Moving with swift, quiet precision, she went to her father's discarded trousers, reached into the pockets, and pulled out every crumpled banknote he possessed. She went to the tin box hidden beneath the kitchen stove and retrieved her own small emergency savings—accumulated rupee by rupee over years of skipping meals and walking miles instead of taking the bus. She packed two changes of clothes for Bala, a single cotton saree for herself, and their birth certificates into a worn canvas duffel bag.

She did not pack a single medical book.

She knelt beside Bala, pulling his limp arm over her shoulder, and hoisted his weight with all the strength in her slender frame. She walked out of the house, pulled the door shut without looking back, and stepped into the midnight darkness. A light rain had begun to fall as she dragged her brother through the deserted lanes to the central bus terminus, boarding the first long-distance government bus that had a board reading "Madurai." As she was boarding the bus, she called me on my phone.

"Sir, I am leaving the city. I may not be coming back any time soon…" and narrated the whole course of events between deep sobs.

"Abhirami, don't go. Come to my home. Let us sit and discuss and decide what to do"

"Sir, I must go to my mother's home town. That is where we will feel protected and in our mother's loving embrace. We need to get far away from our father's influence and all the bad influences on Bala."

But long before I could convince her to stay back, she cut the line and the switched off the phone. I heard a determination and clarity in her voice that told me, that Abhirami was on an important mission.

In the weeks that followed, rumors swept among the students of Abhirami's batch like wildfire: that she had eloped, that she had suffered a breakdown, that she had died in an accident, that her conservative family had forced her into marriage. Sandhya listened to the gossip with a heavy, aching heart, refusing to believe the dismissive stories. She would come and share these stories with me in the clinic. Deep down, Sandhya knew that whatever had taken Abirami away, it was a battle far larger than any examination room. Eventually, the internships began, lives moved forward, and the legendary "Queen of Answers" faded into a haunting memory of their college days.

Four hundred and fifty kilometers away, in the temple city of Madurai, Abirami's new reality began in a cramped, unplastered room in the working-class neighborhood of Sellur.

Her first battle was for Bala's mind and body. She took him to the psychiatric out-patient department of the Government Rajaji Hospital, enrolling him in a strict de-addiction and counseling protocol. For the first three weeks, the withdrawal was brutal. Bala suffered violent tremors, cold sweats, night terrors, and fits of screaming rage where he begged and pleaded for a single dose. Abirami never left his side. She held him down when he shook, bathed his feverish forehead with wet cloths, and sat awake through the long, agonizing nights, whispering stories of their mother into his ear.

To survive, Abirami found employment as a salesgirl in a large, multi-story supermarket near the Madurai bus stand. Her shift began at eight in the morning and lasted until eight at night, standing on her feet on the polished tiled floors, assisting customers, arranging heavy crates of provisions, and managing the billing counters during peak hours. Her feet swelled, her back ached with a dull, chronic throb, and the palms of her hands, once accustomed only to holding delicate fountain pens and dissection forceps, grew rough and calloused.

Six months after their arrival in Madurai, her aunt had called her with some bad news. Her father had been brought into the casualty ward of the government general hospital in a state of terminal decompensated liver cirrhosis with massive upper gastrointestinal bleeding, and had passed away within hours of admission. Abhirami made a hard decision not to go back.

That evening, when Abirami returned from the supermarket, she found fourteen-year-old Bala sitting in the dark corner of their room, his knees drawn up to his chest, his face buried in his arms, weeping uncontrollably.

"Akka..." Bala choked out, his voice cracking with unbearable guilt. "Is it because of me? Did Appa die because we ran away and left him alone?"

Abirami set down her bag, walked across the small room, and knelt down in front of her brother. She reached out, took his tear-streaked face firmly in both her rough hands, and forced him to look directly into her eyes. There was no hesitation, no weakness in her voice—only the fierce, protective authority of a mother.

"Listen to me, Bala. Look at my face," she commanded softly. "You did not kill him. And I did not kill him. The alcohol killed him years before we ever got on that bus. He had destroyed his liver, his mind, and his conscience long ago. Do you hear me?"

Bala sniffled, his chest hitching. "But... we left him, Akka..."

"We left a burning house so you wouldn't burn inside it," Abirami said, her voice steady and clear. "I did not throw away my entire life and carry you on my back to this city just to watch you drown in useless guilt. Our father is gone. But you have me. From this moment on, I am your father, and I am your mother. You do not look back. Your only duty in this world is to wake up every morning, study with everything you have, stay clean, and grow into a righteous, honorable man. Do you understand? Everything else, the rent, the food, the fees, the world outside that door, that is my responsibility. Leave it to me."

Bala looked into his sister's fierce, unwavering eyes and saw not an elder sister who was struggling, but an immovable mountain shielding him from the entire world. He nodded slowly, wiping his tears with the back of his hand.

From that day forward, their life became a disciplined machine. Abirami's extraordinary intellect, once applied to clinical medicine, was now directed toward the supermarket's logistics. Within a year, the management noticed that the quiet salesgirl could calculate inventory deficits faster than their computerized billing systems, managed supply chains with effortless precision, and handled difficult customer disputes with unflappable calm. She was promoted to floor supervisor, and within three years, became the assistant store manager, earning a respectable salary that allowed her to move them into a decent two-room

house and enroll Bala in an excellent higher secondary school. Within a year Abhirami also found herself a night shift job as an assistant in a nursing home. Along with some extra cash, she got the opportunity to keep her clinical skills alive and active.

Bala worked with a ferocious, single-minded devotion, treating every textbook as a sacred debt he owed to his sister. He cleared his twelfth-standard board exams with exceptional marks, securing a merit seat in Mechanical Engineering at the prestigious Government College of Technology in Coimbatore.

Four years flew past in a blur of academic triumphs. Bala graduated at the top of his department, earning three gold medals and securing a coveted, high-paying corporate engineering position with a global manufacturing firm headquartered in Mumbai.

On the evening before Bala was scheduled to board the train to Mumbai to begin his corporate career, the brother and sister walked together to the ancient Meenakshi Amman Temple in the heart of the south Indian city that never sleeps.

After offering their prayers, they sat side by side on the massive granite steps of the temple's Golden Lotus Tank. The evening sky was painted in shades of deep indigo and amber. Across the water, the towering temple gopurams stood illuminated against the twilight, their brass bells echoing in a deep, resonant cadence.

Bala, now a tall, broad-shouldered young man of twenty-three, dressed in a sharp formal shirt and trousers, turned to look at his sister. He looked at her simple cotton saree, her hair tied in the same neat braid, and the fine lines of fatigue and sacrifice etched around her 32 year old eyes.

He reached out and took her hands into his own. He felt the hard calluses on her palms marks left behind by ten years of carrying crates, managing retail floors, and counting invoices to pay for his college fees.

"Abhi," Bala's voice began, thick with emotion.

Abirami smiled gently. "What is it, engineer sir? Getting nervous about living in Mumbai?"

"Do you remember that rainy night we left from Chennai?" Bala asked, his voice trembling slightly. "I was barely conscious, but I remember waking up on that bus, seeing your face against the window glass, wet with rain. I was a thirteen-

year-old junkie, Abhi. I was heading straight into the gutter, straight into the same grave as Appa. You were twelve hours away from becoming a doctor. You had the world at your feet, and you threw it all away into the dust, just so you could drag me out of that hell and keep me alive."

Abirami reached up and gently patted his cheek. "You were my baby brother, Bala. A piece of paper with 'MBBS' written on it would never have kept me warm if I had lost you. Seeing you standing here today, healthy, successful, and proud that is the greatest degree I could ever earn."

"No, Abhi," Bala said, his tone turning fiercely resolute. He reached into his leather shoulder bag, pulled out a thick, sealed white envelope, and placed it reverently into her hands.

Abirami frowned. "What is this?"

"My company's joining advance and signing bonus," Bala said, closing her fingers over the envelope. "And every month from now on, half of my salary will be transferred directly to your bank account. The rent, the groceries, the household—everything is my duty now. Your work as a manager in that supermarket is over. You resigned this morning, Abhi. I went and spoke to your owner myself."

Abirami stared at him, stunned. "Bala, what are you talking about?"

"You sacrificed your youth to give me a life," Bala said, tears glistening in his eyes. "Now it is my turn to give you yours. You didn't memorize every line of medicine just to check grocery bills in Madurai. The medical university allows candidates to re-register and clear pending final-year examinations. I have already inquired with the registry in Chennai. Pack your bags, Abhi. You are going back to Chennai. You are going to write those final papers, complete your compulsory rotating internship, and hang that stethoscope around your neck. You are going to become the doctor you were born to be. Please, Akka... let me be the brother you deserve. Give me the honor of taking care of you."

Abirami looked down at the envelope in her hands, and then up at her brother's earnest, loving face.

For twenty-four years, through her mother's death, her father's drunken rampages, the terror of Bala's addiction, and the brutal grind of retail labor, Abirami had remained an unbreakable fortress, refusing to shed a single tear.

But looking at her brother standing before her as a grown, honorable man offering her life back to her, the fortress crumbled completely.

She leaned forward, buried her face into Bala's shoulder, and broke down into deep, racking sobs. The decades of stored pain, fear, loneliness, and exhaustion poured out in a torrential flood of tears, soaking his shirt, while overhead, the evening sky broke into a gentle, cleansing drizzle that rippled the waters of the temple tank.

Eleven years after that fateful rain in Chennai, inside the administrative cabin of the Mangulam Rural Primary Health Centre located thirty kilometers outside Madurai, Dr. Sandhya sat behind her desk, reviewing the block's antenatal registration rosters.

Sandhya's life had followed a smooth, predictable trajectory. After completing her MBBS, she had married Suresh, an ophthalmologist, completed her diploma in public health, and joined the state government health service. For the past three years, she had served as the Chief Medical Officer of this rural PHC.

A knock sounded on the wooden frame, and the senior public health nurse, Sister Shanthi, stepped in.

"Doctor, the new Junior Medical Officer assigned to our PHC has arrived to report for duty. Her posting orders are from the Directorate of Public Health. Should I send her in?"

"Yes, Sister, send her in," Sandhya said without looking up from her file.

The door creaked open, and a pair of neat, practical leather sandals stepped onto the tiled floor.

"Good morning, Dr. Sandhya."

The voice calm, precise, with that unmistakable, resonant clarity struck Sandhya like a physical shock wave. Sandhya snapped her head up.

Standing before her was a woman in her early-thirties, dressed in an immaculate, starched indigo cotton saree, her dark hair neatly bound in a clip. Her face was unpainted, devoid of jewellery except for small gold studs, but her posture was as straight as a spear, and her eyes held a profound, luminous depth that commanded instant respect.

Sandhya's pen slipped from her fingers, clattering against the glass tabletop. She rose from her chair, her eyes wide with utter shock.

"Abirami?!"

A faint, warm smile touched Abirami's lips. "Hello, Sandhya. It has been a very long time."

"Abirami... Oh my God! Is it really you?" Sandhya rushed around the desk, grabbing Abirami's shoulders in an impulsive, joyous embrace. "Where did you go? Where were you all these years? On the day of the medicine exam, you just vanished into thin air! We asked everyone, searched everywhere! What happened to you?"

Abirami gently placed her hand over Sandhya's arm, her touch steady and comforting. "Life took an unexpected detour, Sandhya. I had an urgent responsibility I had to fulfill. But once that was done, I went back, cleared my final examinations, finished my internship at the government general hospital, and joined the primary health service. And now, I have been posted here under your leadership."

Sandhya stared at her, overwhelmed by a flood of emotions. Looking closely at Abirami, Sandhya realized that the distant, defensive coldness of their college days was gone. In its place was an immense, grounded serenity—the unmistakable poise of a woman who had walked through hell, rebuilt a world with her bare hands, and emerged entirely undefeated.

"I am so proud of you, Abirami," Sandhya whispered, her eyes shining. "You have no idea how much I missed seeing you."

Later that week when Sandhya called me to inform me everything, I was so surprised to hear about Abhirami. Sandhya had stayed in touch with me, but Abhirami had just remained a memory, a fond one that we would periodically revisit during our regular telephonic chats.

The real trial of their reunion occurred three days later, in the grand conference hall of the Madurai District Collectorate.

It was the mandatory monthly maternal and infant death audit meeting, presided over by the District Collector, an austere, no-nonsense IAS officer known across the state for his zero-tolerance policy toward bureaucratic incompetence. The previous month had been catastrophic for the Mangulam block: five neonatal

deaths and one maternal death due to severe postpartum hemorrhage had been recorded in the tribal hamlets under their jurisdiction.

Inside the air-conditioned hall, the atmosphere was freezing, yet Sandhya was sweating profusely. She sat in the front row with a thick bundle of case sheets, her fingers trembling as she reviewed the clinical notes for the hundredth time. She had spent the entire previous night preparing textbook defenses calculating the exact dosages of oxytocin administered, the timings of the ambulance dispatch, the hemoglobin levels upon admission hoping to prove that the PHC staff had followed clinical protocols to the letter.

At the head of the long conference table, the Collector opened the Mangulam file with a sharp snap that made several medical officers jump.

"Dr. Sandhya!" the Collector's voice boomed across the microphone, sharp and cutting as a scalpel. "Five infants dead in thirty days! And a twenty-two-year-old mother dead of postpartum hemorrhage because she reached the tertiary hospital four hours too late! You are the Chief Medical Officer of this block. According to the inquiry report, the village health nurse visited her home twice, yet the mother delivered at home in an inaccessible tribal pocket. What kind of monitoring is this? Why should I not recommend an immediate departmental inquiry and suspension for gross clinical and administrative negligence?"

Sandhya stood up, her throat completely dry, her knees knocking against the desk. "Sir... the patient refused institutional transport... we administered the standard uterotonic protocol as soon as she arrived at the PHC... the protocol guidelines were strictly adhered to, sir..."

"Protocols?" the Collector interrupted, slamming his pen onto the teak wood. "Protocols do not bring a twenty-two-year-old mother back to life! I don't want textbook recitations of what drugs you injected after she was already bleeding to death! I want answers as to why she wasn't in an institutional labor room a week before her due date!"

Sandhya stood paralyzed, tears of humiliation and panic prickling behind her eyelids.

From the second row, a figure rose calmly to her feet.

"Collector Sir, with your permission."

The Collector looked past Sandhya, his brow furrowing in annoyance. “Who are you?”

“Dr. Abirami, Junior Medical Officer at Mangulam PHC.”

“Dr. Abirami, this is an audit of systemic failure, not an academic clinical conference. Do you have a defense for why this patient was brought to the center in irreversible hypovolemic shock?”

Abirami stepped out into the central aisle, standing tall, her hands resting naturally in front of her. Her voice, steady, resonant, and entirely devoid of fear or defensiveness, filled the quiet room.

“Sir, we do not have a defense for a human life lost. And no administrative penalty imposed on Dr. Sandhya will alter the fact that a child has been orphaned,” Abirami began evenly. “With due respect, Sir, we are spending this entire audit demanding answers to the wrong questions.”

A collective gasp went around the room. Several senior deputy directors stared at Abirami in disbelief, gesturing frantically for her to sit down. But the Collector held up a hand, his sharp eyes narrowing as he fixed his gaze upon her.

“Explain yourself, Doctor,” the Collector said, leaning forward.

“Sir, for the past hour, we have been asking: ‘Why did this mother fail to reach the hospital in time? Why did the staff fail to force her onto an ambulance?’” Abirami stated with unwavering clarity. “Those questions are designed only to fix individual blame and close an administrative file. But the real questions we ought to be asking are systemic: Why does a tribal hamlet located only twelve kilometers from our PHC have no motorable road during the monsoon season? Why does our public health outreach rely solely on a single auxiliary nurse midwife who is expected to trek across a swollen river without transport support? Why does our community awareness model treat anemic tribal mothers as passive recipients of iron tablets rather than addressing the structural food insecurity that keeps their baseline hemoglobin at six grams?”

The room fell into an absolute, breathless silence.

“Sir,” Abirami continued, her gaze meeting the Collector’s without flinching, “when a patient arrives at our PHC in hemorrhagic shock, our medicine can only do so much. But if this administration wishes to prevent the next maternal death, we must stop asking for post-mortem excuses. We must start asking the

uncomfortable, proactive questions about infrastructure, nutrition mapping, and mobile health access before labor begins. If you permit me, Sir, I have drafted a preliminary structural proposal for an early-warning maternal tracking system specifically for our block's high-risk tribal pockets."

The Collector sat back in his high-backed leather chair. For nearly thirty seconds, not a sound could be heard in the vast hall except the low hum of the air conditioners. The Collector looked at the junior doctor at her steady eyes, her fearless demeanor, and the unmistakable, penetrating wisdom behind her words.

Slowly, the stern expression on the Collector's face softened, and a rare, genuine smile broke across his features.

"What did you say your name was, Doctor?"

"Dr. Abirami, Sir."

"Dr. Abirami," the Collector said, his voice carrying a warmth that astonished every bureaucrat in the room. "In my fifteen years of civil service, I have sat through hundreds of audit meetings where officers give me a thousand rehearsed answers to save their own skin. You are the first medical officer who has had the courage and the wisdom to tell me that my questions were wrong."

He turned to the Chief District Medical Officer. "Cancel the show-cause notices for Mangulam PHC. Instead, I want a District Maternal Safety Task Force constituted by tomorrow morning, headed by Dr. Abirami and Dr. Sandhya. Doctor, bring your proposal to my office at ten o'clock tomorrow. Let us see if we can ask the right questions together."

"Thank you, Sir," Abirami nodded with a graceful inclination of her head.

As the meeting adjourned and the crowd began to filter out into the wide marble corridors of the Collectorate, Sandhya walked alongside Abirami, watching her friend with a sense of wonder that surpassed even their college days.

Ten years ago, in the lecture halls of their youth, Abirami had been the brilliant, solitary prodigy the "Queen of Answers" who could recite medical texts from memory but kept the world at a safe, unfeeling distance. But the girl who had known all the answers had been broken open by life, by poverty, by the fierce crucible of maternal sacrifice, and by the relentless labor of love.

Sandhya realized that the greatest doctors are not formed by textbooks and examination halls alone, but by the trials of life itself. The girl who had once been the undisputed queen of answers had returned tempered by suffering, elevated by sacrifice, and crowned as a true master of wisdom. I never got to meet the two girls again, but Sandhya still keeps in touch and tells me all about the clarity and decision making brilliance that Abhirami brings with her. As doctors, sometimes our lives are touched by the brilliance of our patients.

A bath that healed the patient and the doctor

Most of my memories from internship are from the General Medicine postings in Government General Hospital, Chennai. It was in the old building. The building was quite dilapidated. In fact, in the months immediately following my postings in the department, the roof of one of the wards caved in and that led to shutting down of the ward and discussions about construction of the new building that stands as the twin towers of the Government General Hospital today. It was the middle of the week and things usually settle to a slow pace at this time. Our priority would be to discharge patients who are well so that we empty some beds before Sunday, which is our admitting day. The smells of a typical Government Hospital ward are not very pleasant. There is a typical odor of hospital disinfectant that is overpowering, as we enter the ward space. As we move closer to the nursing station, the smell of opened medicine vials will mix with the hospital disinfectant smell. On the far end of the ward is the bathroom and toilet space, and as we move closer to the back end of the ward the stench of urine, feces and other bodily fluids blunt all other smells. Sometimes, we have patients with rotting flesh of cancer, or diabetic foot ulcers admitted in the medicine ward for dressings and treatment of other medical conditions. In such situations the smell of rotting flesh undeniably mixes with all other smells and overpowers our olfactory senses. It was a routine weekday, and I was there in the ward at 7 AM. The first thing we do in the morning is to take over patients from the night team. As I was finishing up the take over of patients and scribbling notes into my 'to do' list for the day, I could distinctly smell rotting flesh. I asked my friend, "Let me guess, diabetic foot ulcer? When did the patient get in?"

My friend who had just woken up with a sprained neck because of sleeping leaning on the table in the nursing station all night, gave me an irritated grin, but did not reply. I went to the classroom at the front end of the ward, to put my things in the cupboard there before I can start the day's work. It was a lame excuse of a cup board. It was just a space where I could keep my bag. The door did not lock, in fact, it did not even close fully. As I kept by bag in the cupboard and walked back to the ward, the distinct smell of rotting flesh again welcomed me. I wanted to find out where the smell was coming from.

I could see there were no new patients in any of the beds. They were all patients I had attended to till the previous evening. None of them had any rotting flesh on them. It had to be someone on the floor. In Government Hospital wards, for every patient on a bed, there would be one or more on the floor. These 'floor patients' were usually given a mat and a pillow and they would be accommodated in the

space between the beds. For purposes of identification we would number the 'floor patients' as bed number 12f, 13f and so on, meaning the one on the floor near bed 12 and 13 respectively. Sometimes more than one patient would be squeezed all around bed 12, in which case the numbering would be 12f1, 12f2 and so on. Ofcourse, these numbering are not official. The administration would never allow us to admit patients on the floor and we cannot officially show them on record. The nurses in charge of the ward would juggle the numbers and reports to show all of them on some bed or the other. I realized that the smell was coming from the 'floor patient' 33f. Bed 33 was in the far end of the ward on the right side, just next to the bathroom and toilet complex. I walked to that area and found the patient lying on a gurney top without the wheeled bottom portion that is used to transport patients.

It was a young man covered in a white hospital bed sheet. The first thing I noticed was that he was tall and his hands and legs were dangling out of the gurney top. His head was tonsured. His eyes were closed. His entire body was covered by the sheet and so I could not immediately recognize where the ulcer was that was giving rise to the stench. As soon as I went and stood there near him, the person occupying bed 33 started complaining.

"They brought him and put him there last evening. He has not even moved or opened his eyes once since that time. I can see his chest is moving up and down and so life is still there. But he is completely out. And oh my god…the smell. Can you do something about it?"

Immediately some more occupants of the beds nearby started talking together and saying something about this young man. I moved closer to the young man and the smell got stronger. I squatted down next to his bed and gently moved his sheet to see what was beneath it. The young man was just wearing a white shorts and nothing else on his body. There were multiple cuts, bruises and wounds all over his upper chest, hands, thighs, legs and sides. It looked like he had been beaten up. When I moved the sheet further, I saw the reason for the strong smell. There was a deep ulcer over his right foot. it was filled with pus and maggots. The smell and the sight were revolting. This man must be in severe pain, but he was completely knocked out. I tried rousing him by gently shaking his shoulder and calling him out. He did not respond. I put back his sheet and got up and walked to the nursing station.

"Sister, do you know who is that patient on the floor next to bed 33?"

"Doctor, that case is from Institute of Mental Health. They brought 'it' last night and dumped 'it' in the ward and left. We have not yet taken over the case. So, no need to see or write a case sheet. The plan is to shift 'it' out to Institute of Mental Health as soon as morning rounds are over. So don't bother. See the other patients" Staff in many government hospitals referred to patients using inanimate pronouns in Tamil "athu" "ithu", especially when the patient does not fall within their jurisdiction and is not their 'headache'. The 'it' irked me every time I heard it. But this time it hit me really hard.

This was the first time I had seen an inmate from the Institute of Mental Health being brought into the General Hospital for admission. I was not aware of the formalities and the administrative issues involved. But it did not make sense that a patient lies in the ward unattended, just because the formalities are not completed. But I was just an intern and did not have the courage to do anything more about the bureaucracy. I went about attending to the other work in the ward, seeing the other patients, recording their blood pressure, checking their lab reports, drawing blood samples for tests and filling out their case sheet with progress notes of that day. Each time I would move across bed 33, I would notice him lying there completely unattended and not moving.

The postgraduate residents started their rounds around 8 AM and I accompanied them for their rounds and took down their instructions. Then the senior doctors came for rounds and the was also completed. All the morning rounds were wrapped up and still the young man from IMH had not moved. As the senior doctor was getting ready to leave the ward, I walked up to him and said, "Sir, that patient transferred in from IMH is lying in that corner. What should I do for him sir?"

"Don't you now behave like you are the only responsible person in this ward. We all know that he is there. Even we don't know what is wrong with him. Nobody has handed over the case to us and we have not taken over. You just mind your work. Don't act too smart…run along" he said, dismissed my enquiry and moved on.

I did not realize how time moved on from then. So many things had to be done, and I started working on them. Through all the work my mind kept drifting back to 33f. 2-3 PM is usually the time for writing the daily indent for the ward. It is the process where we take stock of all the medicines and materials required for the next day and write them in an indent note and send it across to the hospital store to procure them. I went to the beds to collect all the case sheets to tally up the requirements of medicines and injections. As I passed bed 33, I noticed that

the young man was awake. He was making moaning sounds. I put all the case sheets on the bed 33 and kneeled next to him. I gently touched his hand and asked him, "What is your name? What happened to you?"

He did not reply. I could see fear in his eyes and he looked away immediately. In a few seconds there were tiny drops of tears welling up in his eyes and they dripped down through the sides of his face. But there was no expression in his eyes. By then the person in bed 33 saw our little interaction and said, "He must be hungry. I think we must give him something to eat. I have some bread, biscuits and a banana" and handed a packet with 3 slices of bread, 2 pieces of biscuit and a ripe banana. I took it from him and showed it to the young man. His eyes widened and he lifted his left hand and grabbed the packet. As he lay there, he opened up the packet and gobbled the bread and biscuits in a few hasty bites and then peeled the banana and ate it too. Three of us, the man in bed 33, another patient standing nearby and me were watching this whole scene and the man in bed 33 reached out and handed over a bottle of water to the young man. The man held the bottle but was unable to drink it. He must have been thirsty, his Adams apple moved up and down in swallowing movements, but he did not open the bottle. I understood that it must be because he was afraid to drink water lying down and wanted to get up. All three of us moved to his gurney top and supported him by his shoulders and propped him up. He immediately opened the bottle and drank the water in rapid gulps. A sudden look of peace and quiet spread all over his face.

"Let him remain seated for some time. Poor guy…." said the man in bed 33. And we moved the gurney top closer to the wall and propped him against the wall with a pillow. Meanwhile the sister yelled, "Where is the intern. Has the indent been written yet?" I ran with the case sheet to the nursing station to finish up the indent. By the time I was done with the indent writing it was already 4 PM. I was very hungry and rushed out to grab a quick and rather late lunch. By 5 PM the post graduate residents would come for evening rounds, and I must be there to take down instructions for the night duty. As the rounds progressed and we came to bed 33, I asked the residents, "Sir, what should we do about this young man? I still haven't worked him up or even made a case sheet." By this time the young man was sitting up and staring at us.

"There is nothing we can do. The seniors must sort out the status of this man. He is neither in our ward census, nor in IMH census now. We have no official handing over documents. Don't do anything. Let us cross this night duty. Tomorrow the

head of the unit will come for grand rounds and at that time we can make a definitive decision" said the resident and moved on.

My colleague who was supposed to do night duty informed me that he will take over later than usual and asked me to hang around till he came. I wound up my work by 6.30 PM and had some time to kill. I went to bed 33. The young man was sitting on the gurney top and picking on some of the scars on the wounds in his hands. As I went and sat on the bed 33, he looked up at me and locked eyes.

"What is your name?"

He did not speak, he just moved his left hand and caressed his tonsured head. The man in bed 33 saw this and said,

"He is touching his tonsured head. Maybe his name is Mottai (mottai in tamil means tonsured head)" and he laughed. Still the young man did not speak, but smiled a big toothy smile. When his face lit up with the smile, I realized that the man sitting there was such a handsome person. A bit of cleaning up and he will look normal.

"Do you want to get cleaned up? Do you want to have a bath?" I asked.

The young man looked at me eagerly. I had never bathed any patient before this. One part of me was wondering whether cleaning up a patient was an intern's job. I was specifically asked to ignore this young man. What if some senior doctor found me cleaning him up? Would I get into trouble?

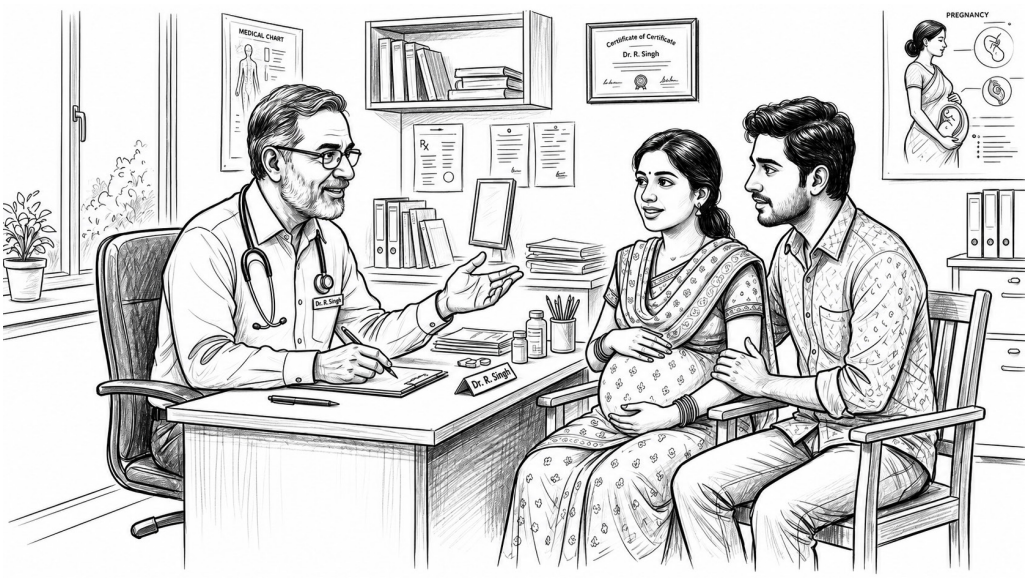
I was too scared to even ask the housekeeping staff to support me. What if she went and reported me to the staff nurse, and what if the staff nurse reported me to the senior doctor. Nobody in the government hospital ecosystem liked extra work, and any request for extra work would be viewed with suspicion and anger. I overcame all the internal struggles and confusions, took a piece of cloth from the ward cup board, took some soap, borrowed the plastic mug from bed 33 and slowly made Mottai get up and walk towards the bathroom.

Mottai walked slowly but eagerly into one of the bathroom stalls. I rolled up my pants and sleeves. I was all set to begin the 'mission clean up'. But then the infected and maggot infested ulcer was staring at my face. I put everything on the shelf nearby and ran to the ward. I picked up a dressing tray when the staff nurse was not watching and rushed back to the bathroom. The man in bed 33 was amused by all this and gave me a thumbs up. I opened the tap and let the water run on Mottai's foot ulcer. As the water fell with force on the ulcer it

washed off the maggots, the pus and the blood clots. Mottai was wincing in pain, but did not move. I knelt down and gently wiped the wound with gauze. Once the ulcer was all washed and cleaned, I poured some antiseptic solution on it and washed it thoroughly. Mottai enthusiastically joined the effort and took the mug of water from me and started pouring it over his head and washing himself off. There were so many cuts and bruises all over that he was wincing with pain. But still he needed the cool water to wash off days of dirt and grime off his wounded body. I helped him apply soap and gently scrubbed off the dirt. When the bathing was done, I dried his foot ulcer and applied an antiseptic cream on it and covered it in sterile dressing.

Amost half an hour later, around 7.15 PM when Mottai and me walked out of the bathroom, there was a happy smile on his face and a look of absolute triumph on mine. A cool bath on a hot summer evening can do wonders to someone's mood. But it was the first time that I realized that the said bath could do greater wonders to the one who gave it. As we walked to the gurney top, my colleague walked into the ward to take over from me. He saw me with my pants wet from all the water splashing and a wet white coat that I was wearing and started laughing. I explained the situation to my friend and asked him to ignore the young man as was the instruction given to us. I rushed to my hostel room and brought back an old t shirt and track pants for Mottai to wear. That night when I walked back to the hostel for my dinner, I felt a sense of satisfaction and happiness that I hadn't gotten over the past several months working as an intern in the hospital. In the bathroom that evening, not only had I washed off the dirt from Mottai's body, but the dehumanization and institutional neglect that is all pervading in a government hospital setting!

Social Complexity, Conflict and Communication



Patient encounters that are complex due to social dynamics and social conflicts, where communication is the only way out!

From young love to parenthood

The young couple were seated in front of me in the bus. I could only see their back. There was something endearing in their body language. I tried not to keep staring at them, but I couldn't. The guy was tall, a full foot above the back rest of the seat in front of me. The girl was shorter I could see only the top of her head from my seat. They were both facing forward, not each other. The way his head tilted to the left made him look like the horizontal crossbar of a T. Through our 20 minute bus ride he would straighten up a bit but would always come closer to her to his left. They looked very young, must be college-going, and such young love is so beautiful to see. It was a cloudy November morning. The time was around 7.30 and there was a pleasant breeze from the window. The guy was sitting close to the window, and his slightly long hair would fly and brush against her head. The visual was so lovely, I thought it would make for a great photograph. The bus came to our clinic, and I stood up to get down. The couple also got down in my stop. I was running late for clinic and so I rushed from the bus stop into the clinic.

As soon as I got into the clinic, I went to the staff room to quickly have my breakfast before I started work. There is a back door between the staff room and the consultation room. But that day the door was locked and so after my breakfast I walked into the clinic through the front door. Just outside the front door is the patient waiting area. There is a bench there for patients to sit. The young couple were sitting there. I was surprised that they had come to see me. The guy was seated in the corner, and the lady was sitting next to him. Her legs were on his lap, and he was pressing her legs. There were lines of worry written over both their faces. As I went past them, they noticed me and looked up. I smiled at the them and walked into the clinic.

After some 5 patients it was the young couple's turn. Before entering, the young man knocked the door and said, "excuse me, may I come in?" It is extremely rare to witness such courtesy in the clinic. I was pleasantly surprised and welcomed them into the room with a warm smile. "Sit down please" I offered them both seats.

"Which of you has come to see me?"

"We have come to get treatment for her…" said the young man.

"Tell me, what is the problem?"

Pavithra was a 19 year old girl. She had just finished studying BSc biochemistry in a private arts and science college in the nearby town. Rajesh was 22 years old. I asked them how they are related. I was pleasantly surprised to know that they were married.

"We got married 4 months ago. From the month of marriage, she has not had her periods even once. We did urine pregnancy test last month. But it was negative. We are not sure what to do. So we came to see you" said Rajesh.

I was surprised that they came to see me. I am not a gynecologist. Moreover, they had travelled from the town into a rural clinic to see me. There were good hospitals and clinics there. Why would they do that? I decided that I will ask them all that later.

"Before marriage were your periods regular?"

"Ohh! Absolutely, like clockwork. Exactly the first week of every month. Never missed even once"

"Let us get a pregnancy test anyways. Last month's test could have been false negative. Let us get it confirmed. If it is negative, we can do some other tests and find out why you did not get your periods" The couple thanked me and went to the lab that is attached to the clinic.

Pavithra collected a small plastic bottle given to her by the lab to collect her urine. As she walked into the patients' rest room she kept looking back at Rajesh with apprehension. Rajesh nodded at her in a reassuring manner. Pavithra stood right next to the rest room door hesitating to get inside. Rajesh ran to her, held her hand, caressed her head. "Let us get it done. I have a feeling it will be negative. Don't worry"

Pavithra gently pushed him away and said, "I told you not to say that. I don't want it to be negative" and she winced.

"Let us see. Whatever is good will happen. You go and get it done."

The couple came back to my room in 15 minutes. The pregnancy test was negative.

"See, I told you, there is no need to worry." Rajesh reassured Pavithra and held her hand. Pavithra freed her hand and looked at me. She did not look pleased.

“Doctor, then why am I not having my periods?” Pavithra was almost in tears. Suddenly something had changed between them. Pavithra seemed very disturbed, but Rajesh was reassured and calm now.

“Doctor, she is not sleeping properly. Not eating properly. She is very stressed. Could that be the reason?” asked Rajesh.

Pavithra placed a hand on Rajesh’s shoulder, looked up at him and said, “Rajesh…please stay quiet. Let doctor speak.”

“Irregular period could be because of low hemoglobin level in the blood. It could also be because of a condition called polycystic ovaries. We can do a blood test and an ultrasound scan to identify if you have any of these”

I wrote down a prescription for the blood test and the scan. I also prescribed the iron tablets anyways, as Pavithra appeared anemic. They thanked me, got up and left. After 10-15 minutes, I saw them through my window. They were not walking hand in hand, as they had come in. Something had strained between them after the pregnancy test came negative. I made a mental note that I must talk to them when they come for the next visit.

The next week Pavithra came alone to show me the blood test and scan reports.

“Pavithra, your husband did not come today?”

“No doctor…” Pavithra dragged the “no…” and did not volunteer any further information.

“Your blood hemoglobin levels are very low as I suspected. I am going to increase your treatment for low hemoglobin”

Pavithra nodded. “After this will I start having regular periods?”

“We can see how things go. Mostly you should become normal after correcting your low hemoglobin levels”

I wanted to try and understand the relationship dynamics between Pavithra and Rajesh and the role it might play in her health.

“Is anything the problem between you two? Is there anything I can do to help?”

That was just the nudge that was required and the ice broke. Pavithra started crying. After composing herself,

“Ours is a love marriage doctor. We married against the wishes of our families. We are living alone now. I miss my parents and brother. I feel like I have nobody.”

“I understand Pavithra. It can be very stressful to live away from the people we love. Is there any way you can re-establish relationship with at least your mother initially and slowly repair the family bonds again.”

“I keep calling my mother on phone and sending her text messages and voice notes. But she is not responding doctor.”

“Stay positive Pavithra. Things will turn around soon.”

“I was hoping that I would be pregnant. Pregnancy will probably bring my parents and family closer to me”

“Don’t worry, we can try and fix your irregular periods. That will help you feel better and maybe even increase the chances of a pregnancy”

“Ok doctor. I am hoping it can happen soon”

“What is the trouble between the two of you?”

“Nothing serious doctor. I want to get pregnant soon. But Rajesh is saying we should go slow and take some more time. That is our problem. We keep arguing about it.”

“Come back next month and tell me if you got your periods. And next time try and bring Rajesh. I will also talk to him.”

Pavithra took leave and left. Her walk was loaded with seriousness and anxiety.

In about 20 days I saw Pavithra and Rajesh again in the clinic. They were in the waiting area as I walked in. As usual, they sat pressed together, as if quietly stealing warmth and comfort from each other’s touch. This time when they looked up at me, I could sense a deep vulnerability in their eyes, as though they wanted to pour out their angst to me and seek help. I went in and started seeing patients.

“Doctor, I got my periods last week. It was normal. I am happy that the periods came. Will it now be regular?”

“That is good news. It should become regular now. I would like you to maintain a diary of when you are getting the periods and let me know”

"Thank you doctor. I was telling you about the pregnancy. I really am keen to get pregnant doctor. Should we undergo some tests?"

"I would request you to please go and get your iron tablets and come. Meanwhile I will have a small chat with Rajesh" I said this and sent Pavithra away to the pharmacy.

"Rajesh, how are you? How are you feeling?" I wanted to get an understanding of Rajesh's expectations and plans regarding the pregnancy.

"I am fine doctor. Did Pavithra say anything about me?"

"No, I wanted to know myself how you are doing"

"I don't know if she told you, we got married against the wishes of our parents. Life is very tough now doctor. I am trying hard to work and bring money home. But however hard I try, I don't know how we are going to manage, if a baby comes into the picture."

Rajesh was doing gig work, delivering parcels and courier for various companies in the town. The obvious hard work, late night parcel delivery, irregular food habits, and sleep habits were taking a toll on him. There were dark circles around his eyes and his eyes were blood shot with all the sleep deprivation.

"Pavithra was telling me that you are having disagreements regarding the pregnancy. She wants the pregnancy now and you want to take time."

"Doctor, we are so young. What is the hurry? She wants to use the pregnancy to get closer to her family. I don't think that is a good enough reason for a pregnancy."

There was some sense in what Rajesh was saying. It was a hard situation. I just listened quietly.

Pavithra came back to get Rajesh.

"Doctor, have you both spoken? What is Rajesh saying?"

I smiled at her and sent them both on their way home with advice to return after 3 months.

They both went out and sat down on the bench outside the clinic.

Pavithra started talking, “Rajesh, please listen to me. My periods have started coming regularly. Let us plan the pregnancy.”

Rajesh looked the other way and avoided eye contact with her. He put his hand through his long hair and ruffled it. He wanted some clarity. They sat like that for a few minutes and then Rajesh got up and walked towards the free condom vending machine kept outside the clinic.

Four months rolled by. One day as I entered the clinic, I saw them seated in the waiting area. They were sitting next to each other, but a little gap had crept between them. While Pavithra smiled at me as I entered, Rajesh had a sour look on his face.

“Doctor, she is pregnant. We came for the check up.”

I knew Rajesh was not very keen on the pregnancy. Pavithra looked elated and she was glowing with happiness and the beauty of her first pregnancy.

After registering her for pregnancy care and doing the routine tests, I sent Pavithra with the staff nurse for her tetanus injection.

“Rajesh, are you ok? How did you decide on the pregnancy?”

“Doctor, I decided that her happiness is more important than everything else. I can work harder and put money on the table. But she was being miserable. If this pregnancy will make her happy, I decided that is what we should do.”

Over the next six months, their visits became a routine marker of time in our clinic. Pavithra’s anemia resolved, and her pregnancy progressed well, but the bright promise she had attached to the pregnancy began to show its cracks.

“Doctor, I called my mother to tell her I am 7 months pregnant and asked her if we can have a baby-shower. She just coldly told me off and asked me not to call her again.” Saying this Pavithra broke down and cried. The realization hit Pavithra like a physical blow: a newborn could not bridge a gap built on pride and broken expectations.

The day she wept in my consultation room over her mother’s rejection, it wasn’t the doctor who comforted her first. It was Rajesh. He pulled his tall, weary frame out of the chair, knelt beside her, and gently wiped her tears.

“Don’t cry, from now it is just the two of us. As long as we are together, we are fine. Now we will also have the little one with us to give us strength.”

In that small, quiet gesture, I saw the young boy from the bus transition into a father. He was still delivering packages late into the night, still exhausted, but the sourness in his posture had dissolved into resolve.

A few weeks after her delivery at the district hospital, I walked out into the patient waiting area on a cool morning. The December air felt remarkably similar to the day I had first seen them on the bus. They were sitting on the wooden bench outside our door. Pavithra was cradling a small, yellow-wrapped bundle in her arms, looking down with a peaceful, settled smile. Rajesh sat right beside her, his long legs folded awkwardly, his left shoulder tilted down so his head rested close to hers—just like the horizontal crossbar of that T, I had observed months ago.

They were no longer two frightened kids leaning on each other to keep from falling apart. They were a family, built not on the approval of the past, but on the quiet, hard-won grace of choosing each other every single day. As I stepped forward to greet them, Rajesh looked up, smiled warmly, and nudged Pavithra.

“Doctor,” he said softly, standing up to hold the door open for his wife and child. “May we come in?”

The chest pain that was not a heart attack

Quiet days in the clinic are deceptive. They are usually when the most complicated stories walk through the door or perhaps, they are simply the days I have the quiet mind to hear them. It was a quiet day and when Manimegalai walked in, little did I know that her situation would be so complex. Manimegalai was a tall woman. Her most remarkable feature was the brilliant smile that she had on her face. She was wearing a dark blue kurta with small white flowers. She was holding a little baby in her hand, which I understood was 'little Manimegalai' because there was a striking resemblance. Both were dusky beauties with resplendent smiles. 'Little Manimegalai' was tiny, very much underweight for her age of 2 years and 10 months. I thought she was my patient, thanks to my cognitive bias of thinking that the malnourished child must be sick! I asked the mother to take her seat. She placed the baby gently on the floor and she sat down. Little Manimegalai rushed out to play with the other ladies and children waiting outside. The mother did not seem to be bothered. As she sat down and made herself comfortable, I took the case record from her hand and started reading the notes. The staff nurse in the front had noted down that Manimegalai had 'chest pain' and had recorded her blood pressure, pulse rate, height and weight. Everything looked normal.

"Please tell me, what brought you to the clinic today?"

"Doctor, I work in an export company as the supervisor. Our company packs biscuits and prepares them for export. My job is to stand all day and supervise the work. I am having severe chest pain since the past 2 days"

She came across as a smart woman. She gave me a lot of details in the few sentences that she spoke. All the while she maintained a very cheerful demeanor.

"Tell me about this chest pain. Does it hurt continuously, or does it appear and disappear periodically?"

"It is there most of the time doctor. It is there even now."

Usually patients would grimace, or wince or point to a place on the chest when they complain of chest pain. She was still smiling. She spoke with a strange, detached calm as if she were describing the pain of a stranger sitting across the room. "Does the pain get worse when you exert yourself like climb stairs, or walk fast?"

"It is there all the time doctor. No change."

I asked a few more questions that would help me identify the origin of the pain. It did not sound like a heart or a lung condition. It sounded like a musculoskeletal pain. I examined her heart sounds and chest using my stethoscope.

"Looks like it is a muscle pain in your chest. I will give you some pain killers. You will be fine."

I took out my pen to start writing a prescription and I looked at her. She did not seem convinced.

The smile had slowly faded from her face and she got up and looked outside the room at her daughter. She convinced herself that little Manimegalai was fine outside and came and sat down again.

"I am afraid I am having a heart attack doctor"

I did not expect this. I had to tread this very carefully. I was quite sure this was not a heart attack. In my experience, a person with a heart attack does not walk in casually and narrate the symptoms like it is happening to someone else. But I did not want to dismiss her doubt like that. I did not want to judge her. I had to keep my judgments and thoughts to myself. I nodded at her acknowledging her concern.

"You should not worry. Let me explain to you why I think you are not having a heart attack…" I started explaining.

I like explaining things to patients. I like making lists while I am explaining things, point 1, point 2, point 3…and so on. It gives me the satisfaction of simplifying complex medical topics. But Manimegalai did not seem to be impressed. The smile on her face faded. It felt like a dark cloud had covered her face.

"Can you please write a prescription for an ECG for me? I think I am having a heart attack."

The change in her smile, the darkness in her mood and this strange request, made me understand that there was more to it than just a fear of heart attack.

"Manimegalai, what happened? Why are you worried of a heart attack? Can you tell me what makes you think that?"

I needed some more room to understand what is running in her mind. I was

not going to scribble a prescription for an ECG just because she asked for it. The underlying problem could be deeper than what it seems.

"Last year my husband developed a sudden heart attack and died. I was there by his side when he held his chest in pain. Before we could do anything he just died."

She blurted it out without expression. The protective armor of her smile vanished, leaving behind only the stark, hollow grief beneath. It all made sense now. The bland affect, the exaggerated attempt at being cheerful, the disembodiment of her symptoms. I gave an understanding nod and wrote out a prescription for the ECG. She quietly took the prescription and walked out of the room. But I felt a vague discomfort. This was because of the conflict between the rational medical brain which did not want an ECG, and the understanding emotional human brain which wrote the prescription.

There was a spurt of activity with a few patients after that and Manimegalai walked back in half an hour. The smile was back on her face. She sat down and showed me her ECG. It was not like she was anticipating my results from the interpretation of her ECG. It felt like she already knew that everything was fine. I did not even have to look at it, but I did anyways. It was normal as I had expected it.

"Thank God! the ECG is normal. It does not show a heart attack. Now please go and take your painkiller medicines. You will be fine soon."

She was not too thrilled or relieve to hear this.

"Thank you so much doctor" as she was preparing to leave little Manimegalai ran towards her and pressed herself against her legs. She lifted the baby and held her in her arms. I smiled at the little one, and she gave me a brilliant big smile!

"Sometimes I wish she looked like her father. But look at her. She looks exactly like a photocopy of me. The only reassurance I have is that she carries some parts of her father for me to remember him."

Little Manimegalai must have been just over 1 year when her father died. She was a cheerful kid, playing with everyone she saw, socializing with everyone. It was a very poignant scene to see big Manimegalai walk out with the little one, the little weight too much for her to bear!

The clinic became quiet after they left. I was thinking about how irrational my case notes on Manimegalai would look. I had documented a detailed clinical history, physical examination findings, and even written that it is not likely to be a problem with the heart. But still I had ordered an ECG and documented the ECG findings as normal. Anyone who audited the case record would wonder why I ordered an ECG when I knew and wrote down clearly that it looked like a muscular pain. From a purely biomedical perspective, an ECG is a mundane recording of electrical currents traveling through cardiac muscle. An auditor reviewing my case notes that afternoon would see a contradiction: a clinician who correctly diagnosed musculoskeletal pain, yet ordered a cardiac test anyway.

But in clinical practice, diagnostic tests do not exist in a vacuum of pure logic. For Manimegalai, the ECG was never a tool of investigation; it was a therapeutic intervention. It was the physical artifact she needed to quiet her grief, ground her trauma, and reassure herself that she would survive for her child. Medicine is often taught as an exact science, but at the bedside, it remains an intensely human enterprise. Sometimes, the most healing thing a physician can prescribe isn't a drug or a lecture—it is a piece of paper that says, "You are going to be okay."

Clinic was closing and it was time to leave. I just felt grateful that I could connect meaningfully with Manimegalai that day.

The hard decision

The peak summer months were over, and the weather had started getting cooler in the early mornings and evenings. Travel to the clinic was relatively tolerable those days. We had just elected a new government to our state and there were some changes which were visible. One of them is the "Lioness task force" of trained police-women who were deployed all through the state to ensure safety of women in public places. With their black shirt and khaki trousers these strong and confident police-women were busy and active everywhere, especially in rural areas. As I travel using the public transport, I encountered these women in the bus stand and railway station, patrolling the area for women's safety issues. When I boarded the bus that takes me to the clinic, I found that there was only one vacant seat on the left side of the bus. This was a seat reserved for women. There was no crowd in the bus, nobody was standing and so I took the seat, of course with an understanding that I would get up when a woman boards the bus and offer her the seat. The moment I sat there, a black-khaki Lioness boarded the bus and came near me. She raised her voice at me and commanded that I immediately vacate the seat. I was just picking up my bag to get up. But the Lioness perceived that small time lag as hesitation to get up and started yelling at me, "You seem to be an educated person and look decent, but why do you behave like this and occupy ladies' seat. You must be ashamed of yourself to be creating a scene like this…" and so on she went. I got up and walked away to the other side. It was an unnecessary distasteful experience. Often when such overzealous programs are launched, they lead to goal displacement. The program was designed to protect women, but the officer had succumbed to goal displacement where enforcing the rule rigidly became more important than the reason the rule existed in the first place. For her, the ladies seat may be empty, but no man should sit on it! After navigating this. social oddity I reached the clinic, had a hurried breakfast and rushed into my room to start seeing patients.

"Good morning doctor. How are you?" Kumari was smiling even as she entered the room. Kumari is a 60 year old lady from a nearby village. She has diabetes and hypertension. Eight years ago, Kumari was an imposing presence tall, broad-shouldered, with thick hair pulled into a tight, defiant bun atop her head. She had a deep, hoarse, loud voice, which can be quite intimidating when she is unhappy. But that was not the Kumari who was standing in front of me now. I could barely recognize her. She had lost so much weight her skin was hanging loose on her neck. There were deep wrinkles all over her face. She also had a

stoop and looked substantially shorter than she usually does. Her voice was also husky and soft.

"Good morning Kumari, long time since I saw you. What happened? I don't even recognize you. Are you ok?"

Kumari flashed a big bright smile and showed me thumbs-up. "I am doing very well doctor. Everything is fine"

I did not believe what she said. It is unlikely that everything was fine. Kumari looked wasted and sick. But I did not want to sound rude. Anyways she has come for some problem, she will start talking and I can explore the reasons for her sickness. I gestured her to sit down in the chair next to me. "Tell me, what brought you here today?"

Eight years ago when Kumari started coming to our clinic, she was younger and in her middle-aged prime! She would walk in authoritatively, push her way into the clinic ahead of others who were waiting and would always have things her way. She would command the staff nurses and housekeeping staff and would make them dance to her whims. Her authority did not spare anyone.

"Your sugar is very high. It is more than 600 mg/dl. The glucometer is not even able to read the value accurately. I must start you on insulin now" I had once told her.

"I can't take insulin. Give me tablets." This was her consistent reply. I have tried everything in my power to try and get her on insulin to get her blood sugar levels under control. I have given her calm and patient explanations. I have threatened her with dangerous complications of uncontrolled diabetes. But nothing seemed to work. Kumari had a mind of her own, and it was a very strong and uncompromising mind. She decided that she will not take insulin and she will not have it any other way. She kept ordering me to write out a prescription for tablets. Though I was conflicted to keep writing the same tablets for her despite the high sugars, I did that, because if I refused to write her the tablets she would just use that as an excuse to not take any treatment!

After a brief pause Kumari started answering my question. I could sense that she had significantly slowed down and her voice was much softer.

"I came today to just see you and say that I am doing very well. I am taking the Siddha (A Tamil traditional system of medicine) medicines regularly. They are working like magic. I wanted to see you and tell you that I have finally found

the correct treatment for me." Kumari smiled saying this. I did not reply. I could sense a tone of derision in what she said. Knowing her for over 8 years, I could read between the lines of what she was saying. "I came to shove it in your face, that the Siddha medicine did what your treatment failed to do. I came to tell you what a waste your treatment was." I could hear this in her sonorous voice in my head! If it has been 8 years ago, she wouldn't have hesitated to say this. Her physical weakness and frailty had mellowed her significantly!

There was a distinct disconnect between what she was saying and the way she appeared. She was emphasizing repeatedly that she was doing well, but she looked very sickly.

"I am happy that you found your appropriate treatment. Is there anything I can do for you today?"

"Since you are asking repeatedly, and since I have come all this way, can you just check my blood sugar once?" Kumari sounded like she was doing me a favor by letting me test her sugar levels. I requested the staff nurse to get her finger stick capillary blood glucose test and let me know. The staff nurse came with Kumari's blood sugar values. It was greater than 600 mg/dl. Her uncontrolled sugar, loss of weight and sickly appearance told me that things weren't very good for her. But I had enough bad experiences with both Kumari and her son, that I was a bit wary of engaging in a discussion with her.

"Your blood sugar is still high. I know you don't like my saying this. But I still think you need insulin. You look very sick and seem to have lost a lot of weight. You need immediate insulin."

Kumari laughed a weak laughter, and got up and left. I wanted to get up and go after her, convince her to come back and take insulin. But at the same time, I was also irritated and felt bitter about the show-down that we had a few years ago.

Some 4 years ago there was a major show-down between Kumari, her son and me in the clinic. She had fainted and fallen in the field when she was working ten days ago. She was taken to the government general hospital, admitted there and found to have uncontrolled blood sugars. She was treated there with insulin in the hospital for a week and discharged home with instructions to continue insulin. She came to the clinic the very next day after getting discharged from the hospital.

"I will not continue insulin. Write me a prescription for my tablets" she came and ordered me. I was irritated. I usually do not lose my calm, but Kumari pushed me over the edge.

"You cannot keep doing this repeatedly. You have just had a serious medical event, been hospitalized and have come back. Even after this you cannot keep saying that you want tablets. I cannot give you tablets. You must take insulin"

Kumari got furious, stormed out and came back in an hour with her son. Both were furious. Her son yelled at me, "Why are you denying the treatment that my mother asks you? We have the right to take the treatment that we want. We are poor and so we come here. If we had enough money we would go to a pharmacy and nobody would dare to question us. You are not doing us any favors by giving us medicines. We are paying you for the medicines that we are buying. So don't show your high and mighty attitude to us. Give the tablets that my mother is demanding. If you refuse, I will complain to the authorities and make sure that your life gets miserable." He was abusive and threatening.

"Sir, I am not refusing to treat your mother. She is doing things which are harmful to her. She is refusing to take insulin. Insulin is a life saving medicine for her. Tablets are not working for her. I am advising her about this and she is not paying heed. I cannot just continue writing tablets for her when I fully know it is harmful for her. Please calm down and leave the clinic. I want you both to leave the clinic now. I cannot treat you anymore if you keep insisting on taking tablets." I firmly and clearly said this and sent them out. I hadn't seen Kumari or her son after that for 4 years.

In the present, within a few minutes of Kumari walking out, her son walked in. I was seeing him after 4 years since the altercation.

"Why did you send my mother away even after seeing her high blood sugar?" there was no note of courtesy in his voice. I did not even respond to him. He continued, "You are doing this consistently. You are just ignoring us and turning us away when we come seeking your help. Don't you see she is so sick? Do you want her to die?"

"Sir, you must calm down now. I did not send her away. Call her in, let us ask her what happened."

Kumari walked in with a weak smile on her face. I raised my eyebrows at her questioningly? She smiled and said "No doctor, my son is saying that I should take insulin. I will take it for some time now. Please give me insulin."

"What about the Siddha medicines? Are you taking them now?"

"The Siddha medicines are none of your business. We have asked the Siddha doctor and he says it is ok to take your treatment along with his medicines temporarily till the sugars are controlled. You just give us Insulin" said the son.

"Just like how you asked the Siddha doctor whether you can take both medicines at the same time, why did it not appear to you that you should ask me? I don't know what reactions will happen if I give you insulin and you continue to take the Siddha medicines" I said.

"You don't worry, just write your insulin prescription" said the son.

"This whole thing is a game for you, isn't it? I cannot write a prescription for insulin without knowing what other medicines you are taking. Insulin will reduce your sugar drastically. If you take some other herbs or medicines in addition, it can adversely affect your health"

"Are you once again refusing to treat us? Tell me. Tell me clearly that you won't treat my mother. Don't beat around the bush" By now the son's voice had raised several decibels. He stepped into my personal space, pointing a finger inches from my face.

I had three choices in front of me. I could ask them to get out and see another doctor. I was strongly inclined to do that. Second option was for me to use this golden opportunity to resume the treatment and write a prescription of insulin for her. The third option was to give her some immediate care and refer her to a higher centre for further management. I took the second option, wrote out a prescription for insulin and sent her away. I couldn't face them in the eye. I handed over the prescription in Kumari's hands and in a low voice said, "I can only say one thing. I hope you continue to take insulin and keep your sugars under control." I couldn't help but feel defeated in the battle!

It is not easy when a 'difficult patient' who has constantly disregarded my medical advice, and who has previously entered an unfair altercation with me calling me negligent, comes back to me seeking my care. It would have been easier if they had come straight out and said they wanted insulin treatment. I would have understood the change of heart and taken over her treatment. But

their approach was to once again berate me as being negligent and demand insulin from me. It made the decision a very difficult one. The moral distress associated with treating someone knowing very well that it is not the optimal treatment is very severe. The previous show-down between Kumari's son and me had damaged the doctor-patient relationship significantly. Though my duty as a doctor is to always put Kumari's best interests ahead of everything, I am human and the way her son treated me was not something I could forget that easily. It was difficult to feel the same motivation to treat someone with whom the therapeutic alliance had broken.

There are positives and negatives of my decision to take over her treatment despite a strained therapeutic relationship. I grabbed the opportunity to do what is best for Kumari. Though it was a hard decision against my instincts, it was patient-centered. It was rational and evidence based medicine and it was what I always wanted to do. It was a solution for my moral distress. I was happy that my goal did not get displaced like that of the Lioness with power and authority. I did not dismiss Kumari because she made me feel powerless! When a therapeutic alliance has been shattered by abuse, putting the patient first requires overcoming intense human instinct. I grabbed the opportunity to give Kumari what her body needed, but I knew this was a fragile victory. Was she taking insulin only as a temporary stopgap until her Siddha doctor told her otherwise? Had I truly served her best interest, or had I simply allowed myself to be bullied into a high-risk situation that will inevitably lead to burnout?

Medicine teaches us the science of prescribing, but it rarely prepares us for the emotional scar left behind by difficult encounters. I wrote the prescription, but the questions and the lingering bitterness remain.

Breaching the fortress

It was 12 noon, the clouds had closed on the sky, and it was dark, there was a gentle drizzle. The out-patient clinic was lean. The past few days had been oppressively hot and suddenly it was cloudy and pouring since morning. So, people had chosen to stay home. A man about 40 years of age, wet in the rain walked into our clinic. He was wearing a dark blue striped lungi, and a red shirt with the top 3 buttons open. His hair was unkempt and wet in the rain. There was a thick salt and pepper stubble. The look on his face was one mixed with anger, apathy, and irritation. Behind him stood a woman, much smaller than him in stature, with a grim look on her face and her eyes red, probably due to a lot of crying. His body language exuded reluctance as he walked slowly into the clinic and took the seat in front of me.

"Tell me sir, what brings you to the clinic," I said in my usual kind demeanour. He gave a shocked look filled with irritation and looked away. He remained quiet. The lady who had come with him spoke gently, "Sir, he has high sugar. We have come for treatment." The man immediately flashed a very angry look towards her and gestured at her with his hand to shut up. The lady immediately went quiet.

"Do you have weakness or tiredness? Have you lost weight? What is the problem sir? What brought you to the clinic today?", this was me in my politest manner. The man continued to glare at me. Very reluctantly, in a very low voice, looking away from me, not establishing any kind of eye contact he muttered, "sugar is high". I was not getting any information from him. Things were getting frustrating.

"Sir, I understand your blood sugar is high. But before I can review the laboratory test results, I need to understand what is happening in your body. Could you please answer my questions?" I tried to be very polite. He was almost ready to get up and dash out of the room. He just shifted in the chair and moved to the edge of the seat. Then looking away from me he moved an envelop, the lab test report towards me.

I looked at his wife, as she stood behind his chair. "Tell me ma, what happened? Why have you brought your husband here?" The demure lady looked at me with an expression of helplessness. So I gave up the idea of getting any information from her. I have a strict policy that I would not review any laboratory reports without first talking to the patient and examining them. I deviated from this policy and opened the lab report that he had thrust towards me. I saw that his

fasting blood glucose was in the 200s and the post meal blood glucose level was in the 400s. As I was reading the report and copying the values down into his case file, I could see from the corner of my eye that he did not even turn to my side. He was seated on the edge of the seat ready to leave at the earliest opportunity.

"Your sugar values are indeed high. Is there anything you would like to say? Are you having any problems?" I tried one last time.

"Give me medicines" those were the only words that came out of him. It went against my clinical training to just write a prescription based on lab reports without talking to and examining a patient. But I just wrote out a prescription of diabetes medications on his case sheet and as soon as I put my pen down, the man snatched the sheet and got up and walked out. I asked the wife to sit down so that I can talk to her to get some information. As soon as the man stormed out the wife said, "I am so sorry doctor. My husband did not want to come. I literally dragged him here" and she left too. She was too scared of him.

The rain outside seemed to match the cold, guarded wall he had built around himself. Sitting alone in the quiet clinic after they left, I stared at the blank space on my desk where his file had been. Palani was not an exception. There were so many like him. To Palani, my soft-spoken questions weren't welcoming; they were soft, weak, and invasive. He was a man built on a rigid code where toughness was celebrated, vulnerability was mocked, and accepting kindness was an admission of weakness. My polite, solicitous tone felt like condescension, or worse, a trap. Every time I softened my voice, I was unknowingly asking him to lower his shield, relinquish a little control, and a man like him would rather bleed out than lay down his arms.

In medical school, I learned that empathy is my greatest instrument. It works very well with most of my patients. They taught me that a soft voice, an open posture, and gentle questioning will unlock even the most guarded patient. But watching the door swing shut behind Palani, I realized my textbook warmth hadn't opened a door—it had threatened to tear down a fortress he needed to survive. When I see patients like Palani, I treat them the way they prefer.

One month later, the rain was gone, replaced by a humid, heavy afternoon. I was reviewing charts when I saw Palani standing in the doorway. He was wearing the same dark blue lungi, his stubble thick and grey, his face set in that familiar, granite glare. He didn't come in; he just stood at the threshold, waiting to see if I would launch into another round of pleasantries and questions. All the cold

glares and monosyllable replies were waiting in his head, ready to be hurled at me if I even began a pleasant exchange. I didn't smile. I didn't soften my posture. I didn't ask him how he was feeling. I pulled out a fresh case sheet, kept my eyes fixed on the paper, and gestured curtly to the chair. "Sit."

He hesitated, taken aback by the shift, then walked in and sat down—not on the very edge this time, but firm in the chair. His wife didn't accompany him; he had come alone. He looked far more comfortable today compared to the previous visit. Maybe it was the absence of his wife, maybe it was familiarity with the clinic, or maybe it was just the absence of any pleasantries and kindness.

"Blood reports," I said, extending a hand without looking up. "Did you take the medicines?" I did not even look up at him. I kept my eyes on the case sheet and kept my voice deep and emotionless.

"Yes," he grunted, sliding a familiar looking envelope across the desk.

I flattened the lab report with the palm of my hand. Fasting sugar: 160. Post-meal: 240. Better, but far from safe. I picked up my pen and tapped the paper sharply.

"Sugar levels are down, but they're still bad," I said, keeping my tone flat, transactional, and dry. "This is not reason to be happy. The sugars must come down further." In any other patient, I would have expressed some excitement and would have appreciated them as an act of motivation. But not for Palani.

He blinked and gave a faint nod. The stern language, devoid of pleasantness and kindness was at last getting across to him and making him notice.

"Looks like you drink alcohol," I stated. It wasn't a question, nor was it a moral judgment. I stated it like how I would state that he has a mole. Just a factual, value-neutral observation.

His jaw tightened, defensive energy surging back into his frame. "A little."

"Alcohol and diabetes do not sit together very well. Alcohol can worsen diabetes control" I said, looking him dead in the eye with zero warmth, only raw clinical facts. "I don't care about your drinking, and I'm not here to give you advice on how to live your life. I am not going to ask you to stop drinking. But I am just telling you that drinking alcohol worsens diabetes. It is entirely up to you to drink or to stop. I just want to give you the facts."

The expression on his face changed. The lack of any judgment or advice hooked him. He had started thinking.

“Can I drink once a week? Just on Sundays?” he snapped, his voice rough.

“No doctor in their sane mind would prescribe any patient to drink alcohol. But if you are going to cut down from drinking daily and bring it to once a week, I would recommend that strongly.” I replied instantly, not offering an ounce of sympathy for how hard that might be. “Again, let me make it clear. I don’t care whether you drink or don’t drink. If you continue drinking and take these medicines, you are only wasting your money”

He stared at me, searching my face for pity or disgust. He found neither—just cold, hard reality. He gritted his teeth, stood up, and reached for the new prescription I had swiftly written out. “Take these twice a day. With food. Come back in a month,” I said, already pulling the next patient’s file toward me, effectively dismissing him.

As he walked out into the bright afternoon light, I leaned back in my chair and let out a long breath. I hadn’t offered him a single kind word, a smile, or a warm check-in. But by stripping away the softness he mistook for weakness, I had given him something he could actually use: a battle he knew how to fight, and a doctor who spoke his language.

Two months later, the dry heat of mid-summer had settled over the village. The clinic was bustling, but when he walked through the door, the shift in his presence was undeniable.

He looked different. The salt-and-pepper stubble was clean-shaven, his hair oiled and nicely combed back, his red shirt was buttoned neatly, and the dark blue lungi was crisply washed and starched. His posture was still rigid, his shoulders still broad and set like an iron gate, but the raw, volatile anger that used to radiate from him had settled into a calm and quiet determination.

He sat down in the chair fully, without balancing on the edge of fleeing and placed a neat folder on the desk.

“Reports,” he said. His voice was still low and gravelly, but there was a small tinge of softness and respect in it.

I pulled out the paper. Fasting sugar: 110. Post-meal: 150.

I looked at the numbers, then looked up at him. I kept my face neutral, maintaining our tacit agreement. Inside my mind I was jubilant. This was unimaginable success. I had somehow reached out to a tough guy, communicated to him and the results were showing. I wanted to celebrate. But at the same time, I did not want to lose the awkward relationship that we had built.

"The sugars are normal," I said, keeping my tone matter-of-fact. I fought hard to stop myself from getting up, patting him on his back and appreciating him generously for the successful sugar control.

He sat back slightly, crossing his arms over his chest. He didn't say anything, but I noticed a subtle shift in his eyes. He didn't look away this time. He was looking directly at me, his chest expanded just a fraction more, waiting. He looked like he was expecting me to say something, probably an appreciation? The silence that hung between us was not awkward like all the other times. It was a pregnant silence, loaded with expectation of an appreciation.

He had fought a silent, brutal war over the last two months. For a man who defined his entire worth and masculinity by his ability to eat a lot, binge on meat and alcohol and indulge in all kinds of pleasures of the senses, cutting back alcohol and changing his diet and activities was not just a tweak; it was a massive, agonizing effort of will. And for the first time, behind that tough, unyielding exterior, I caught a glimpse of something very human: a man who had done the heavy lifting, achieved the impossible, and was waiting to be recognized for it. He didn't want soft, patronizing pity. But he wanted credit where credit was due.

I capped my pen with a slow, deliberate click and laid it flat on the desk. I let go of my matter of fact attitude and warmed up. "This isn't just the medicine," I said, leaning forward slightly and looking him in the eye. "Medicines alone cannot reduce the sugars like this on their own. It takes discipline. Real discipline. You did the hard work. It is a result of your work."

A slow, subtle change washed over his face. The tight line of his mouth relaxed. His jaw unclenched. The guarded, suspicious look in his eyes softened, replaced by a quiet, proud glint. He didn't smile. That would have been too much to expect. But he gave a single, firm, meaningful nod. It was obvious that the appreciation had gone and hit its mark and made him happy.

"It wasn't easy," he grunted softly, the admission slipping out almost before he could stop it. Also unsaid was the emotion, "I have never done this for anyone else. I did it for you."

"I know," I replied, keeping my voice calm, steady, and respectful, peer to peer, man to man. "Most people can't do it. You did."

He looked down at his hands, rough and calloused, resting on his lap. For the first time in all our meetings, the heavy armor he wore didn't feel like a defense mechanism to keep me out. It just felt like the natural frame of a man who had finally found a way to earn his own respect and could now accept a little bit of someone else's.

"Shall I come back next month?" he asked, looking back up.

I decided not to hold back anymore. From the man who was ready to run away from that room at the earliest opportunity to this man who was eager to come back, he had come a real long way. I smiled, a real, unreserved smile this time, no longer a threat to his pride.

"Next month," I agreed. "Keep doing what you're doing. It is helping you greatly!"

He stood up, adjusted his shirt, and gathered his folder. As he reached the door, he paused, turned around fully, and looked me straight in the eye.

"Thank you!" he said. The words were quiet, but they carried the weight of a long, hard-won peace. When I heard those two words my heart skipped a beat. I never expected that gratitude.

He walked out into the afternoon sun, his head held high, leaving the door wide open behind him.

Reassurance

"Preethi, get up it is getting late" Rama shouted as she quickly tossed the chopped onions into the pan and started frying them. It was a hectic Thursday morning. As she was tossing the onions in the pan, she turned off the gas as the cooker had let out 3 whistles. As the Pongal was getting ready in the cooker and the onions for the chutney were getting cooked, she also picked up her mobile phone that was lying on the kitchen counter top to check if any new messages were there in her inbox. Thank heavens there were no new messages. As a single parent, not only was it hard to balance work and home, but it was also difficult dealing with Preethi's 15 year old rebellion on a daily basis. "Preethi, this is the last call. If you don't wake up now, I am coming there to pour water on your head" This time Rama's voice was a shrill scream. Rama tossed the chopped tomatoes on the pan to cook them. After both were cooked, she had to grind them to a fine paste for the chutney. After tossing the tomatoes for a bit, she switched off the gas. The clock was showing 7 AM. She had to do something to now wake up Preethi. Otherwise, she would once again be late to school. Opening the towel that she had wrapped around her wet hair, Rama barged into Preethi's bedroom. Preethi was still sleeping under the covers.

"How many times should I call out for you? Are you waking up now or not? Should I drag you out of your bed?" Rama rushed to the foot end of the bed and pulled at her blanket. Preethi was curled up in a ball inside the cover. She went and touched Preethi to shake her. The moment her hand met Preethi's skin over her back, Rama withdrew her hand in shock. Preethi was burning up with fever.

"Oh! Sweetie! What happened? You are burning up…" Rama's voice suddenly went soft. She walked up to the head end of the bed and sat down next to Preethi. She took her daughter by her head and placed her head on her lap. "Baby! Get up now, sit up. Let me check what is happening?" Preethi got up groggily. She sat up and Rama placed a pillow behind her back and made her sit up. She touched Preethi's forehead with the back of her hand and the fever was very high. She caressed Preethi's forehead, cleared up the hair from her forehead and got up and went to the kitchen. She brought a moist cloth and wiped Preethi's face. She then gave her a cup of water and a bowl and asked Preethi to rinse her mouth with the water and spit it into the bowl.

She gave her a glass of drinking water, "Here, drink this water. I will bring you something to eat."

Rama rushed to the kitchen, heated up some milk and brought it to Preethi. "Drink this now. After this I will give you a paracetamol tablet."

She had to call Preethi's school and inform the teacher that she will need a sick day. She was not sure how to handle her work situation. She cannot take leave. Financial audit was going on in the company and there was no way her boss would grant her a leave.

"Don't worry! It must be a simple fever. I will make some tomato soup and toast some bread. I will leave it on the kitchen counter. After some time get up and have some soup and take rest. Call me from the home phone any time you need me. I must go to office today."

Preethi nodded for everything. She did not even have the strength to utter a word.

"Shall I asked neighbour aunty to check in on you during lunch time?"

"No ma…I can take care of myself. I will have the soup and rest. You go to work."

Preethi hated it when her mother got all worried and concerned like this. She was not a child anymore!

Rama reluctantly kept the soup bowl, bread and some crackers in a small box on the counter top and left for work. She locked the door from outside so that nobody would disturb Preethi.

Though Rama was at work, running around doing all the paper work for the financial audit, her mind was fully with Preethi. She dialled the home phone by 1 o clock to check on Preethi.

"Baby, how are you? Did you have the soup?"

"Yes ma, just now I had the soup" replied Preethi groggily.

"Has the fever come down?"

"It came down after having the tablet in the morning. But now I am getting the fever again. My back is also aching ma"

"Take one more tablet and go to sleep. I will come home soon and take care of you"

Though Rama reassured Preethi, she was least at peace. She wished that she was at home with her daughter. Preethi was only a child! As she cut the call she kept staring at the phone as though she would enter it and travel to her daughter if that was even possible.

Her mind wandered back six years. It was the fitful day when Vinod, Preethi's father had left them. 9 year old Preethi had fallen while playing and broken her forearm. Vinod was never an involved father. That evening their disagreements had peaked and reached bitter ends. "Vinod, if I must handle everything on my own, I don't see the point in you being here. I have asked you so many times, begged you, and single handedly managed this for 9 years. Through each sickness, each milestone, I have done everything on my own. All the missed birthdays, anniversaries, parent-teacher meetings, I don't know how to say this….you must leave." Vinod had walked out that day. The next year of Rama's life had been torn apart by the divorce proceedings.

The rest of the day passed in a daze. Rama was somewhere between the paperwork and her sick child lying all alone at home. She was broken up somewhere between the call of duty and the guilt of an absent mother.

That evening when she wound up work for the day and left, her heart was beating fast. She wished she could fly to be with her daughter. As soon as she reached home, she put her key in the lock and opened it. It was 6.15 PM. The house was dark and silent. She rushed straight to Preethi's room. The time between opening the door and seeing Preethi was filled with intense fear. Her mind conjured up all the worst possibilities. Preethi was lying down on her bed, deep asleep. She stood there and watched Preethi sleeping. She looked so exhausted with the fever. An intense heaviness rolled itself into Rama's throat. She could not swallow, nor breathe. She leaned against the cupboard and tears started rolling down her cheeks. As she stood there watching Preethi, sick and exhausted, an intense guilt overwhelmed her.

As though on cue, Preethi woke up.

"Ma…when did you come?" Preethi's voice was low and weak. Rama wiped her eyes and rushed to Preethi's side. She sat by her child and took her in an embrace.

"You still have the fever. Did you take the paracetamol this afternoon?"

"No ma…I did not. Soon after having the soup, I came and lay down. I am having a splitting headache…" saying this Preethi started retching.

Rama supported and took Preethi by hand and led her to the bathroom. She bunched up Preethi's hair and held her head as Preethi vomited into the sink. She gave a mug of water for Preethi to wash her mouth and face. Then she led her to the dining room and made her sit on the table.

"I will get you some kanji with some mild rasam. You will feel better after I give you the rasam."

Rama rushed into the kitchen and started making the rasam and the rice porridge.

"What am I going to do now? Should we go to the hospital? Should I call amma to come and take care of her while I go to work? Will amma come? I hope this is just a simple fever." All these thoughts were running in Rama's head.

As she served the rasam and rice porridge for Preethi to have, she said, "This is probably just a flu. You were swimming a lot last Saturday. I asked you to stop, but you would never listen to me. One more day of paracetamol and rest and you will be fine. Don't worry" Rama was reassuring herself more than Preethi.

As Rama led Preethi to the bedroom, she noticed that Preethi could barely walk. She made her sit up on the bed, gave her one more paracetamol tablet and gave her water.

"Take this tablet and go to sleep. Everything will be fine by tomorrow."

As Preethi lay down to sleep, Rama decided to sleep in the same room, just in case Preethi needed her in the middle of the night. She spread a mat on the floor next to the bed and lay down. She could barely sleep. Her mind was full of worry about her daughter.

The fever was relentless. Preethi started moaning in discomfort through the night. Rama had to give her one more Paracetamol around 1 AM in the morning. The fever was getting worse and Preethi was suffering from severe pain. As a result of the pain she was moaning all through the night. Around 6 AM, Rama got up and went to the kitchen. Her back and legs were aching from sleeping on the floor. The day felt like a continuation of the previous one without any break in between. She boiled milk and took it to Preethi to wake her up and give her some hot milk.

“I don’t think it is right for me to wait like this. The fever is not letting go. Maybe I should take her to the doctor. Let me take her to the doctor and then decide whether to call amma to come and help.” Rama called her mother anyways.

“Ma, can you come here? Preethi is down with a bad fever. Last night neither of us could sleep. Can you come here and help out for this week?”

“Rama, ohhh God! I wish I could come! How can I come now? You know construction work is going on in your brother’s house, and I am here helping. How can I leave all that and come now? It must just be a simple fever. Preethi is always jumping and playing. She must have over-exerted herself. She will be fine. Just give her some pepper and cumin rasam. It will help.”

Rama knew her mother had no voice in her brother’s house. After their father’s passing her brother felt entitled to his mother’s services and refused to let her have her own choices and decisions.

“Ok ma, I will take care. You do what you must do.”

As Rama cut the call, she sat down with her hand over her head.

Rama texted her boss. “Preethi unwell. Taking her to the doctor. Will be at work by noon”

Her boss was online and he replied immediately, “Ohh! Take care. Hope she feels better soon. You better be here before 12. If you are not there, the auditor will lose his mind. The last few days of audit, everyone is working the weekend… you know how it is…”

Rama woke up Preethi, cleaned her up with a wet towel. Preethi put on some clothes and they got an autorickshaw and went to the hospital.

“Doctor, she has been having a high fever since yesterday. The fever is just not letting go.”

The doctor looked up at Preethi, held her pulse, touched her forehead to check for fever, and looked away.

“Don’t worry, everything seems to be ok. This just looks like a simple fever. I will give you a prescription for fever medicines. Take them and she will be fine in 2-3 days”

Rama was not convinced.

"Doctor, I am already giving her the paracetamol tablets. I am worried this is not a simple fever. It is nothing like the fevers she had already had"

"I understand. Don't worry. We cannot do anything till 3 days of fever have passed. We should allow the fever to take at least 3 days before we can do any tests."

Rama was irritated, "What is with the 3 day rule doctor? Cant we do the tests today? I am a working mother and I cannot take time off to be with her as much as I would love to. I also do not have any other support systems at home"

"I can obviously not resolve your family issues and social problems for you. My job is to tell you that this fever looks like a viral fever and will go away in 5 days. Just fever medicines and plenty of fluid intake should do."

Rama walked out reluctantly with a prescription for the paracetamol. The clock was ticking 10.30 AM. She had to go home, get Preethi settled down and then get to work. She wished she knew what was the problem and how to solve the problem. After getting home, tucking Preethi in, keeping the rice porridge and rasam on the counter top Rama got out of the home and locked it from the outside. She rang the door bell of her neighbour, "Kala, would it be possible to please check on Preethi around afternoon today? She is sick with fever. I am unable to take leave at work. I will come back as soon as I can. I have kept food for her on the kitchen."

"Don't worry Rama, I will check on her"

When Rama walked out to get the cab to go to work she kept looking back at the balcony of her second floor apartment. She wished she could see her child sleeping through the balcony! She wished she could just be at home and at work at the same time.

The evening at 5.45 PM when she entered the home, it was silent and dark as yesterday. Preethi was balled up in the bed. She had shrunk significantly compared to morning. With a lot of trepidation Rama touched her only to find out she was still running a high fever. "Ohh God. The body will get damaged if she continues to have so much fever." As soon as Preethi felt her mother's touch, she stretched, but when she stretched her calf had a severe cramp and Preethi screamed in pain. "Amma….Amma….it is hurting." Rama rushed into the kitchen, opened the refrigerator and pulled out an icepack. She rushed to Preethi and placed the icepack on her calf. The cramp slowly let go. Preethi

stopped shouting and settled down. The fever was taking a toll on Preethi. Now Rama was serious worried, "Baby, don't worry, I will give you some nice soup. We will have the tablet and try to get some sleep tonight. Tomorrow will be a better day." Rama was saying all this, but her head was messed up with worry. She did not believe that everything was going to be ok. That night was equally bad. Preethi was tossing in bed and moaning in pain. She got up and cried, "Ma…the back pain is severe. I am unable to lie down. Neither can I sit up. Do something…"

"Ohh….baby…please don't cry. If I can I will take all your pain. I will get you a hot water bag."

Rama rushed into the kitchen brought a hot water bag and massaged all over Preethi's back. Preethi calmed down a bit and went to sleep. Time was already 5 AM.

Rama was exhausted. Two consecutive nights without sleep. More than the physical exhaustion the emotional exhaustion was horrible. She gave Preethi a glass of warm water, one more paracetamol and went into the bathroom to take a bath. She opened the shower and stood below it. As the water rushed all over her body, an inner dam broke and she couldn't control herself. Wave after wave of sobs washed all over her and she stood there crying under the shower. After 3 minutes of relentless sobs, her mind calmed down. "I am taking her to another hospital today. If needed I am getting her admitted. I cannot watch her suffer like this anymore" With a firm determination Rama walked up, dried herself in her towel and came out to prepare for the day. She took her phone out and texted her boss, "Cant make it today. Preethi is very sick. We are going to the hospital. I will ask Sandeep to sit with the auditor. Major part of the accounts are taken care of" Her boss just posted a single word reply, "OK"

Rama and Preethi walked into the hospital reception counter. Immediately they were welcome by the front desk person who made them fill out a form. "Please deposit 1000 rupees as registration and consultation fees" said the person and guided Rama to the cash counter. Within 5 minutes Rama and Preethi were ushered into a posh airconditioned consultation room. The doctor examined Preethi and said, "We have to do blood tests, X Ray of the chest, urine tests and then we can start the treatment". She pulled out a form and filled it up and gave it to Rama. Rama was once again directed to the cash counter. The bill was 8000 rupees. That was a lot of money. For a single income household in the city, spending close to 10,000 rupees for a simple fever was not easy. Rama was conflicted. "How am I going to manage this fee? After rent, electricity, water,

food, essentials, there is barely 10,000 rupees left in the account each month. It is only 20th of the month and I must push another 10 days before the salary. How am I going to manage this?" The question was running in her head. On one hand the fever seemed to be serious. But was it really that serious as to spend 10,000 rupees? She withdrew the slip from the cash counter, and rushed out of there. She went to where Preethi was sitting. She sat next to her, touched her head gently, caressed it and supported her and led her out of the hospital. Just as they were walking out Rama's phone rang. She picked it up, "Hello!"

"Rama, boss told me that Preethi is not well. Is everything ok?" It was Sandeep. Sandeep was her junior colleague at work. Sandeep was a nice guy, a respectful perfect gentleman. Sandeep's voice was so calming. Rama loosened up, "We are in this big hospital. The doctor has ordered a truck load of tests. It is too expensive. I am not sure if I can afford it. I don't know what to do"

"Take an auto and come straight to MM Nagar. There is a small clinic there next to the bus stand. It is called the MRK Clinic. You will see a big yellow board with red colored letters. Come there. I will also reach there now"

As the auto stopped just outside the clinic Rama helped Preethi get out of the auto. Preethi clinged firmly to Rama's hand. The rigid adolescent rebel had morphed into a weak clingy little child. As Rama felt Preethi's pressure on her arm, her maternal instinct swelled and she embraced her warmly and firmly. They stepped onto the road and the sun was hitting harshly on their face. Sandeep was already there. "Come come…Preeths!! Boss…what happened! Was it all the swimming!" Sandeep cheered up Preethi with his jolly banter! Preethi rushed to him and fell in his arms. Sandeep and Preethi had become very close friends. Sandeep's presence gave Rama so much strength.

"What is this place Sandeep?"

"Just don't ask any questions. The doctor here is just too good. I know him personally. We will go and see him now. You wont regret this. Trust me"

All three of them walked into the tiny clinic. There was just one more person sitting in the waiting area. Preethi went ahead and took a seat. A nurse was loading an injection and checking it. Sandeep went to her, "Sister, we are here to see the doctor for this child. She is having a fever."

"Sure, go right in and see the doctor. I am going to give the injection for this gentleman. I will join you as soon as this is over"

As the three of them walked into the clinic Rama felt a sense of familiarity. But she had never been in this part of the city before. As soon as the opened the door and looked inside, the doctor was seated and he was reading something. He was a middle aged man with dark rimmed glasses. He had a chubby face that seemed to be a perennial smile. The moustache was out of place on his child-like face, but it gave him a definite look of seriousness. As they walked in he looked up from his book. The already smiling face broke into another huge beam of smile. He nodded welcoming the three of them inside. He locked his eyes on Preethi, recognizing her as his client and pointed her to sit on the patient's chair.

"Come, come. Looks like you are not well. Do you mind telling me what is wrong?" The voice did not suit the face. It was a deep and baritone voice. It felt like the voice was dubbed over.

Rama felt an immediate level of comfort. She started speaking in a tense register, "Doctor, my daughter has been sick for the past 2 days. I know this is not a simple fever. It is very severe doctor. We have not slept in two days. Please help us"

"Sure, let us take a look and find out what is wrong"

The doctor examined Preethi gently. He looked into her mouth and throat. Then he put his stethoscope on her chest and carefully listened to her chest. He examined her hands and legs. Then he made her lie down and thoroughly examined her abdomen, touching it and feeling if there are any swellings there. All the while he was whispering to her what he was doing. His voice was so soothing and calm that Preethi relaxed and gave in to his examination completely. It was so comforting for Rama to watch.

The doctor finished his examination and came to the seat and sat down.

"Looks like Preethi is having a severe type of fever. I just need one blood test to find out what it could be. Can you get it in the next door laboratory and bring it to me"

Rama was scared about the cost and did not want to go through all the experiences that she had in the big hospital. "Doctor, I am sorry to ask this question, but can I know how much it would cost me?"

"Of course. You should not be sorry to ask this important question. It will cost you about Rs. 50. The results will be given to you immediately"

Rama was so relieved to hear that. They walked out to the next door to get the test. Rama was a bit doubtful. She looked at Sandeep, "Sandeep, how can it be so different? The big hospital charged 8000 rupees and this doctor charged 50 rupees. Do you think this doctor is doing the right thing?"

Sandeep smiled at her calmly. "Don't let your hyperactive thoughts go all over the place. Wait. Let us see how things go"

They got the results immediately and walked back into the doctor's clinic.

"I think Preethi should lie down in the bed for a little bit. I will read the result and talk to you."

Rama watched tensely as the doctor was carefully going through the results. It was worse than a student who has written an examination and was waiting for the results.

"It looks like Preethi has dengue fever. The hemoglobin is low and the platelet count is also lower than normal."

"Ohh…dengue? Is it anything to be afraid doctor? Do we need to get admitted?"

"Don't panic Mrs. Rama. Don't worry. She has dengue fever. Dengue fever symptoms are severe like this. It causes a lot of pain. But it is nothing to be worried about. We can treat her and relieve her body pains and the fever. We will give her intravenous fluids and also make her have plenty of fluids at home. She will recover soon enough. Don't worry"

These words were so reassuring and calming for Rama. The doctor ordered intravenous fluids for Preethi. The staff nurse set up the drip. As Rama stood against one wall of the treatment room, she looked at Sandeep who was sitting next to Preethi on the cot. A calmness and overwhelming exhaustion swept all over her body. Tears of peace started flowing down her cheeks. Rama slid down on the wall and sat down on the floor. Her shoulders relaxed and she closed her eyes. Her thighs were shivering. It felt like she was sitting and relaxing for the first time in 2 days. The past two nights rushed past her mind. She felt that things were going to be better. She craved her bed and some peaceful 'me time'. "We know what is wrong, and we are going to be able to fix this." As her mind thought this, she was over-awed by how reassuring the words of the doctor had been. She got up and went and sat on Preethi's bed near her head. Preethi opened her eyes and lifted her head and placed it on her mother's lap. Both mother and daughter felt at home.

Isolated in Illness



Encounters with patients and their caregivers who experienced extreme loneliness due to their illness

The good patient

I don't remember when I started seeing Raji for her diabetes. It must have been many years ago. But I recall one point from when I remember her personally by her name.

"It is my son's wedding next week. I don't know what I am going to do with this now!"

Early that morning Raji had woken up with saliva drooling down the left side of her face. When she got up and went to the bathroom to brush her teeth, she saw that her mouth was crooked and twisted to the left. She was unable to shut her left eyelid and there was a numbness and tingling sensation on the left side of her face. In a panic, she called her son, Sanku who, as fate would have it, was getting married the following week. They rushed to the clinic as soon as they could. On examining Raji, it was clear that she was having a partial paralysis of her facial muscles on the left side. This condition is called Bell's palsy. In most cases we do not know the reason for this. Sometimes it is due to a viral infection. It is usually a very benign condition. With some medications and physiotherapy it completely resolves.

"I don't want to appear with a crooked face during my son's wedding. Is there anything you can do for this to become alright before that?"

That was a tough question. Bell's palsy usually takes time to recover despite treatment. There are cosmetic procedures like injection of botox, but none of these are without complications and they are not so readily available in our part of the world.

"You should not worry about it Raji. You can get good make up such that it won't be obvious. You are healthy and moving about and will see Sanku get married happily. You should focus on that."

I remember Raji being very unhappy with that response, but she understood. I remember that episode so clearly because that was the first time we were having a very intimate conversation on matters that were very personal and important to her. She came back 2 months after the wedding with a normal face. We both laughed about the ill-fated timing of the Bell's palsy episode, thanks to which she had a crooked face on all her son's wedding photos.

Raji was what a doctor would call 'a good patient'. While there is a lot of discourse about who is a 'good doctor', among doctor circles there are also talks about the 'good patient'. When I was a young doctor, I used to believe 'good patients' are those for whom the diagnosis and treatment was easy. Things would smoothly fall in place, and the clinical encounter would end without any distress for me. Those days, 'good patients' were also those who complied to all my instructions meticulously. 'Good patients' were also those who asked the least questions and took in everything that I had to say without debates and discussion. But my perspective of 'good patient' has changed over the years. I say Raji was a 'good patient' because she would be honest and transparent. If she did not take her diabetes medicines for a month, she would tell me that and explain why. She wouldn't be overly apologetic, not too guilty about it, at the same time she wouldn't be nonchalant. She would put the matters in right perspective and say them as a matter of fact. She listened to what I had to say, engaged with it and asked well-meaning questions. She would tell me, "I don't think this treatment is going to work for me doctor. It sounds difficult". She wouldn't nod affirmatively to me and go and do something else. Such patients are a delight to treat. We know at all points what we are dealing with. Our relationship strengthened over the years, and it came to a point when I started missing her if she did not come for her routine monthly visits. A good dose of Raji once a month became essential for me to recharge myself.

Last year, around the end of April, Sanku called me on my mobile on a weekday evening.

"Doctor, may I bring amma to see you this Sunday? Something is seriously wrong. She is not telling me what. But she is not eating well or sleeping well."

I could hear the anxiety in Sanku's voice. He now had a job in a factory in Chennai and was living there. He was visiting once a week to meet his mother. I had just seen Raji two weeks ago and she seemed alright.

That morning when Raji walked in, she looked her usual self. But Sanku walked in behind her, and the worry lines were written densely all over his face.

"What happened Raji? I just saw you two weeks ago. Are you ok?"

"I am perfectly fine doctor. Sanku is unnecessarily creating panic."

Sanku put a reassuring hand on Raji's shoulder and nudged her to speak up.

"Amma, stop pretending like everything is normal. Doctor, amma told my wife that she has a swelling in her breast. She is too shy to speak about it to you."

Raji was squirming. She was very uncomfortable with the conversation.

"Raji, don't worry, I will ask the nurse to come and examine you in private. She will come and tell me about the swelling. So don't be embarrassed"

The nurse took Raji to the examination room. Within a couple of minutes, the housekeeping staff came to my room and summoned me that the nurse is asking me to come inside the examination room. I immediately understood that something must be wrong. I got up and walked to the room. The examination room is just some 30-40 steps away from my room. But that one minute was filled with anxiety and anticipation of what I was going to discover in the examination room.

Raji was lying down and the staff nurse was standing by the bedside. I could see a very grim expression on the staff nurse's face. I went near and examined the lump on Raji's left breast after warning her that I am there and I am going to examine it. It was a stone hard swelling the size of a ping pong ball on the upper outer part of the left breast. It is never a good feeling when the hand touches a lump like that. The tactile sensation beneath my fingertips immediately triggered that familiar, heavy dread, the unspoken associations of pain, wasting, and death. It was unmistakably a malignant mass. A deathly silence hung over the entire room. The staff nurse and I exchanged uneasy looks. Raji giggled shyly out of embarrassment of exposing her left breast to me, covered herself immediately and sat up. The look of worry on our faces was not lost on her.

"Tell me doctor. What is wrong? Is it anything to be worried about?"

"Shall we go out and sit and talk with Sanku also in the room?"

"No, please tell me now. I want to know first. Then we can decide what we are going to tell Sanku"

I placed a comforting hand on Raji's shoulder and faced her eye to eye and told her, "Looks like this lump has been here for some time now. It looks suspicious. It may be cancer. We must do some tests and confirm it. After that we must see how far it has spread and do the treatment. Nowadays treatment for breast cancer is good. Many patients get cured completely."

The tear that was collecting in Raji's eyes spilled down her cheeks abruptly. But she did not break down.

"I don't want you to say anything to Sanku doctor. I will go home and find the right time to talk to him. Just reassure him and send us home now"

Raji immediately took charge of her diagnosis. She wanted to have full control over what would happen from then forward. I wrote down a referral letter to the cancer hospital and referred them there. She was diagnosed to have advanced stage cancer that had spread all over her body. They decided that she was not a candidate for surgery. She was referred to the cancer hospital in Chennai. There they started her on palliative radiation therapy. All this happened between May – August of last year. Sanku kept me posted over text messages and phone calls. I did not get to see Raji throughout this period. I kept thinking about Raji and missed meeting her and hearing her cheerful "hello doctor" every month. But I also did not want to cross professional boundaries and impose into her personal space, especially during such a difficult time in her life. It felt like Raji was so far away.

In September last year out of the blue, Raji came to the clinic. I did not recognize her immediately because she had lost all her hair and had lost so much weight. She had aged several years and looked very pale, but her eyes were sparkling! She smiled a big broad smile at me and came and sat in front of me.

"When I was undergoing radiotherapy, I thought I was going to die. I kept scolding you and shouting at you for making the diagnosis and getting me trapped in this treatment and suffering"

Raji would always speak her mind to me. She told me her most honest feelings. I appreciated her honesty this time too. I genuinely apologized to her.

"What is wrong with you doctor? Why are you apologizing? Do you think I did not know that you did what is right for me? I know you well enough that you would not intend harm. I am just pouring my heart out to you like I always do. I never questioned your integrity even once."

I smiled at her.

"In fact, I am grateful to you for compelling me to go to the cancer centre in Chennai. Two weeks ago Sanku's wife delivered my first grand-daughter. If you hadn't sent me there, I would have died long ago without holding the little one in my hands" Saying this Raji broke down and started shaking. Sanku was

standing behind her shedding his own silent tears. Sanku knew the battle with cancer was not over. The doctors in the cancer centre in Chennai had warned him that they had only bought a few months of time and Raji's cancer was rapidly consuming her.

After that visit Raji started coming almost every week. We would give her intravenous fluids with some vitamins infused into it. She felt that the intravenous fluids were making her feel better. "All I want to do is to watch the little kid play about" she would say. I could see her slowly fading out.

Around early January this year Sanku called me one evening to say that he has received a promotion in his job and has been asked to be transferred to Vishakapatnam. "It is a substantial increase in salary. The increase is salary is crucial for our family. But I don't want to leave amma alone here and go." He said this and broke down and cried on the phone. By then Raji had stopped coming to the clinic. She was not ambulatory anymore. Raji and her elderly mother were living in the village. Sanku, his wife and baby were in Chennai. Sanku was visiting Raji weekly. Now if he takes up the promotion, he would not be able to come that often.

That Sunday, Sanku came to meet me in the clinic. Every time I saw his text, his phone call or when I saw him in person my heart started racing anticipating the news of Raji's demise. But Sanku said Raji was ok. He said he has come to visit me for his own health problem. When I was examining his blood pressure, he held my hand bent down his head placed it on my open palms and started crying. He was unconsolable. I stood up, walked around the desk, and held his head against my shoulder, giving him space to weep. "I am leaving to Vishakapatnam tomorrow doctor. I must take up the promotion and job. Yesterday I prayed to God to take amma away. I am not able to see her like this. Single handedly she brought up my sister and me. She gave me an excellent education. She was strict with me and prevented me from getting into bad habits like alcohol, smoking etc. My life used to revolve around her. But yesterday I prayed that she must die. I don't know whether I prayed because she is suffering, or whether I prayed because I want to leave and go to Vishakapatnam"

Sanku's guilt was pouring out. His entire body was shaking.

"I do not have anyone with whom I can share all this. Nobody understands my situation like you."

Sanku broke down and cried for some time and then picked himself up and walked away.

Two weeks after that Raji's mother came to the clinic to get her treatment. I did not ask her how Raji was because I was not sure if it was an appropriate question to ask. At the end of the encounter Raji's mother told me Raji had passed away during her sleep 10 days ago. Her expression was empty.

"Raji suffered a lot within a short time. It is good that God took her."

I never saw or heard from Sanku after that. Raji's mother continues to come for her health check up regularly.

Even today when I write about Raji, her exuberant smile, laughter, honesty, clear articulation of what she wants from the treatment, transparent disclosure of anything she did or did not do, and intelligent engagement in discussions come in front of my eyes like a film reel. I can never forget her because she typifies a 'good patient' for me. Her 'goodness' does not come from some extraordinary feats or heroic traits. Her 'goodness' is not her perfect adherence to treatment, not strict discipline in life, not absolute courage and resilience in the face of a terminal illness. Her 'goodness' is in her absolute 'ordinariness'.

Toothless is tasteless

Irusamma is about 65 years old. She has one of the prettiest toothless smiles I have ever seen. She lost all her teeth over the past 5 years. She walks with a straight back, but very slowly because of the pain in her hips. All the cotton sarees that she wears when she comes to the clinic are weathered, softened by years of washing with detergent and are so soft to touch. They are all in soft pastel shades—not by design, but faded into tenderness by years of washing. I like giving Irusamma a warm embrace for two main reasons, firstly she is such a happy and cheerful woman and huggable, and secondly the softness of the fabric of her saree is so inviting! Irusamma lives alone in a small government-built house in the village. She told me once that she has a single light bulb that shines an orange glow at night and a table fan which makes more noise than it blows air. On most days she prefers to sleep outside in her coir cot that is laid just outside her home. Only on rainy days Irusamma sleeps indoors.

"Of the six I gave birth to, only one is alive now" she said, talking about her only surviving son, who now lives in the city and visits her with his family once a month. Irusamma lost her three daughters and two older sons at various times points during their childhood to various illnesses. Her husband passed away 10 years ago due to alcoholism and liver complications. Despite the depth of the tragedy in her life, Irusamma is one of the most cheerful women I have seen in my clinic.

"I am so bored and tired of drinking porridge every day. Is there no way out for me?" As she did not have teeth, she could not eat anything solid. She had completely given up her favourite fish curry and chicken kozhambu after she lost her teeth. On certain days she would overcook the vegetables and mash them up and eat them. But even that took a lot of effort and so she stopped having any vegetables or meat. Now she was purely subsisting on porridge made of rice flour, wheat flour or ragi powder. Tea and coffee are her staple food, and she also drinks plenty of water. But Irusamma was bored. She wanted to taste food, she wanted to feel all the textures in her mouth.

"When someone chews loudly in the neighbor's house, or chews something crispy, and if I hear those sounds, I feel extremely irritated. I walk up to them and give them a sound piece of my mind"

"The other day my son came to visit me. He brought a big bag full of vendakka (ladies finger)" Saying this Irusamma started laughing uncontrollably. In between

squeals of hearty laughter, she was piecing together words, "Potatoes I can understand, brinjal also I can grind into a paste and have it. I don't understand ladies finger. How is one without teeth supposed to eat ladies finger?" She lapsed back into a hearty laughter. "vendakka, vendakka, vendakka…" she kept repeating and laughing. Behind all this raucous and laughter, I could sense an undercurrent of longing to eat good food. Irusamma doesn't have diabetes, no hypertension, no heart disease. If only she had teeth, she could eat anything she wanted, no restrictions. But not having teeth was her biggest problem. There was no money to buy dentures or to undergo dental implantations. Five years ago, when the dentists had suggested that she get a full denture, her son had denied and said "Which beauty pageant are you going to? Why do you need teeth?" They never anticipated that they would come to a stage where not having teeth would grossly limit her ability to eat anything.

I have been talking to Irusamma and insisting that she must go see the dentist in the government dental hospital where they would fit in dentures free of cost. At least she can eat something which would help her get some nourishment. She was becoming pale and losing muscle mass rapidly. At this rate, she would soon lose bone strength and would be at a high risk of falling and breaking her bones. But she would always laugh it away and say the same thing her son had once asked, "Why does an old woman like me need new teeth?"

One day Irusamma came to the clinic with severe abdominal pain.

"I am rolling on the bed from last evening with severe pain in the abdomen" even as she said this there was a bout of colicky pain in her abdomen. She held her hand against her tummy and bent over in pain. I made her lie down and slowly and gently touched her abdomen. The abdomen was soft and caved in. In some points when I touched her abdomen, she winced with pain.

"Looks like your stomach is empty. Did you eat anything?"

"I drank some kanji late last evening. After that the pain started. I haven't had anything to eat since then"

"Have you vomited?"

"I am just having dry heaving, but nothing is coming out"

She looked severely dehydrated. Severe dehydration can cause abdominal pain. I started her on intravenous fluids and some medicines for the acidity in

her stomach, which was probably causing the pain. As soon as the fluids started flowing into her veins, Irusamma went into a deep sleep. I decided it was time the son got more involved. Irusamma needs better quality of life. Her son needs to see what is happening with her and how she is struggling without teeth. I took his phone number from our medical records and called him. I requested him to come down to the village that weekend and bring his mother to meet me in the clinic. I hoped he would come.

"Tell me doctor. I got scared that day when you called me. I was in my office. What happened? What did you want to talk to me about?" asked Irusamma's son. He and Irusamma had come over to the clinic that Sunday.

"I wanted to talk to you about your amma. She got very sick last week and got admitted here. You know she has not been eating properly, don't you?"

"Why doctor, what is wrong? I buy her rice, dal, vegetables whenever I come. I also send her 500-1000 regularly. I have not kept her wanting for anything."

"Yes, amma has told me how you take care of her and buy her vegetables and grocery. But she has not been eating properly."

Irusamma's son got irritated and raised his voice against his mother, "Why are you insulting me like this in front of others? Am I not providing for you? Why are you not eating well? Tell the doctor that it is your mistake. Tell him I am providing enough food and money to you, and it is you who is choosing not to eat."

Irusamma withered under his words, bowing her head in quiet submission. It was clear she had anticipated this anger, and it was precisely why she had kept her suffering to herself. She briefly looked up and met my eyes.

"Amma, is there anything you would like to say?" I asked gently looking at her. Irusamma did not say anything.

"Please do not shout at her. She never told me that you are not providing for her. She always talks very positively about you. But do you know why she is not eating properly?"

"How will I know unless she tells me?"

"You remember your mother lost all her teeth five years ago?"

"Yes, I know. I only took her to the dental hospital and got all the treatment done."

"OK! Since the time she lost her teeth, she has not been able to chew any food. Chewing food is essential for health. If she doesn't chew, it automatically restricts the choices of what she can eat."

There was a sarcastic look on her son's face. It looked like he was not a big believer in choices. What choices do people have anyways? He seemed to be asking.

"As she does not chew food, the food barely remains in the mouth for a fraction of a second. It is swallowed immediately. This leaves very little time of contact of the food in the mouth. So she has not been able to taste anything properly."

"I had never thought about that…." Her son's voice faded with doubt.

"The food she is eating is also not containing enough nutrients. So her taste cells in her tongue are also not working well. In summary, she lost one of the most important pleasures we all have in life, the pleasure of feeling the texture, warmth, taste and flavors of tasty food in our mouth."

Irusamma had her head bent down. She did not look up or say anything. Irusamma's son abruptly sat down next to her and placed his hand on hers.

"When we chew food properly, some amount of the food is digested in the mouth and it becomes easy to digest the rest in the stomach. But for people who do not have teeth, undigested food enters the stomach. Often this can be a lot of load on the stomach."

The look on her son's face got more and more grim.

"The overworked stomach can get sick at times. This can cause stomach aches. Some important nutrients never get digested or absorbed by the body if the food is not chewed properly. That is the reason she became so sick last week."

"Oh God! Nobody told me all this. What should I do now doctor?"

"It is not a good thing for people to be without teeth. You must take her to the government dental hospital and get her proper dentures."

"If she gets dentures will she be able to chew food properly?"

"Yes, a well fitted denture can help her. Not only will she be able to eat properly for her nutrition sake, but she will also get back an important pleasurable activity in her life, eating tasty food."

I could see that Irusamma's son had already started planning the visit to the government dental hospital.

Irusamma came back two months later with a full set of dentures. She smiled at me, her usual happy smile, just a lot more toothy and shiny!

"He still buys me vendakka, doctor!" she laughed, showing off her new teeth. "You taught him how to see life without teeth, but you haven't taught him that I still don't like ladies' finger!"

I hugged Irusamma, something I like doing, and told her, "Give him time Irusamma, one thing at a time!"

Irusamma's son was not a cruel man; he suffered from a quiet, pervasive lack of perspective. He believed that fulfilling his duty meant supplying sacks of rice and vegetables. He lacked the imaginative empathy to realize that without teeth, food ceases to be nourishment and becomes an ordeal.

Society often expects the elderly to quietly withdraw into spiritual detachment and bodily endurance—as if the desire for flavor, texture, and joy naturally expires with age. But cheerful souls like Irusamma still have a whole innings left to play. When we lose our senses or our ability to chew, our world shrinks.

Clinical medicine often focuses on organ systems and disease metrics, but sometimes the most transformative medical intervention isn't a complex drug or a surgical procedure. Sometimes, it is simply acting as a translator between a mother and her son—helping them see that joy in old age can be as simple as restoring the simple, sensory pleasure of eating.

The Dawn of Hope

The sky was heavily covered in dense grey clouds, hanging low and suffocating over the village of Singampatti. A heavy, eerie stillness settled over the world—the deceptive quiet that always precedes a devastating storm.

Kanimozhi stood by the barred iron window of the dark kitchen, staring out at the motionless leaves of the drumstick tree in the garden. For two whole days, the weather forecast on the television had blared warnings of torrential rain, yet not a single drop had kissed the parched earth. Nature withheld its relief, but inside the house, Kani's eyes wept relentlessly. The silent, continuous tears streaming down her hollow cheeks seemed to mock the arrogant, barren clouds outside, challenging them to a duel.

"Hey! What are you doing sulking in the dark? Crying your crocodile tears again?" a sharp, grating voice snapped from across the hallway.

It was her mother-in-law, Meenakshi. She stood with hands on her hips, her eyes flashing with disdain. "Come here this instant! The lentils need grinding, the vessels are piled high as a mountain asking to be washed, and the floor isn't swept. You think you're a queen in a palace? Move your lazy bones!"

"Coming, Attai," Kani whispered, quickly wiping her cheeks with the faded edge of her cotton saree.

It had been barely a year since September 24, 2018—the day she was married, a date carved into her memory with the cruel sharpness of a knife.

As Kani lifted the plastic pot to fetch water from the courtyard pump, memories of her childhood home rushed back, spreading over her bruised heart like a soothing balm.

When she was a little girl, her mother, Parvathi, would make fresh, piping-hot *athirasam* for festive days. The heavenly aroma of jaggery, cardamom, and fried gingelly oil would fill their modest house. Little Kani would sneak into the kitchen, hastily stuffing two or three sweets into her mouth all at once, terrified that her two elder brothers would swoop in and steal them.

Standing there with her cheeks stuffed round, sweet oil glistening on her chin, her face glowing with pure joy, she would look up at her mother. Parvathi would

shake her head with an affectionate smile, wiping Kani's face with the pallu of her saree.

"My precious Rasathi," her mother would coo softly, pinching her cheek. "You shouldn't stuff yourself like this, darling. Too much sweetness will cloy your tongue. Your stomach will ache terribly. Moderation is life's greatest secret".

Kani never listened, of course. Later, when the inevitable stomach cramps struck, she would sit by the hearth, sipping bitter herbal *kashayam* and replaying her mother's gentle words in her mind.

"There must be a limit to everything in life, my child," Parvathi had often told her. *"A limit to happiness, and a limit to sorrow. Excess of either will break you"*.

Kani thought of her wedding day. *How boundless my joy was back then,* she thought bitterly. *I let myself believe I had found paradise. And here is the price for having too much happiness—standing alone in a stranger's house, swallowing insults and tears*.

Kanimozhi's life had fractured long before her marriage.

She was seventeen, studying in the twelfth standard, when the world first slipped from under her feet. During a blazing morning school assembly, as the national anthem echoed across the courtyard, Kani's vision blurred. A cold sweat drenched her uniform, her knees buckled, and she collapsed onto the hard gravel ground.

The teachers rushed her to the government hospital, where her panic-stricken father, Rathnam, met them. The diagnosis was swift and unforgiving: severe juvenile diabetes.

"Her pancreas has failed to produce insulin, Rathnam-ayya," the doctor said, holding up a glass vial. "She will need daily insulin injections for the rest of her life. Without them, she cannot survive".

That night, an unyielding silence fell over their household. But instead of despair, an extraordinary mantle of love enveloped Kani. From that very evening, Rathnam, Parvathi, and her two elder brothers transformed their lives.

“From today, no sweets enter this house,” her eldest brother, Murthy, declared firmly, pushing away his bowl of payasam. “If Kani cannot have sugar, none of us will”.

When the crucial +2 board exams arrived, Kani’s condition proved humiliatingly difficult. Due to fluctuating blood sugar, she could barely sit through an hour of the three-hour exam without experiencing an uncontrollable urge to use the restroom. Sitting near the open window of the examination hall, her eyes would well up with distress as she looked out into the schoolyard.

There stood Murthy, leaning against the banyan tree, his eyes never leaving her. The moment he saw her agonized signal, he would run to his friend Kala, who was waiting nearby. Kala would rush into the hall, obtain permission from the invigilator, and gently escort Kani to the restroom. Her brothers shielded her from shame, giving her the courage to finish her exams.

Kani had no desire to pursue college. Her parents and brothers never pressured her; they simply surrounded her with warmth. She stayed home, carefully administering her insulin, helping her mother with tailoring work.

“Kani, come here, ma,” Parvathi would call out, sitting before the pedal-driven sewing machine. “Cut this blouse piece along the chalk mark. Thread this fine needle for me, my eyes are growing dim”.

Kani would sit beside her, her nimble fingers working the fabric.

“Learn every cut and seam properly, Rasathi,” her mother would say softly, guiding her hands. “A skill learned with your own two hands is the only true ally a woman has. You never know when life might demand it of you”.

When Kanimozhi turned twenty-two, Rathnam began actively searching for a suitable groom.

The first family arrived from Singampatti. They drank the tea, eyed Kani up and down, and murmured, “The girl is too thin, like a dried stalk. How will she manage household work?” and left.

Then came five more prospective alliances. Each party found a flaw: her height, her complexion, her horoscope.

One night, sitting on the veranda after Kani and the boys had slept, Parvathi's voice broke as she whispered to her husband, "What will become of our Kani? Not a single proposal is coming through".

Rathnam wiped his weary eyes. "We haven't even mentioned her diabetes to any of them yet. Just seeing her frail frame makes these suitors turn up their noses. If we reveal she depends on daily injections, not a soul will ever cross our threshold".

Parvathi hesitated, looking down. "There is a boy in our distant relation... His first wife passed away—she hung herself, they say, after some domestic quarrel. They are looking for a second match for him. Should we ask for Kani?"

Rathnam flared up, his chest heaving. "A second marriage? For our innocent Kani? If Murthy and his brother hear you suggest this, they will tear the house down! They hold their sister higher than their own lives!"

Yet, as months dragged into years, the cruel reality of societal prejudice broke Rathnam's pride. After endless hushed discussions and sleepless nights, the alliance was finalized with that very man—Sundaram.

Kani felt a surge of innocent joy. Sundaram was polite, educated, and well-settled. Five years of uncertainty, whispered taunts from neighbors, and anxiety had finally melted away.

The doctor said as long as I take my insulin and eat properly, I will live a healthy, normal life, she thought happily. *Father and Mother only want my happiness. Everything will be fine now*.

The wedding was celebrated with traditional rituals, laughter, and blessings.

The illusion shattered within a week.

One morning, Sundaram walked into the bedroom unannounced and caught Kani lifting her saree slightly to inject insulin into her thigh. The look of absolute horror on his face sent a chill through her spine. Without listening to a single word, he stormed out of the room and summoned his mother and elder sister.

Meenakshi burst into the room, her eyes wide with rage, spitting venom. "You deceitful wretches! Did your entire family conspire to trap my son? A chronic disease! Incurable rot in your blood! You won't be able to bear him children. You won't even be fit to share his bed! You are a walking corpse! We must

discard you immediately. My son is a prince—I will find him a pristine bride within a month!"

Kani stood frozen, the harsh words piercing her like shards of broken glass. She had grown up surrounded by adoration; nobody had ever spoken to her with such cruelty.

Trembling, she looked at the door, hoping Sundaram would intervene. "Please, Attai... do not say such things. Let him speak. If he tells me to go, I will leave".

Her faith proved entirely misplaced.

The next morning, the entire family convened a mini court in the living room. Sundaram's relatives sat around the hall, while Kanimozhi was made to stand in the center, isolated, an accused criminal facing a ruthless trial.

Her mind flashed back to the school assembly—when she had fainted, her father, mother, and brothers had rushed forward, lifting her in their arms, surrounding her with protective love. She felt a suffocating lump in her throat, wanting to collapse and weep, but there was not a single hand to catch her. It felt as though an entire mountain had collapsed upon the fragile wings of a sparrow.

Sundaram looked at her, his expression cold and devoid of affection.

"Tell me, Kani, am I wrong to want a family of my own?" he said, crossing his arms. "Why did your father hide this illness from us? My mother is right. How can I build a life on deceit? Sign the divorce papers and leave. I don't want this fake marriage".

The decree that she was fundamentally defective and unfit for motherhood struck Kani's chest like a heavy iron crowbar. The educated husband she had trusted was no different from the rest.

She did not cry out. She did not beg. With quiet, agonizing dignity, Kani turned around and walked out the front door, leaving the house behind to wander into the unknown world, searching for a life that had meaning.

In the heart of Chennai, the scorching midday sun tore through the polluted sky with merciless ferocity. The heat hung thick, suffocating the crowded streets of Triplicane.

Inside a dismal ten-by-fifteen-foot room in a dilapidated bachelor mansion, the lime plaster was peeling from the damp walls like dead skin. Crude graffiti, lewd sketches of male and female anatomy, and vulgar scribbles covered the walls. The air was thick with the foul, stagnant stench of stale sweat, and piles of unwashed clothes.

In the darkest corner of this room, stretched out on a frayed reed mat, lay Alagesan.

He was emaciated beyond recognition. Bare-chested and wearing only a faded lungi, his ribcage protruded sharply beneath paper-thin skin. Every muscle had withered away, leaving only skin tightly stretched over bone. Rivers of sweat trickled down his hollow temples.

His mobile phone began to ring on the floor, vibrating against the dusty tiles. With immense, trembling effort, he reached out his stick-like arm and pressed it to his ear.

"Alagu! Thambi!" his elder sister, Valli's frantic voice cried over the speaker. "Where are you? Why didn't you come home this weekend? What happened to you? Tell me! I haven't been able to swallow a morsel of food out of worry for you!"

Alagesan exhaled a ragged breath, his voice dry and raspy. "Stop it, Akka. You always make a scene over nothing. I am fine here in Chennai. I'm just feeling a little tired, that's all. Don't start howling like a madwoman".

"How can I not cry?" Valli wept openly. "Ever since Amma and Appa left us, you only have me, and I only have you in this world! Don't I have the right to worry? Tell me the truth—are you taking your insulin injections properly?"

Bitterness flared in Alagesan's chest. "Since you got married and had your own children, you only look after them! When do you ever have time for me? Stop preaching! Where will I get money for insulin? I lost my job two months ago. I can't even afford a cup of tea, and you expect me to buy insulin vials?"

Furious, he dragged himself up and sat against the peeling wall. He caught a glimpse of his reflection in the cracked mirror hanging opposite him.

He recoiled. The man staring back looked like a living skeleton out of a nightmare. His face was sunken, his eyes buried deep within dark sockets, his skin wrinkled and sallow. Though only thirty years old, he looked like a decrepit man of sixty.

He lifted his arm slightly and caught the sour odor of his own unwashed flesh—a smell of neglect and decay that disgusted even himself.

Valli was still speaking on the other end, her voice muffled and distant, but Alagesan could no longer hear her words. Self-hatred and despair welled up inside him, and silent tears traced paths through the grime on his hollow cheeks.

Fifteen years ago, when Alagesan was a lively schoolboy, he had been diagnosed with Type-1 diabetes. For the first five years, his parents managed his condition meticulously, ensuring his insulin and diet were never compromised.

Then tragedy struck. A devastating fire broke out during the annual temple festival in their village, claiming both his parents in an instant.

Orphaned and adrift, Alagesan gave up his studies and took up menial labor, while his maternal uncle stepped forward to marry his elder sister, Valli.

Alagesan moved into his sister's house, working hard and dreaming of saving enough money to start a small tea stall of his own. But his failing health made consistency impossible. One evening, bedridden with a severe, racking cough, he overheard his uncle arguing bitterly with Valli in the kitchen.

"Look at your precious brother!" his uncle had hissed viciously. "He doesn't go to work regularly, and when he does, he hides his wages from us! Talking about saving money for his own business! What an outrage! It's hard enough feeding you and the children, and now I have to provide for this useless freeloader?"

The humiliation felt like acid poured into an open wound. That very night, ignoring his burning fever, Alagesan packed his few belongings and slipped away to a friend's house. He sent a brief text message to Valli and boarded a midnight bus to Chennai.

In the unforgiving city, he took on odd jobs—cleaning tables, carrying loads, running errands. The dream of starting a business was abandoned; the daily insulin injections were soon discarded. He did not know which hospital to go to, which doctor to see, or where he would get free insulin. His only option was the private clinic where treatment was expensive. When a man could barely afford one meal a day, how could he purchase costly insulin vials and needles? Isolation, chronic illness, and poverty slowly ground his body to dust.

Eventually, he found a modest job as an office assistant. One evening, the company threw a lavish celebration party. Alagesan had always avoided social

gatherings—he could neither eat the rich food nor afford to let anyone see his trembling hands. But his colleagues insisted, dragging him along.

At the party, his loose, faded shirt hung on his bony shoulders like a shroud. The office manager and a group of employees, holding their drinks, surrounded him, laughing loudly.

"Look at our Alagu—named 'Beauty', but there's nothing on him except bare bones!" the manager jeered, pointing a finger at his chest. "Hey, Alagu, if you ever marry a girl, how will you manage? It looks like she'll have to do all the heavy lifting in the house... maybe she'll even have to find someone else to make babies for her!"

Cruel laughter erupted around him. Alagesan felt the blood rush to his face, his ears ringing with intense humiliation and fury. He couldn't speak. Nobody knew about his diabetes. Maybe if they had known they would have shown some kindness. Maybe some sympathy. He shrunk into himself, shivering with shame, before quietly slipping out of the banquet hall. Not a single person noticed his departure.

Back in the village, unable to bear the gnawing anxiety in her chest, Valli took the first bus to Chennai and tracked down the filthy mansion in Triplicane.

When she pushed open the door and saw her only brother lying like a bundle of sticks on the dirty floor, she broke down.

Alagesan looked up, his eyes widening in sorrow. "Akka... why did you come to this filthy gutter?" he choked out, burying his face in her lap and sobbing like a little child.

"Hush, Alagu, hush," Valli cried, stroking his matted hair, her tears soaking his back. "To hell with all pride and grudges! You are coming home with me right now. If your uncle utters a single word, I will leave him! You are my life, Alagu. Did I survive our parents' death just to watch you turn into a corpse?"

She helped him up and brought him straight to the government general hospital in their district town.

After a battery of tests, the chief physician emerged from the intensive ward, his expression grim.

“He has neglected his insulin for months,” the doctor explained, reviewing the medical charts. “His blood glucose levels are dangerously elevated. He has developed active tuberculosis in his lungs, and severe fungal infections have spread across his mouth, throat, and intestines. His condition is extremely critical”.

Valli collapsed against the hospital wall, weeping inconsolably. “Doctor, please! Do whatever it takes to save my brother! He has no mother and no father. I should have protected him like a mother, but I prioritized my own family and neglected him. Please don’t let him die!”

The doctor offered a few words of comfort and hurried away to tend to his rounds.

Valli staggered into the crowded hospital waiting corridor and sank onto the only vacant metal chair. Her body shook with heavy, wracking sobs.

Beside her sat a young woman wearing a simple, neatly ironed cotton saree. Seeing Valli’s profound distress, the young woman leaned closer and placed a gentle, comforting hand upon Valli’s trembling shoulder. With quiet grace, she unscrewed a clean steel water bottle and offered it to her.

“Drink some water, Akka,” the young woman said softly.

Valli took the bottle, drank a few sips, and wiped her eyes. “Thank you, ma,” she whispered, her voice hoarse. “Why are you here at the hospital? What is your name?”

“My name is Kanimozhi,” the woman replied with a faint, gentle smile. “I have diabetes. I come here regularly to collect my insulin and get my checkups done”.

Valli let out a heavy sigh, her eyes filling with fresh tears. “My younger brother has diabetes too. He neglected it for so long, and now the doctor says he is in critical danger.”

Kanimozhi looked at the older woman with deep empathy. Having walked through the dark valley of chronic illness, loneliness, and societal cruelty herself, she recognized the pain in Valli’s soul.

“Don’t lose hope, Akka,” Kani said reassuringly, holding Valli’s hand firmly.

Kanimozhi supported Valli gently by the arm, guiding her out of the sterile hospital corridor into the open courtyard.

A warm, gentle breeze rustled through the green leaves of the ancient tree. They sat together at a stone bench, sharing their stories—of pain, abandonment, survival, and quiet resilience. In that open space, two wounded souls found an unspoken kinship.

Far in the horizon, the blazing sun of a harsh day began to set, painting the twilight sky in soft, golden hues. As the day peacefully surrendered to the dusk, a profound new chapter of companionship, solidarity, and courage was born.

In the very hour the sun was setting, a brilliant dawn of hope was quietly rising in their lives.

Absent sons and empty pill bottles

The morning heat in the village was already settling thick and heavy, carrying the scent of dry red earth and cow dung smoke through the open windows of the clinic. Overhead, the old three-blade ceiling fan groaned with every rotation, doing little more than shifting the warm, dusty air from one corner of the room to another. Outside, a line of patients sat patiently on long wooden benches under the shade of a tamarind tree, waiting with their faded clinical notes and prescription slips.

When the wooden door creaked open and the woman stepped across the threshold, her presence commanded immediate clinical attention.

Even from across the room, the physical signs were strikingly unmistakable. At the base of her throat sat a prominent, symmetrical, butterfly-shaped mass that lifted and lowered with every swallow. The skin over her neck was thin and stretched, and through it, the neck veins pulsed violently, throbbing like tiny trapped engines against the surface. Her eyes carried a wide, startled stare, the upper lids retracted just enough to expose a rim of white eye above the iris. Any third-year medical student completing a basic rotation in the surgery department would have stood up and diagnosed her within five seconds: severe thyrotoxicosis, an overactive thyroid gland pouring dangerous excess hormone into the bloodstream.

She looked to be around fifty years of age, though the wear of hard manual labor made her look older. She walked with rapid, unsteady steps, her hands trembling faintly at her sides, and sank down onto the modest wooden chair placed across the consultation table.

“Vanakkam, Doctor,” she spoke. Her voice was thin, breathless, and raspy, trembling with an inner agitation she could not suppress. “For two months now, my body has not been under my control. If I take even ten steps toward the village water pump, my chest heaves and my breath catches in my throat. My heart gallops inside my ribs like a horse. Look at my hands—they shake so much I spill rice when I try to eat. I cant hold a glass of water steadily. My legs feel like dried sticks, drained of all strength. I couldn’t endure it any longer, so I walked here to see you”.

“Don’t be afraid, Amma,” I replied. “Take a deep breath and sit comfortably. Drink a little water. We will examine everything carefully and figure out exactly what is making you feel this way”.

Her name was Semparuthi—the Tamil name for the crimson hibiscus that graces village courtyards.

The name instantly brought a wave of quiet nostalgia. While hibiscus flowers bloom in various shades, pale pink, yellow, and white, it is the classic deep scarlet flower with five wide, velvety petals and a golden pollen column that commands absolute reverence. In the modest home of my own childhood, a lush hibiscus bush had stood tall right beside the front veranda. Throughout the year, regardless of the season, it yielded magnificent red blossoms. Every morning at sunrise, my mother would carefully pluck the freshest flowers to lay at the feet of the household deities. Whatever remained on the branches, my sisters and I would eagerly harvest to decorate our miniature clay idols during our playful temple games in the courtyard.

Looking at the weary woman sitting in the chair, her name felt like an echo from that distant past. But the clinical examination required immediate focus.

A systematic physical examination confirmed what the initial gaze had suggested. Placing two fingers upon her radial artery, her resting pulse raced at over one hundred and twenty beats per minute, bounding and irregular. The mercury column of the sphygmomanometer shot up, showing markedly elevated blood pressure. When she extended her arms with fingers spread wide, a piece of paper placed over her knuckles fluttered rapidly from the fine tremors racking her muscles. Placing a stethoscope over her enlarged thyroid gland revealed a distinct, rushing murmur, the sound of blood surging through a hyperactive organ.

I took out the prescription pad and ordered blood tests for thyroid hormones.

Semparuthi looked at the paper with deep apprehension, her trembling fingers clutching the edge of her faded saree. “Doctor... is it something terrible? Have I contracted a dangerous, incurable disease? Why are you writing for blood tests?”

“Nothing like that, Amma, do not worry, there is a small butterfly-shaped gland in your neck called the thyroid, which controls your body’s energy and speed. Right now, it is working too fast and releasing too much hormone into your blood. The blood test will tell us the exact numbers so we can give you the right medicine to calm it down. With proper pills, you will get completely better. Go to the district hospital lab, get the blood drawn, and bring the results back to me next week”.

She pressed her palms together in a grateful *vanakkam* and walked out into the heat.

That night, sleep proved elusive, overshadowed by thoughts of Semparuthi's difficult reality.

During the consultation, she had shared the painful contours of her solitary life. Her husband had succumbed to a sudden illness five years ago, leaving her alone in the world. She had raised two sons with everything she had, but as soon as they grew into men, both migrated to distant industrial towns to seek their fortunes, eventually marrying and starting their own households. Neither son sent money back to the village; neither returned to check on the aging woman who had given them life.

Semparuthi survived day-to-day on the meagre, irregular wages she earned from hard physical labor under the government's Mahatma Gandhi National Rural Employment Guarantee Act (the hundred-day work scheme), digging canals and clearing brush under the blistering sun.

When the physical toll of her failing health finally forced her to apply for a small government old-age pension, the local village panchayat president had sneered at her across his desk: *"You have two grown, earning sons working in the city! Why should the government waste taxpayers' money on giving you a pension? Ask your boys to feed you!"*

Too proud, heartbroken, and ashamed to publicly admit before the entire village that her own flesh and blood had discarded her, Semparuthi had swallowed the insult in silence, turned around, and walked away empty-handed.

Loneliness is a bitter pill to swallow in one's youth, but in old age, combined with grinding poverty, social abandonment, and a terrifying physical illness, it becomes an almost insurmountable agony.

When sleep finally came, it arrived wrapped in a restless, symbolic nightmare. In the dream, Semparuthi sat in the clinic chair, a brilliant red hibiscus tucked into her greying hair, her wide eyes staring in silent terror. Suddenly, the scene shifted to the old ancestral courtyard where the childhood hibiscus bush stood in full, radiant bloom. From the shadows, a dark, faceless figure rushed toward the bush, ruthlessly tearing the red blossoms from their stems, trampling the petals into the dirt. A desperate attempt to run and protect the flowers was met with heavy, leaden legs that would not move. The heart pounded violently against

the ribs, cold sweat pouring down the neck as the injustice of the destruction screamed in the mind.

With a sudden gasp, the dream broke. Sitting up in the darkness of the room, breathing hard, the realization settled that it was only a nightmare. After wiping the cold sweat from the forehead and drinking a glass of cool water from the clay pot, sleep slowly returned.

A week later, Semparuthi returned to the clinic, carefully carrying the brown envelope containing her laboratory results.

The printed values confirmed the clinical picture: her free T3 and T4 levels were excessively elevated, while her Thyroid Stimulating Hormone (TSH) was suppressed to nearly undetectable levels. Her body was essentially in a state of metabolic hyper-drive.

She required immediate medical therapy to halt the runaway synthesis of thyroid hormones. A prescription for carbimazole under the well-known brand name Neomercazole was written, along with instructions to take the small round tablets diligently three times a day after meals.

“Take these tablets without fail every single day, Amma,” the instructions were given firmly. “They will stop your gland from overworking. Come back and see me after two months so we can check your progress”.

Semparuthi took the small plastic bottle of Neomercazole into her trembling palms as if it were sacred prasadam from a temple. A look of quiet reverence and relief washed over her weathered face. She offered another humble *vanakkam* and stepped out into the village street.

Two months slipped by in the steady rhythm of rural clinical practice.

One bright morning, the clinic door opened, and a woman walked in whose radiant appearance made the entire room feel brighter. It was Semparuthi, but she looked almost entirely reborn.

She had gained healthy weight, filling out her previously hollow cheeks and neck. Her teeth were stained with the cheerful crimson juice of betel leaf, areca nut, and lime, which she chewed with relaxed contentment. Tucked neatly into her oiled, combed hair was a fresh, fiery red hibiscus bloom. She wore a bright red cotton saree, draping her like a beacon of restored life.

Her hands rested steady upon the table, devoid of any tremor. Her pulse had settled to a calm, steady seventy-four beats per minute, and her breathing was deep and effortless.

"Doctor!" Semparuthi beamed, her face breaking into a wide, joyful smile. "These little white tablets in this wonderful bottle are pure magic! Since the day I began taking them, every single misery has left my body. I can walk to the pond and back carrying a full pot of water without losing my breath. My heart beats quietly, my hands are as steady as a rock, I eat with a hearty appetite, and at night, I sleep like a baby without any frightening dreams. It is this little bottle that gave me back my life. May God bless your hands, Doctor!"

"I am truly delighted to see you looking so healthy, Amma," came the reply, smiling at her renewed vigor. "The medicine did its job to bring down your excessive thyroid hormones. Continue taking them as you have been doing. In another two months, we will repeat the blood test to see where your levels stand and decide whether we should continue or alter the treatment".

"Whatever you say, Doctor," she agreed happily, clutching her bottle close before heading home.

Another four months passed, and Semparuthi presented her second blood report.

However, reviewing the printed numbers on the lab paper brought an immediate clinical surprise. Her previously sky-high thyroid hormone levels had plummeted dramatically, and her TSH had spiked far above normal limits. Her body was no longer in a hyperthyroid state; she had crossed over completely into severe hypothyroidism, low levels of the thyroid hormone.

The clinical trajectory of her condition suddenly became crystal clear. Semparuthi was not suffering from standard hyper thyroidism. Her illness was subacute thyroiditis, an autoimmune inflammatory episode of the thyroid gland.

In subacute thyroiditis, the patient's own immune system launches an attack against the thyroid tissue. During the initial inflammatory phase, the destruction of the thyroid gland causes all the pre-stored thyroid hormones to leak uncontrollably into the bloodstream, producing a transient, violent storm of hyperthyroidism. This was the phase during which Semparuthi had first visited the clinic.

However, as the disease progresses and the stored hormones are exhausted while the thyroid cells remain damaged and unable to synthesize new hormones, the condition naturally burns itself out. The patient then inevitably transitions into a hypothyroid phase, where the body suffers from a dangerous deficiency of thyroid hormones.

In medical management, the initial hyperthyroid phase requires hormone-suppressing antithyroid drugs like Neomercazole to protect the heart and body. But once the gland burns out and hormone levels crash, continuing antithyroid medication is disastrous. The patient must immediately stop the suppression tablets and switch to synthetic thyroid hormone replacement, Thyroxine to supplement the body's deficient levels.

Looking across the desk at Semparuthi, the necessary medical course was explained: "Amma, your condition has changed its course, just as expected. Your thyroid gland is no longer overproducing hormone; in fact, it has stopped producing enough. Therefore, you must stop taking the Neomercazole tablets immediately. If you continue taking them, your body will become dangerously sluggish. Instead, I am prescribing this new tablet, Thyroxine. You must take one small Thyroxine tablet every morning on an empty stomach to provide the hormone your body now lacks".

Semparuthi stared at the new prescription slip, her expression darkening with profound distress and disbelief.

"What are you saying, Doctor?" she cried, her voice cracking with wounded betrayal. "How can you ask me to throw away the medicine that rescued me from the edge of the grave? That little bottle of tablets brought me back to life when I couldn't even stand on my two feet! It is my life-saver, my miraculous medicine! And now you are telling me to stop it and take some strange new pill?"

"Amma, please understand," the doctor urged gently, placing a hand on the table near hers. "The illness was in one stage then, and it has moved to another stage now. The old tablets will harm you if you take them now. Trust me, take the new Thyroxine tablets as I have written, and you will remain perfectly well".

Semparuthi took the prescription with a sullen face, her eyes filled with sorrow, and walked out without her usual warmth.

Three months dragged by without any sign of Semparuthi at the clinic.

Then, on a sweltering Tuesday afternoon, a young man on a dusty motorcycle pulled up abruptly outside the clinic doors. He rushed inside, sweating profusely, looking anxious and impatient.

"Are you the doctor?" he asked briskly. "I am Semparuthi's elder son. My mother is lying at home in a terrible state. She has lost consciousness and will not wake up. The neighbors told me you are her doctor. Please come immediately and see her".

A knot of dread tightened in the stomach. Grabbing the emergency medical kit and stethoscope, the doctor climbed onto the pillion of the young man's motorcycle, and they sped off through the narrow dirt tracks of the village.

During the ride, the doctor shouted over the roar of the engine, "How long has she been like this? Has she been eating? What happened to her?"

The son shrugged dismissively without turning his head. "I don't know any details, Doctor. I work in an auto-parts factory two towns away. The neighbor called me on my mobile this morning saying Amma had not opened her door for two days. I had to take unpaid leave to come here. I have so much urgent work back at the plant—please see what is wrong with her quickly so I can figure out when I can leave and get back to my job".

The callous indifference in his voice stung deeply. To this man, his aging mother's critical condition was merely an inconvenient interruption to his daily schedule.

The motorcycle screeched to a halt in front of a desolate, low-roofed thatch hut situated on the barren outskirts of the village. The hut had no windows, and in place of a front door, a tattered burlap sack hung limply across the doorway.

Pushing the burlap aside and stepping inside, the air was cold, damp, and heavy with the smell of neglect. The interior was so cramped that there was barely enough room for a single person to enter; one could not even stand upright without hitting the thatch roof and had to crouch or kneel on the dirt floor.

There, upon a frayed reed mat spread across the cold earth, lay Semparuthi. She was covered up to her chin by a heavy, threadbare cotton blanket. Her face was severely bloated, her eyelids puffy, and her breathing was shallow, slow, and raspy.

Kneeling beside her on the floor, the doctor reached out and took her hand. Her skin felt like marble—icy, dry, and cold to the touch. Her radial pulse was barely perceptible, ticking sluggishly at fewer than forty beats per minute.

"Amma! Semparuthi Amma! Can you hear me? Open your eyes!" the doctor called out, gently tapping her swollen cheeks.

There was no response. Her head rolled limply to the side, lost in deep, unresponsive stupor.

Glancing around the dim hut, the doctor's eyes fell upon the dirt floor right beside her tattered pillow.

There sat three empty plastic medicine bottles, lined up neatly side by side.

Reaching out and picking them up, the labels were examined under the dim light filtering through the thatch: all three were Neomercazole.

Looking around the rest of the hut, there was not a single bottle or blister strip of the newly prescribed Thyroxine anywhere to be found.

The horrifying reality became clear. Semparuthi had never taken the Thyroxine. Instead, driven by her blind faith in her "magic" medicine, she had taken her old prescription to local private pharmacies, purchasing bottle after bottle of Neomercazole and continuously swallowing the hormone-suppressing drug.

Her thyroid levels, already low from the burnout of her thyroiditis, had been crushed to absolute zero. Without thyroid hormone to sustain her bodily functions, her body had failed plunging her into the rarest, deadliest complication of untreated thyroid failure - hypothyroid coma.

"Call an emergency ambulance right now!" the doctor shouted to the son waiting outside. "Your mother is in a deep, life-threatening coma! If we don't get her to the government hospital intensive care unit immediately, she will not survive the night!"

The following evening, the doctor traveled to the government medical college intensive care unit where Semparuthi had been admitted.

Passing through the heavy double doors of the ICU, the atmosphere shifted instantly into a sterile world of polished linoleum, cold air-conditioning, and the relentless electronic beeping of cardiac monitors.

Semparuthi lay in bed number seven, still completely unconscious. Her frail body was hooked to an array of complex medical hardware: a breathing tube through her mouth connected to a mechanical ventilator was breathing for her; central venous lines delivered high-dose intravenous medicines; arterial lines monitored her blood pressure; and multiple infusion pumps hummed softly around her bed.

Standing beside her bed, looking at the complex machinery, a sense of profound detachment settled in. The last time such equipment had been encountered was during compulsory rotating internship years in medical school.

Grassroots rural primary care is an entirely different world of medicine. In the village, healing does not depend on multi-parameter monitors, dialysis machines, or arterial blood gas analyzers. The primary tools of a rural doctor are time, patience, empathy, the human touch, an attentive ear, and an understanding of the patient's daily struggles, family tragedies, and societal fears. It is about sitting on a wooden bench, looking into a patient's eyes, holding a trembling hand, and understanding why a mother weeps or why an elderly laborer skips meals.

Standing inside that hyper-technological ICU room, surrounded by cold steel and flashing digital readouts, felt like being an alien dropped onto an unfamiliar planet.

Presently, the chief ICU intensivist walked into the bay. He was impeccably dressed in a tailored suit beneath a pristine white coat, wearing an expensive silk tie, holding an imported stethoscope in his pocket, and scrolling rapidly through his smartphone with his thumb.

Approaching him respectfully, I spoke: "Good evening, Sir. I am the primary care physician from the rural health center who referred Semparuthi here. I wanted to brief you on her underlying clinical history. She had an initial thyrotoxic phase from subacute thyroiditis, during which she was treated with Neomercazole. When her disease naturally transitioned into hypothyroidism, she was switched to Thyroxine, but due to psychological attachment, she failed to stop the antithyroid drug and continued taking carbimazole, which precipitated this profound coma".

The intensivist did not even raise his eyes from the glowing screen of his smartphone. Without pausing his typing, he flicked a dismissive hand toward the corridor.

“Okay. We’re managing the case. Thanks,” he uttered in a flat, clipped tone.

He did not ask a single follow-up question; he did not look at the clinical referral notes; he did not even look at the rural doctor’s face. With that curt dismissal, he turned his back and walked toward his air-conditioned cabin, his eyes perpetually glued to his device.

Walking out of the hospital into the warm evening air, the stark class division within the medical profession weighed heavily on the mind.

Modern society and institutional medicine have created an unwritten, arrogant hierarchy. Physicians who wear expensive suits, work in multi-crore air-conditioned super-specialty hospitals, speak fluent English peppered with incomprehensible Western medical jargon, terrify vulnerable families, and scribble illegible prescriptions for several expensive drugs are hailed as the elite, superior class of healers.

Meanwhile, those who choose to wear simple handloom khadi shirts, carry a canvas shoulder bag, wear worn leather sandals, and sit on temple verandas, school benches, or under the shade of village tamarind trees to treat the poorest of the poor with patience, love, and basic tools are looked down upon as inferior, second-rate practitioners. The technological arrogance of modern super-specialized healthcare had made itself painfully visible once again.

A month passed, and the rhythms of the rural clinic continued.

One morning, the familiar wooden door pushed open, and in walked Semparuthi.

She was alive, walking on her own two feet, and fully conscious. The bloating had receded from her face, her eyes were clear, and her vital signs were stable.

“Vanakkam, Amma!” the doctor greeted her with genuine warmth, stepping out from behind the desk to welcome her. “Look at you! When I saw you lying in your hut a month ago, you were between life and death. Today, you are walking and looking so clear and strong. Come, please sit down”.

Instead of smiling, Semparuthi sat on the wooden chair, lowered her head, and let out a long, heavy sigh of utter dejection. Large, heavy tears welled up in her eyes and began to stream steadily down her cheeks.

“Oh, Doctor... please don’t talk about it,” she sobbed, wiping her face with the pallu of her saree. “My heart is completely broken”.

“Why, Amma? What is the matter?” the doctor asked with deep concern. “You survived a critical coma, you are healed, and your health is returning. Why are you weeping like this?”

Semparuthi looked up, her voice trembling with raw grief. “Doctor, it was that first bottle of medicine you gave me that saved my life from misery! When I took those little tablets, all my suffering vanished. I felt human again. When you told me to stop taking them, I tried... I swear to God, I really tried to obey you! But the moment I stopped swallowing those tablets, an unbearable fear gripped my chest. My heart began to thump with anxiety, I broke into cold sweats, and sleep deserted me completely. I felt that if I didn’t take that medicine, I would die immediately!”

She paused, wiping her nose, her shoulders shaking with sobs. “So, I took my old empty bottle to the medical shop in town, bought more, and started taking them again every day. I was feeling peaceful! And then, suddenly, everything went black, and I woke up weeks later in that frightening big city hospital with tubes in my throat. And now, all those big city doctors screamed at me, warning me that I must never, ever touch my beloved bottle of medicine again! How can I live without the only thing that protected me?”

Listening to her heartfelt lamentation, the clinical and human puzzle suddenly fell into perfect clarity.

In classical pharmacology, drugs that alter brain chemistry like sleeping pills, are known to create physiological chemical addiction. But carbimazole (Neomercazole) possesses zero chemical addiction properties. It does not act upon dopamine pathways or induce euphoria.

Semparuthi’s profound dependency was not pharmacological; it was deeply, heartbreakingly emotional.

In her solitary, abandoned life, where her husband was dead, her sons had forsaken her, the village panchayat had rejected her, and society had left her to starve in a windowless hut, those small white tablets had provided the very first taste of comfort, relief, and hope she had experienced in years.

In her fragile subconscious mind, she had built an unbreakable belief: *These little tablets are the only entity in the entire world that truly cares for me; they are my saviors, my magic, my absolute shield against death*.

The medicine bottles had become the surrogate children she never had. Where her real sons had failed to support her, the little plastic bottles had stood by her side every morning and night. To ask her to discard that bottle was, in her mind, equivalent to asking a mother to abandon the only loyal child who had ever stood by her in her darkest hour. That was why she had clung to them so desperately, even unto the very gates of death.

The doctor pulled the chair closer to Semparuthi, signaled to her elder son, who was lingering awkwardly outside the door, to come inside, and had them both sit down.

For the next two hours, the clinic consultation was set aside for a long, unhurried, heartfelt conversation.

"Semparuthi Amma," the doctor spoke softly, looking directly into her eyes. "Tell me something. Do you remember the day you got married, thirty years ago, when you had to leave your parents' village and come live here as a young bride?"

Semparuthi blinked, surprised by the question, and nodded slowly.

"Wasn't it terrifying and terribly painful?" the doctor asked gently. "Leaving behind your mother, your father, your loving brothers, your sisters, your childhood friends, and the only house you had ever known to come live among total strangers? Didn't your heart ache with grief in those first few weeks?"

A soft, nostalgic smile flickered across Semparuthi's tear-stained face. "Aiyo, Doctor, why do you remind me of that? When I first arrived in this village as a newlywed girl, I cried every single day for two straight months. I refused to eat, I missed my mother terribly, and I felt I could never survive in this new place".

"And then what happened, Amma?" the doctor asked with a warm smile.

"Then... slowly, day by day, I got used to this house, to my husband, to the neighbors," she answered softly. "I learned the new ways, and eventually, this village became my true home".

"Exactly, Amma," the doctor said, holding her hand reassuringly. "Your body and your medicine are standing at that exact same crossroad today. That old bottle of Neomercazole was like your mother's home—it protected you, nurtured you, and cured your sickness when you were in desperate need. But that phase of your life has ended. Now, you must pack your bags and move into the new

home—this new bottle of Thyroxine. In the beginning, just like those first two months of your marriage, your mind will feel anxious, unfamiliar, and sorrowful. But you must give it time. Slowly, day by day, your body will accept the new tablet, and it will become your lifelong friend and protector".

Semparuthi sat in silence, absorbing the metaphor, the tension slowly draining from her face.

The doctor then turned to her son, speaking with firm, unyielding clarity.

"Look at your mother," the doctor told him. "She almost died not because of an incurable disease, but because of extreme loneliness and fear. Medicine alone cannot heal an empty heart. Right now, her mind needs healing just as much as her thyroid gland. Take her with you to your town today. Let her stay in your house for the next one or two months. Let her play with her grandchildren, let her eat warm food with her family, and let her feel that she is loved. You personally supervise her every morning to make sure she takes one Thyroxine tablet on an empty stomach. Once she is fully settled and healthy, she can choose whether to stay with you or return to the village. Can you do this for the mother who gave you life?"

The son looked down at the floor, his face flushed with shame, before looking up at his frail mother with tears in his eyes. He reached out and held her rough, calloused hands.

"I will take her with me today itself, Doctor," he promised softly. "I will look after her every single day".

Semparuthi looked at her son, then at the doctor, and finally down at the new bottle of Thyroxine tablets resting on the table. With a calm, steady hand, she picked up the new bottle and placed it securely inside the folds of her red saree. The heavy shadow of grief had finally lifted from her face, replaced by a quiet, deep-seated peace.

Semparuthi walked out of the clinic that afternoon with her son by her side, leaving behind a profound, unforgettable lesson in the true philosophy of healing.

Modern medicine will always need its dazzling technological advancements—the multi-million-dollar imaging scanners, the high-precision biochemical assays, the state-of-the-art intensive care ventilators, and the specialized critical care units are all indispensable in saving acute physiological lives.

Yet, all the advanced technology in the universe remains fundamentally incomplete without a physician who possesses genuine compassion, unhurried patience, and deep cultural and social empathy for the human being sitting in the patient's chair.

A doctor who only looks at lab numbers on a computer screen and prescribes drugs from behind an ivory tower could never have diagnosed or cured Semparuthi's true ailment. To heal her, one had to understand her poverty, her abandoned motherhood, her social humiliation, and the profound emotional attachment she had formed with a tiny plastic bottle of pills.

Though grassroots rural doctors may forever face institutional condescension, lack of corporate prestige, and daily systemic challenges, the quiet, transformative work of healing human souls beneath the shade of village trees remains an unmatched, noble calling—the true, beating heart of medicine.

Calm in a storm

Mohan is a 40-year-old man from the village near our clinic. His tall and athletic build indicated that he must have been involved in some kind of sports during his younger days. He is a lorry driver who does regular long distance inter-state transport of goods and products from Tiruvallur all the way to Punjab, Gujarat and Delhi. His clothing style choice is quite interesting. He always wears a round neck T shirt and a white shirt on top which he does not button and a lungi of traditional blue, black and white stripes. The T shirt is always light colored, a stark contrast to his dark complexion. He would always have his hair impeccably combed back in waves. People in the rural areas where I work do not believe that you need to be well groomed and dressed while going to a clinic. After all, one is sick when they go to the doctor. Some don't even wash their face or brush their teeth. Every time I have seen Mohan in the clinic, he would be well dressed and groomed. It was such a stark contrast to other people. But one thing that was disturbing about Mohan was that he would always reek of a combination of engine oil, petrol, and alcohol.

Alcoholism is a major problem in the villages where I work. Many men consume alcohol daily. Some binge on alcohol during weekends. Not even one clinic passes without a man coming in with stomach pain, vomiting and inability to keep in food because of alcohol induced gastritis. We also get many men with acute alcohol withdrawal symptoms which range from tremors and irritability to outright seizures and loss of consciousness.

"You are back again! I really don't understand why you guys drink so much" the staff nurse was telling Mohan as she recorded his blood pressure and documented it in his case report. "This has become your routine. Every month you come here just for your alcohol issues!" Saying this she nudged Mohan into the consultation room. On some days drunk people misbehave with the staff and talk to them in abusive language. So there is a sense of judgment and defensive approach when such regular patients come to the clinic.

"Long time since I met you. How are you? What brings you to the clinic today?" When I asked him this, I was expecting the usual story of going out with friends, or a celebration in the village, and binge drinking. That was Mohan's routine. At least once a month he would come with a heavy alcohol binge, and associated stomach problems or headache.

"Five days ago, when I was returning from Mumbai in the lorry. I had parked in the highway to have my dinner. When I finished dinner and came back to the

lorry, the spare wheel fell and bounced on my foot. It scraped on my foot and the skin got peeled off. It is swollen and oozing pus. Can you please take a look?"

The smell of alcohol was absent this time and he seemed to be sober. Sober Mohan was a very different man. He looked calm, composed and had a gentle smile on his face. The sober look, the smile and his usual dress style made him look much younger than 40.

I examined his foot. There was a huge gash in the space between the great toe and the second toe of his left foot. The gash looked pale, there was a lot of pus and surrounding flesh looked unhealthy grey and had started rotting. It is unlikely for a simple injury to start rotting like this.

"It looks like the wound is heavily infected. We may have to clean it and do dressings. We also must start antibiotic medicines to remove the infection. Do you have pain?"

"No, there is no pain"

It was strange, because with such a frank infection, pain is invariable present. Why was there no pain? I started wondering whether Mohan had diabetes. Because when a person has diabetes, they lose the pain sensation over the feet. Also the infection becomes more severe due to lower immunity in patients with diabetes. I started wondering whether I am dealing with a diabetic foot ulcer here.

"Do you have diabetes?"

"Yes, I was told I have diabetes last year"

I was not aware that he had diabetes. For someone who keeps traveling all the time, treatments happen at different cities. Alcohol binge happens mostly when he is back home and so our clinic only catered to his alcohol related problem. He was getting diabetes treatment elsewhere. But Mohan was not on any medicines for diabetes.

"Before I start the cleaning and dressing of the wound, I would like to know how much your blood sugar is. Let us get it tested."

The sugar was very high, and I decided that we may have to start him on Insulin. But that would be a later conversation. I had to attend to the wound now.

What initially appeared as an infected wound in the foot was now a diabetic foot ulcer. In the rural clinic where I work, diabetic foot ulcers are extremely common.

Rural life involves bare foot walking, getting into the mud and dirt and working in the fields. This and poorly controlled blood sugar make for a perfect recipe for diabetic foot ulcers. Foot ulcer dressings are a routine in our clinic. Mohan's ulcer required what we would call a debridement. Debridement means removing dead, infected and damaged tissue from a wound, and cleaning it thoroughly. It removes the load of infection from the body, and by exposing healthy tissue, it hastens healing of the wound. A thorough debridement requires a lot of cutting away of damaged flesh and is a bloody and messy procedure. People like Mohan, who have diabetes, and heavy alcohol users, generally have numbing of all the nerves in the feet and so they rarely feel the pain of all the cutting and cleaning. I positioned Mohan in the dressing room over the procedure couch, placed a wide bowl under his foot to collect all the fluids, blood and mess, and took out the dressing kit and started the procedure. As I was removing pus and tissue, I had a conversation with Mohan to distract him and to engage him.

"Tell me about your family. Who are all living with you in your home?"

"I live alone. My parents died when I was 20-22 years old. I had an elder sister. I took loans and got her married. She has moved to Tiruvannamalai and lives there with her family. She has two daughters. I live here alone."

"So, how do you manage food? Do you cook?"

"I know to cook. But on most days, I don't. I am traveling 15-20 days of the month. So I eat all my meals outside. The remaining 10 days that I am here, I cook and eat."

"Do you like your job?"

"What is there to like? It goes on. Salary comes regularly every month. Sometimes I wonder why I am earning all this money"

Throughout this conversation I was concentrating on cleaning and debriding his wound and listening to him. I looked up and saw his face. He looked calm and had a gentle smile on his face. It was surprising to see him smiling and there was something very nice about the smile. It was natural and it involved every part of his face. The mouth, cheeks, eyes, forehead, eyebrows, everything was smiling. I was wondering if Mohan was enjoying the little conversation that were having. Maybe this was the first time someone had asked him to talk about himself and was listening. It was an infective smile, like all genuine smiles are, and I found myself smiling ear to ear.

There was some tough tissue in the wound which needed some sharp cutting. I requested the staff nurse to give me a new blade. The conversation had stopped now because my attention was fully drawn to the tough tissue that I needed to cut out. The tissue was tough and I was giving gentle pressure to cut it out. After a few seconds of persistent pressure, the tough tissue gave in and came off. Along with it came a huge spurt of blood. The blood gushed out in a large volume. I took a clean piece of gauze and pressed it hard on the bleeding area. Usually, infected tissue are very fragile and they bleed a lot. All it usually requires is some pressure. The bleeding stops. But as I kept pressing gauze after gauze it got soaked with blood. The blood oozed into the bowl that was kept below his foot and all the wash water on the bowl got bloody. This was not a gentle ooze, it was not a trickle, it was a heavy gush of blood. The staff nurse standing next to me said, "Doctor, looks like the bleeding is quite heavy"

I looked up at Mohan; he looked back at me with the same gentle smile. He was oblivious of what was happening down there in his foot.

I removed the pressure just to get a clear look at what was happening. The moment I lifted the pressure the blood spurted like a fountain. Immediately I knew I had cut a crucial blood vessel and when a blood vessel is cut, it is impossible to arrest the bleeding by just applying pressure. I had to tie the vessel using a suture material to arrest the bleeding. It was time to act fast.

"Sister, emergency. Get me some pads. Clean up this area. I need a suturing tray immediately. Get me the one that has the artery forceps and the suturing kit. Need to make some sutures and arrest the bleeding." I shouted out for help. Immediately two staff nurses who were attending to other patients and dispensing medicines rushed into the dressing room. One quickly removed the bowl, changed the rubber sheet that was there underneath. The other nurse brought a suturing tray and started arranging it on the table nearby. A third nurse came. I looked at her and said, "Sister, wear your gloves and apply pressure here" and I showed her the bleeding spot. Meanwhile I changed my dirty gloves and wore a new pair.

"Mohan, while cleaning your wound, I accidentally cut an artery, and it is bleeding heavily. I am going to try and suture it and try to control the bleeding. Don't worry, it won't take long"

Mohan was still smiling and he nodded at me. He leaned back quietly as though he was moving aside to let way for us to do what we were attempting.

The blood was flooding the wound, and I just couldn't see where it was bleeding from. I tried to blindly take a stitch to tie the bleeding vessel, but it did not work. Before it is too late, before some heavy blood loss occurs, I wanted to rush him and show him to an experienced surgeon who may be able to easily control the bleeding.

"Sister, I am unable to localize the vessel. Unable to stop the bleeding, we must rush him to the hospital nearby. Please prepare the car" We do not have an ambulance. Our clinic is far away, that even if we called an ambulance it was going to take more than 30 mins for it to even reach us. So, I took some heavy gauze pads and held them pressed against the bleeding wound and we transferred him to the trolley and wheeled him outside. "Mohan, do not worry, I was unable to locate the bleeding and unable to stop it. But we are rushing you to a hospital where an experienced surgeon will control the bleeding immediately." By now I was sweating profusely with all the effort and the adrenaline. Mohan gave me the same calm look and smile and moved along with us and just gave in to everything that we were doing. He nodded at me in an understanding manner. That one nod greatly calmed me.

Within the next 15 minutes we were in the casualty department of the secondary hospital. On the way to the hospital, I had already called the surgeon and spoke to him. He said, he is available and will see the wound as soon as we reach the casualty. By now my fingers were cramping with all the pressure I was applying on the wound to stop it from bleeding. The bleeding had reduced significantly but it had not completely stopped. The moment I removed my hand from the wound, the surgeon saw the blood spurt, and then and there in the casualty he took a stitch on the wound and tied it. The gush of blood slowed down and in just those 2 minutes the bleeding completely stopped and the remaining blood in the wound just clotted and became a jelly. I felt elated to see the blood stop gushing and wanted to pump my fists and jump. But it was just another routine day in the casualty for the surgeon and his team. The surgeon looked at me and nodded. I thanked him profusely. His junior resident took over and cleaned up the wound and applied a dressing on it. After he was done in 5 minutes the wound appeared like nothing had happened. I looked at Mohan and he still had the same smile on his face. He looked like nothing happened there. Mohan and I were exchanging looks, me with a frenzied elation of seeing the wound so clean, and him with a calm and composed expression.

The return trip to the clinic in the car was quiet. No frantic phone calls, no cramping fingers pressing on the bleeding wound, no sweating or palpitations.

"Mohan, I am sure you must be wondering what just happened. I am so sorry for all the trouble. I accidentally cut the blood vessel and could not stop the heavy bleeding."

The smile on Mohan's face widened and he raised his eyebrows, "Doctor, what is this? Why are you saying sorry? Please doctor. Don't say sorry" That is all Mohan said. I looked outside the window into the traffic on the road. Mohan's calm demeanor and his brilliant and bright smile spoke volumes. Mohan understood that what had happened was an accident. He got that I had good intentions when I tried to remove the dead flesh from his wound. He had surrendered his foot to us in that faith. He was seeing at each step that we cared and did all that we could to save his foot and stop the bleeding. I was overwhelmed by emotions through the rest of the ride. Patients understand when they are genuinely cared for. Sometimes even if outcomes turn out to be negative patients understand acts that are done in good faith. They smile and acknowledge the efforts through the pain and suffering. Mohan gave me hope and the motivation that I must continue doing what I do for patients in the belief that they will understand like how Mohan did.

When we got down from the car in the clinic, Mohan walked inside. I walked behind him. His white shirt and the t shirt were drenched in sweat. His hair was also wet. Despite the adrenaline and all the panic, which has led to this profuse sweating, Mohan still had the calm smile on his face. We both got into the consultation room and sat down. Mohan sat slouched in his chair. He looked exhausted and his hands were trembling.

"Did this whole thing scare you?" I asked.

"For a little while I was afraid, I was going to lose my foot. If I lose my foot, I do not have anyone to take care of me. I do not know any work other than lorry driving. I started thinking about all that."

"But Mohan, you appeared so calm throughout."

"I know you are doing your best. I don't want to add to your confusion." Even as he said this Mohan's smile widened.

Suddenly the Mohan who would look always drunk and dazed, smelling of stale booze, appeared as the most handsome and inspiring man in the room.

Mysterious Medicine



Some strange and mysterious clinical encounter with patients.

The miraculous pregnancy

The morning sun was just beginning to climb over the rooftops of the city, casting a soft, golden warmth across the terrace. The damp clothes I had rinsed smelled cleanly of detergent and fresh water. Moving slowly under the weight of my swollen, full-term belly, I reached up toward the nylon clothesline. Every small movement in these final days required conscious effort. My doctor had confirmed earlier this week that the baby had engaged; delivery was imminent, likely within the next few days.

As I lifted the final bedsheet, smoothed its corners, and pressed a plastic peg onto the line, an electric jolt struck the base of my spine.

It was unlike any ache I had felt before. A sharp, radiant spasm gripped my lower back, clawed around my pelvic bone, and coursed down my thighs. The breath caught hard in my throat. I dropped my hands, gripping the terrace parapet as the brick wall grounded me against the violent wave. During the antenatal classes at the hospital, the midwives had described true labor contractions in detail, emphasizing that the first delivery was often a prolonged, grueling passage that unfolded over many hours.

It has begun, I thought, my heart beating in a mix of terror and reverence.

I stood still, breathing in through my nose and exhaling slowly until the tightening receded. There was no need to panic. I picked up the plastic laundry basket, slid my feet into my slippers, and carefully navigated the narrow flight of stairs down into my apartment.

I picked up the phone and dialed Sunitha.

“Hey,” I whispered, holding the receiver against my ear. “I think the pains have started. Can you come over?”

“Stay right there! Don’t you dare move! I’m leaving right now!” Sunitha’s voice erupted through the speaker, breathless and high-pitched with instant panic.

I smiled faintly as I set the phone down. It was almost comical that my sole birth partner was an unmarried, childless young woman who became anxious at the sight of a minor cut. In normal circumstances, a woman in labor is surrounded by the steady hands of a mother, an aunt, or an experienced grandmother whispering ancestral assurances. But I had neither mother nor father; I was

a child who had grown up within the whitewashed, echoing corridors of a charitable orphanage. I had never known the warmth of a maternal lap. Now, I would have to be the anchor, steadying Sunitha's fragile nerves even as my own body prepared to tear itself open to bring life into the world. Sunitha was my dearest friend—we had shared fifteen years of life, food, secrets, and quiet griefs—but when it came to childbirth, she was utterly innocent. I would have to protect her while she tried to protect me.

I walked into the small puja alcove in the corner of the living room. Framed pictures of Lord Ganesha, Lakshmi, and Saraswati stood beside portraits of Shirdi Sai Baba and Lord Rama with Sita, Lakshmana, and Hanuman. The jasmine garlands I had offered the previous morning were wilted and dry. I took them down gently, placing them in the disposal bin. I poured fresh sesame oil into the brass standing lamp, trimmed the cotton wick, and struck a match. The flame flickered to life, bathing the deities in a warm, amber glow.

"Lord," I murmured, pressing my palms together over my chest, "let everything go smoothly. Let this child arrive whole and healthy. Grant me the strength to carry this baby back through this doorway and place it before your feet."

I lit an incense stick, letting the fragrant smoke drift through the quiet apartment, and eased myself into the armchair in the hall. Sitting there in the stillness, the river of memory broke its banks.

I remembered entering the mechanical engineering department years ago. In a lecture hall of one hundred students, only twelve of us were girls. Being conventionally attractive, I had to deflect constant attention; every few weeks, some boy from the senior or junior batches would confess his sudden affection. In time, they realized I was only interested in solid, uncomplicated friendship. Wonderful camaraderie blossomed with many of my peers, but Praveen was different. Praveen was relentless. For two continuous years, he pursued me with an earnest, unyielding devotion that was completely free of arrogance. He was an exceptional student, quiet yet brilliant.

Praveen carried the soothing, timeless charm of Ilaiyaraaja's rustic melodies, the kind of music that seems unassuming at first, but slowly anchors itself deep into your soul until you cannot imagine a day without its resonance. A profound love bloomed between us. By the time our final semester ended, both of us had secured respectable engineering jobs in campus placements. We were twenty-two years young, naive, and fiercely convinced that the world could not keep us

apart. We decided to marry, oblivious to how little we truly understood of life's cruelty.

The sharp chime of the doorbell pulled me from the past.

I braced my hands against the wooden armrests and stood up carefully, walking to the entrance. When I unlatched the door, Sunitha almost fell into the room. She wrapped her arms around my shoulders with extreme, comical gentleness, barely touching my back as if terrified that the slightest pressure might cause the baby to pop out right onto the doormat.

"On the way here, I called Ravi Anna," Sunitha stammered, her eyes darting frantically over my stomach. "He's out of town on work, but he said he has arranged a car to pick us up and take us to the hospital."

She hurried into the bedroom to inspect the overnight duffel bag we had packed the previous week. She unzipped it, muttering under her breath as she checked off the list: baby clothes, sterile cotton pads, hospital admission files, clean towels, and maternity gowns. Ravi Anna was our older friend, a brother in every sense of the word. He and Sunitha maintained a constant, jealous rivalry over who was truly my closest protector, a quiet tug-of-war whose warmth had sheltered me for years.

Suddenly, a violent contraction whipped across my lower abdomen like a heavy lash. The sheer force of the spasm dragged my center of gravity downward; my knees buckled, and my hands clawed at the air.

"Ah!" I gasped, bending double.

Sunitha dropped the bag, rushed across the room, and caught me under my arms, guiding me back into the armchair. Beads of sweat broke across her forehead; her hands were shaking violently. Seeing her sheer terror, I took a ragged breath as the cramp subsided, reached out, and gently rubbed her back.

"Sunitha, why are you trembling so much?" I asked with a faint chuckle, trying to lighten the crushing air. "Who is going into labor here, you or me?"

Her cheeks flushed crimson. "Shut up! You have no idea how terrifying you look when your face turns white like that!"

We sat by the bags, waiting for the sound of the car.

My mind slipped back into the shadowed corridors of memory. Marrying Praveen had not been a simple matter of choosing a date. I was a girl without pedigree, an orphan whose lineage began and ended at the gates of an asylum. Praveen belonged to a traditional, conservative family that prized ancestral reputation above all else. His parents viewed my existence as a stain upon their lineage. One evening, his mother called my phone. The venomous words she spat into the receiver—calling me an opportunistic stray who had snared her son—pierced my soul so deeply that the scars remained etched forever. On that very day, with quiet resolve, I made two promises to myself: I would marry Praveen because our love was untainted, but I would never speak another word to the woman who had sought to demean my dignity.

A sharp horn sounded from the street below.

Sunitha shouldered the duffel bags, and together we made the slow, agonizing descent down two flights of concrete stairs. As Sunitha turned the key in the front door lock, I rested against the banister and managed a weary smile.

"Lock it carefully, Sunitha," I whispered. "Keep the key in your purse. In three days, when we carry our little one home, I don't want you searching your pockets saying you lost it."

"Oh God! I just put the clothes to dry on the terrace! Can you get them when you come back from the hospital later tonight?"

"Just get down the stairs safely," she scolded, though her eyes softened with affection.

We climbed into the back seat of the hired car. The moment the driver shifted into gear and pulled onto the main road, another contraction struck. This one was monstrous, a crushing vise that seized my diaphragm and stole the oxygen from my lungs. The resolve I had kept to remain silent shattered. A raw, guttural cry was torn from my throat.

"Hold on, hold on!" Sunitha cried, gripping both my hands in hers, squeezing with all her might as I leaned my head back against the faux-leather seat, weeping through the wave of pain.

When the contraction faded, the silence in the moving car felt heavy. I closed my eyes, seeking sanctuary in the memory of Praveen.

I remembered that frantic evening when Praveen had called at six-thirty, his voice cracked with strain. The fury in his family home had escalated to ultimatums and threats. 'Pack what you have and meet me at the central bus terminus,' he had told me. 'We are leaving.' I packed a single bag of clothes, took five thousand rupees—my entire month's savings—and walked through the crowded streets to the terminus. When we boarded the interstate bus and felt the cool night air rush through the open window, the heavy burden of fear melted into intoxicating freedom. We held hands through the long night, our fingers locked together as the bus rumbled into the dawn.

The car screeched to a halt, breaking my reverie.

I opened my eyes to find my hand tightly clasped in Sunitha's. As I tried to swing my legs out of the car, another violent contraction seized my abdomen. My legs gave way completely. Before I could hit the tarmac, two duty nurses rushed out with a wheelchair, catching me under the shoulders and easing me into the seat. The spasms were coming in rapid succession now, offering no respite between the crushing peaks. Sunitha sprinted ahead to present my medical charts at the triage desk, and within moments, the heavy doors of the labor ward swung open, swallowing me into the sterile world of clinical urgency.

Inside the preparation bay, the pain briefly paused, leaving me bathed in a cold sweat. I closed my eyes once more, desperate to keep Praveen beside me in spirit.

When we had first arrived in Chennai, Praveen's cousin Prakash and his wife Malathi had offered us shelter. They welcomed us not as runaways, but as family. After dinner on that first night, Malathi had sat beside me and spoke with gentle grace. 'Please don't misunderstand me,' she had said kindly. 'Until the wedding rituals are legally complete, you will stay here with us. Praveen will stay at my brother's flat down the road; he is a bachelor and has extra space. It is only a matter of two weeks, isn't it?'

Her respect for our dignity was touching. We had waited through years of quiet longing; waiting another fortnight to hold each other as husband and wife was a small sacrifice. I could still see the wistful, loving smile on Praveen's face as he stepped out into the night, waving back at me from the gate.

"Amma, open your eyes please," a voice called out.

A young junior resident in a green scrub suit was tapping my shoulder. As she began her preliminary examination, another contraction seized my body. I bit down viciously on my lower lip to suppress the scream, tasting the metallic warmth of blood as the skin broke. I barely felt the split lip; the agony in my pelvis consumed every sensory nerve in my existence. It felt as though my entire identity had ceased to exist, reduced entirely to a contracting uterus and the fragile child pushing to be born.

The junior doctor stood up, her brow furrowed. "Everything looks stable, but I need to call Dr. Thilagavathi," she said softly. "She will perform the internal examination to check cervical dilation and station."

I lay on the gurney, staring at the white ceiling tiles, remembering the day our world ended.

We had found a small rented house in the city and paid the deposit. The appointment at the sub-registrar's office was fixed for ten in the morning on October twenty-sixth. I arrived in a simple red silk saree accompanied by Prakash and Malathi; Praveen arrived from his bachelor accommodation, looking radiant and handsome. With two signatures and the exchange of a simple garland, we became husband and wife.

As we stepped out of the registrar's office to head to our new house for the traditional milk-boiling ceremony, a sudden monsoon downpour drenched the streets. Praveen raised an umbrella and stepped into the rain to hail an auto-rickshaw across the road. I stood sheltered under the building's concrete portico, watching his back.

A heavily loaded truck, skidding on the slick asphalt, lost control and slammed violently into the road divider.

In a single, deafening crash, the world shattered. Beneath the massive steel tires lay the crumpled black fabric of the umbrella and a spreading pool of dark crimson blood. Ten minutes after becoming my husband, Praveen was lying dead upon the rain-swept asphalt. People were screaming, running, and shouting into the rain, while I stood frozen—breathless, voiceless, and utterly destroyed.

"Deep breaths, take deep breaths," a voice instructed.

Dr. Thilagavathi stepped to the side of the labor bed, her face calm and reassuring.

“Doctor,” I gasped, clutching the side rails, “the pain is unbearable. How much longer? How much has it opened?”

“Do not be afraid,” Dr. Thilagavathi said kindly, pulling on a pair of sterile surgical gloves. “I have told you before: a primigravida requires time and endurance. Patience is essential. Let me examine you.”

She proceeded with the internal examination to evaluate the cervix. As her fingers moved, her demeanor shifted abruptly. Her clinical composure vanished, replaced by profound bewilderment and concern.

“Is something wrong, Doctor?” I asked, cold panic cutting through the labor pains. “Why are you looking like that? Please tell me.”

Dr. Thilagavathi did not answer me immediately. She turned sharply toward the junior resident. “Is Dr. Vasantha in her office? Go fetch her immediately. Tell her I have an extremely confusing, anomalous case in Labor Bay Three.”

My heart sank. For nine months, every ultrasound, blood panel, and fetal heart check had been completely normal. Why was there confusion now? Where was Sunitha? Was my baby in distress?

The door opened, and a senior physician with silver hair and a serene countenance entered the bay. This was Dr. Vasantha, the chief obstetrician. She carried an aura of quiet wisdom that immediately softened the tension in the room.

“What is the issue, Thilaga?” Dr. Vasantha asked gently.

She approached the bed and laid a warm palm upon my tense abdomen. At that exact moment, a violent contraction ripped through my pelvis. The agony shattered all restraint, and a scream tore from my throat. Through the haze of pain, the urgent conversation between the two doctors reached my ears.

“Madam, look at this,” Dr. Thilagavathi murmured in astonishment. “She appears to be an absolute virgin. Her hymenal ring is completely intact and unyielding. I cannot perform an adequate digital assessment of the cervix because the introitus is entirely virginal. In all my years of practice, I have never seen a full-term pregnant woman with an intact hymen. I don’t know how to proceed with the examination.”

“What are you saying, Thilaga?” Dr. Vasantha replied, raising an eyebrow. “How can a woman carry a full-term pregnancy with an intact hymen? Let me examine her.”

Dr. Vasantha pulled on sterile gloves and approached me. “Rasathi, my queen, do not be afraid,” she said in a soothing tone that felt like the gentle voice of a mother I had never known. “Let me examine you quickly, alright?”

I nodded tearfully, yielding to her touch.

Dr. Vasantha completed the examination and looked up at her colleague in utter awe. “You are right, Thilaga. Her hymen is fully intact and infact tightly closed. What a marvel.”

Dr. Thilagavathi looked at me, unable to contain her professional curiosity. “Child, your chart says you were married, and that your husband passed away. But you are anatomically a virgin. How is this possible? Is there an explanation for this?”

I looked up through my tears, the contractions gripping my body. “Doctor... does this mean I cannot deliver normally? Will my baby be harmed?”

Dr. Vasantha stepped in, gently patting my shoulder before turning to the younger obstetrician.

“Thilaga, monitor her labor progression closely,” Dr. Vasantha commanded with authority. “Perform a bedside ultrasound immediately to verify cephalic engagement and pelvimetry. If the descent continues smoothly, allow her to progress. When the baby’s head crowns at the perineum, perform a controlled hymenotomy and a standard episiotomy to allow passage. If there is any arrest of descent, alert me immediately. I will be on standby.”

Dr. Vasantha leaned down, wiped the sweat from my brow, and smiled down at me with profound reverence.

“This is truly an immaculate conception,” she whispered softly.

As the doctors moved to arrange the ultrasound monitor, I closed my eyes and let the tears flow freely down my cheeks.

They did not know the battle that had brought me to this delivery bed. Having our legal marriage certificate signed that morning had been a miracle of timing.

When Praveen's body lay in the mortuary, I knew that the only way his existence would continue on this earth was if I carried his lineage within me. When I begged the doctors to perform post-mortem sperm retrieval before his body was cremated, his parents had vehemently opposed it, desperate to erase every trace of our union. But because we were legally wed, the law stood firmly on my side. I fought them in the court of law, established my legal rights, and secured my husband's genetic legacy.

The month after Praveen's death, reproductive specialists performed an artificial intrauterine insemination. By the grace of the divine, my body received the seed on the very first attempt.

The past nine months had not merely been a pregnancy; they had been a sacred, solitary penance. A living part of Praveen had been growing within my womb, keeping me company through every lonely night and every tear-stained morning. Even in death, Praveen had never left my side.

Another contraction rose within me, powerful and unyielding. In a few short hours, our child would take its first breath in this world. This child had not been conceived through physical union, but through the boundless power of devotion, resolve, and pure love. It was, in the truest sense of the word, an immaculate creation. Realizing this, I opened my eyes, gripped the hospital bed, and prepared to push with all the strength of my soul.

The vanishing tumour

The morning was hesitant to wake. At six o'clock, the eastern sky was a wash of orange and pink with a few shades of yellow showing here and there. The rustic rural area had not yet gotten busy with the day. The world was still hushed, suspended in an ethereal silence broken only by the delicate rustling of leaves and the distant, rhythmic chirping of birds.

I pushed open the weathered wooden door of my quarters and stepped out into the open courtyard. The air was crisp, washed clean by the nocturnal mist, carrying the damp, earthen fragrance of fertile soil, crushed grass, and Indian borage leaves. I walked over to the wide stone veranda that skirted the boundary of the clinic. The granite slabs were delightfully cool beneath my feet. I picked up yesterday's newspaper resting on the ledge, snapped the dust from its folded pages, and let my gaze wander across the waking countryside.

As far as the horizon could reach, the landscape was a sprawling sea of emerald green. In the adjoining garden, vibrant bitter gourd vines had woven a lush, intricate tapestry over the wooden fence. Their bright yellow blossoms glowed like tiny lamps in the pale dawn, swaying gently under the weight of the gentle breeze. It was the kind of morning that felt ancient, almost sacred, where time moved at the pace of a deep breath.

I had arrived in this secluded hamlet, roughly eighty kilometers outside the bustling metropolis of Chennai, barely two months prior. The setup was simple: a modest, tiled-roof clinic with a small cottage adjoining it for my residence.

Those early weeks had tested my patience and resolve. In rural areas, modern medicine is not accepted merely because a doctor has all fancy education and degrees. Trust here was a sacred covenant, earned through discussions in tea stalls, temple courtyards, and weekly markets. My patients were few, their eyes guarded and cautious, waiting to see if this new, young doctor had what it takes to help them. I possessed neither a grand reputation nor local roots; I was dressed differently, spoke a different dialect, and even looked very different; my only capital was interest to learn, work and be of use.

Suddenly, a frantic, thunderous banging shattered the tranquil morning symphony.

Thud! Thud! Thud!

The iron knocker struck the clinic's heavy metallic gate with desperate, reverberating urgency.

I dropped the newspaper. Muthamma, the housekeeping staff who arrived promptly every dawn to unlock, sweep, and prepare the clinic, was nowhere to be seen yet.

I hurried past the courtyard toward the entrance, still dressed in the loose, rumpled cotton nightclothes I had slept in.

Through the iron gate, I saw them. An old, dust-coated motorcycle stood tilted on its side-stand nearby. Supporting a frail, middle-aged woman with both arms was a young man, barely in his early twenties. His face was drawn tight with raw panic, his broad shoulders trembling under the weight of the woman who leaned heavily against his chest, clutching her side and groaning in agony.

"Brother! Hold on, the clinic isn't open yet!" I called out urgently as I approached. "What happened? Is she your mother? What is wrong?"

The young man had been knocking so furiously that he hadn't noticed me approaching from the courtyard. He spun around, his eyes wide, bloodshot with terror and exhaustion.

"Sir! Please, Sir!" he choked, his voice cracking like dry wood. "It's my mother... her stomach! She has been writhing in pain all through the night. She couldn't even weep aloud by the end, only trembling. As soon as the day broke, I brought her here. Please see her, Doctor!"

"Set her down gently on the stone ledge right here," I instructed, placing a steadying hand on her shuddering shoulder. "Don't let her strain. I will run inside and grab the keys."

I bolted back into the cottage, grabbed the ring of brass keys, and unlocked the heavy double doors of the clinic. The familiar scent of antiseptic, spirit, and hospital linen met me. Together, the young man and I supported his mother, guiding her slow, agonizing steps into the examination room and helping her recline onto the vinyl examination table.

The morning sun began to cast long, golden shafts through the slatted windows, illuminating the dust particles dancing in the air.

“Breathe, Amma. Just slow, gentle breaths,” I whispered softly.

Her face was etched with exhaustion, her lips pale and dry. She kept her eyes shut tight, her knuckles white as she gripped the edges of the bed sheet.

I pulled a stool beside the bed, warmed my hands by rubbing my palms together, and gently lifted the hem of her faded cotton saree to expose her abdomen.

I began my examination, expecting perhaps the localized guarding of some serious disease of the abdominal organ. My fingertips pressed lightly on different parts of her abdomen. She did not flinch.

Then, my hand moved to the left lower abdomen and the back.

My breath caught in my throat. My fingers froze.

Beneath the thin layer of abdominal muscle lay something monstrous. It was a massive, perfectly distinct, rounded swelling. I pressed deeper, testing its borders. It was unyielding, dense, and solid as a rock. It filled the left flank entirely, roughly the size of a giant, ripe Salem mango.

“Does this hurt, Amma?” I asked, applying firm, deliberate pressure against the formidable lump.

The woman simply shook her head weakly, not a murmur of pain escaping her lips.

A chill crept down my spine. The absence of pain over a hard, solitary mass was the classic, ominous signature of something far more sinister than an infection. In the books of clinical medicine, an immobile, rock-solid abdominal mass of that magnitude indicated advanced cancer that had grown silently.

I glanced up from the examination couch. The young man stood in the corner of the room, his arms crossed tightly over his chest, watching every micro-expression on my face. His hair was wild and uncombed, and deep, dark rings of sleepless anxiety encircled his eyes. He shifted his weight nervously from his left foot to his right, rocking back and forth like a man waiting for some exam result.

How do I tell this boy? I thought, my chest tightening with profound sorrow. *How do I look into those pleading eyes and tell him that his mother is carrying a cancer inside her belly?*

I forced my voice to remain calm, adopting that gentle, reassuring cadence every physician must summon when walking into a storm.

"Amma, do not worry," I said, patting her hand. "I am going to give you an injection to take this pain away, and a small tablet. Lie here quietly and rest. Let the medicine do its work. Once the pain eases, you can go home, alright?"

She gave a faint, weary nod, too spent to speak.

I drew an analgesic medicine into a syringe, swabbed her arm, and administered the injection. I placed a sedative tablet on her tongue with a sip of water. Then, I signalled the young man to step out with me into the veranda.

"Listen to me, brother," I said softly, looking directly into his exhausted face. "I have given her an injection to stop the pain. Let her rest on the bed for a while. You take the motorcycle, go to the tiffin shop at the junction, and fetch some hot idlis for her. When she wakes up, she should have something light to eat. In the meantime, I will take a quick shower, change into proper clothes, and return. Then we will sit and discuss everything properly."

"Yes, Doctor. Thank you, Doctor," he stammered, his eyes brimming with gratitude as he rushed out.

Just as he left, Muthamma arrived at the gate, tucking the end of her saree into her waist.

"Muthamma, there is a patient resting in the examination room," I told her. "Keep an eye on her while I get ready."

"I'll take care of her, Ayya," she replied warmly, picking up her broom.

Back in my quarters, the cold shower did little to wash away the gloom that had settled over my spirit. The cold reality of rural healthcare was unforgiving. A massive tumor meant advanced blood tests, expensive scans, surgical oncologists, and tertiary cancer hospitals in Chennai—resources this young family likely could not afford. The tragedy of it cast a shadow over my entire morning.

By eight o'clock, I returned to the clinic dressed in formal attire, my stethoscope slung around my neck.

As I walked into my office to organize the table and all my medical equipments, the door leading to the examination corridor stood half-open. Through the frame, I could see directly into the exam room.

I stopped in my tracks.

The scene unfolding before me felt like a surreal hallucination.

The mother was sitting upright on the edge of the bed, a stainless-steel plate balanced on her lap, happily eating steaming white idlis. Beside her stood her son, leaning over with an aluminum tiffin box, carefully pouring a ladle of drumstick sambar over her plate.

The metamorphosis was astonishing.

Barely two hours ago, she had been a writhing, moaning spectre of pain, and he had been a trembling ghost of fear. Now, the young man looked completely transformed. He had clearly sped home, taken a bath, and changed into a fresh, crisp shirt. His hair was neatly parted and glistening with oil, the lines of anguish completely erased from his bright, youthful face. They were chatting and laughing softly, sharing bites of breakfast as though they were seated on the sunny portico of their own home.

A suffocating knot formed in my throat.

They have no idea, I thought, my heart aching with dread. *They think the storm has passed simply because the rain has paused. How am I going to shatter this fragile peace? How do I tell this boy that the mass inside his mother's body is a cancer and that the pain will soon return with a vengeance?*

Unable to face them at that moment, I busied myself with the morning inpatient ward rounds. I checked on post-natal mothers, dressed chronic wounds, and monitored fever charts, yet my mind remained trapped in the examination room. I swallowed a hasty breakfast of cold idlis in the staff room, my stomach churning with anxiety.

By ten o'clock, the morning outpatient clinic commenced. I walked back into my consultation room and summoned the mother and son.

They walked in side by side, their faces glowing with sheer relief. Seeing me enter, both of them stood up respectfully, hands folded.

“How are you feeling now, Amma? Is the stomach pain tolerable?” I asked gently, pulling out my chair.

The young man beamed, his white teeth flashing against his shiny dark skin.

“Doctor! She is completely fine now! In fact, she has been nagging me for the last half-hour that we should go home immediately. She says the pain has vanished without a trace, like it was never there!”

Every word he spoke felt like a tiny cut. I knew that the temporary relief was merely the antispasmodic masking the agony of the malignant mass. In a matter of minutes, I would have to drop the hammer of truth and destroy their fleeting happiness.

The profound distress must have bled through my practiced professional composure, for the young man suddenly stopped smiling. His eyes searched mine, narrowing with sudden worry.

“What is it, Doctor? Is something wrong?” he asked, his voice dropping an octave.

His devotion to his mother was so pure, so palpable, that I couldn’t bring myself to blurt out a devastating clinical suspicion without confirming my examination one more time. I needed to touch that mass again. I needed to verify its margins, its mobility, and its consistency before handing them a referral letter that would change their lives forever.

“No, no, brother,” I said smoothly, masking my agitation. “I just want to ensure that everything has stabilized. Let me examine her abdomen one more time on the bed, and then we will talk.”

I stepped to the doorway and called out down the corridor. “Sister Shanthi! Are you there?”

Staff Nurse Shanthi came scurrying down the hallway, adjusting her starched white saree and pinning back stray strands of hair.

“Sister Shanthi, you are late again today,” I remarked with mild reproach. “Come quickly, I need to palpate this patient’s abdomen again. Please prepare her.”

“Forgive me, Doctor, the morning bus from the highway broke down near the canal,” Shanthi muttered defensively as she bustled around the bed. She helped

the mother lie down, gently pulled the saree down past the pelvic brim, and had her flex her knees to relax the abdominal wall.

"All ready, Doctor," Shanthi said, standing by the head of the bed. "What is the provisional diagnosis, Sir?"

"There was a distinct, hard mass in the left lower quadrant during the early morning exam," I replied in low, clinical English, deliberately choosing terms the mother and son would not understand. "I strongly suspect an advanced neoplasia."

I didn't want the word cancer echoing in the room, terrified of triggering a wave of panic before I was certain of my ground.

I stood over the patient. I took a slow breath, rubbed my hands together once more, and placed my right palm firmly over the left lumbar region, pressing down into the left iliac fossa where the giant mass had resided.

I pressed.

Softness.

I paused, blinking. I adjusted my angle, sinking my fingers deep into the retroperitoneal space.

Nothing.

A sudden jolt of disbelief surged through me. I applied bimanual palpation, placing my left hand under her flank to lift the kidney forward while my right hand pushed deeply beneath the costal margin.

There was only supple, healthy, relaxed muscle tissue.

My heart began to race. I shifted my hands across the midline, furiously palpating the other areas, the right upper and lower quadrants.

The mass was gone. Completely, entirely vanished.

A bead of cold sweat rolled down my temple. *What is this sorcery?* I screamed in my head. Just four hours ago, a rock-hard, unyielding rock the size of a giant mango had practically screamed its presence beneath my fingers. It had been impossible to miss. It had possessed sharp, distinct borders, a dense, stone-like texture, and indisputable anatomical permanence.

Now, there was nothing. Not a ridge. Not a nodule. Not even a trace of induration.

A dizzying whirlwind of self-doubt and confusion battered my intellect. *Did I imagine it? Was I half-asleep during the six o'clock examination? Could the hyper-toned, hardened abdominal muscles of a lifetime of manual agricultural labor have felt like a calcified tumor in my groggy state?*

No! my rational mind screamed back. *I am a trained physician! I know the difference between muscular spasm and a discrete, mass the size of a fruit!*

Besides, if it were merely muscle, it would still be here right now. How could a physical object of that magnitude simply evaporate into thin air within a hundred and eighty minutes? It defied every law of pathology, every chapter of anatomy, every clinical certainty I had spent years studying.

I stood up, wiping my brow, staring at the woman's flat, peaceful belly.

"Sister Shanthi," I managed to say, my voice sounding distant even to myself. "Ask the patient and her son to come into my consultation room immediately. I need to speak with them."

I walked back into my office and sank into the revolving chair, staring blankly at the desk. Fortunately, the morning queue had temporarily thinned, and no other patients were waiting outside.

The wooden door creaked open. The young man and his mother walked in and took their seats across from me.

"Tell me, brother... what is your name again?" I asked, leaning forward on my elbows.

"Murugan, Doctor. Murugan," he replied, smiling warmly.

"Murugan, tell me very carefully," I said, looking from the young man to his mother. "After I gave her the injection and tablet this morning and left for my house... what exactly happened in that room?"

"Nothing out of the ordinary, Doctor," Murugan answered, scratching his head. "When you left, Amma was still twisting and groaning with that terrible pain. But after maybe fifteen or twenty minutes, the groaning stopped. She became completely still, as if she had fallen into a deep sleep. I got scared, Doctor. I thought she had lost consciousness, so I went to the bed and gently shook her shoulder to wake her up."

Murugan turned to his mother. “Go on, Amma. Tell the Doctor what you told me.”

The elderly woman smiled, her eyes crinkling at the corners. She folded her hands over her lap and spoke in a quiet, matter-of-fact tone.

“Doctor Ayya... I was lying there, burning with that agony inside my left side. Then, all of a sudden, deep inside my belly, I heard and felt a loud *’PATT’* sound! Like something bursting wide open! Like a tight bubble bursting!”

I stared at her, mesmerized. “A pop? And then?”

“The very second that pop happened, Ayya, the pain disappeared,” she said, raising her palms upward in wonder. “It didn’t just fade—it vanished completely, like a flame blown out by the wind! And then, barely a minute later, a tremendous urge to pass urine struck me. It was so heavy, so urgent, I could barely hold it in! I rushed to the bathroom behind the ward and passed water. It came out in torrents, Doctor—bucketfuls of it, like a dam breaking open! After that, every ounce of heaviness left my body. My stomach felt as light and empty as an empty pot. That’s all, Doctor. Everything became completely peaceful.”

I sat frozen in my chair, utterly dumbfounded.

A mass that snaps with a pop? A tumor that dissolves into gallons of clear urine in twenty minutes?

In all the medical treatises I had ever devoured, in all the grand rounds of my under and postgraduate training, no pathology behaved like this. This sounded less like clinical medicine and more like the mystical folklore of wandering village bards. It was scientifically impossible for a solid tissue mass to liquidate and discharge through the bladder in a flash of lightning.

Yet, the empirical reality was undeniable: the mass was gone, and the patient was cured.

“Murugan,” I said, reaching for my prescription pad and scribbling a referral note with an unsteady hand. “Listen to me carefully. I want you to take your mother to the Government District Headquarters Hospital tomorrow morning. Take this letter directly to the radiology department and get a full abdominal and pelvic ultrasound scan done. Do not skip this. Bring the reports back to me immediately.”

“I will do it without fail, Doctor,” Murugan said, taking the envelope with deep reverence.

Throughout the rest of the day, as I examined feverish children, dressed farm injuries, and titrated hypertension medications, the mysterious phantom mass refused to leave my mind. It haunted me through the afternoon heat and followed me into the night.

Three days later, the clinic door opened, and Murugan stepped inside. He was alone.

“Doctor, I went to the district hospital just as you ordered,” he said proudly, handing me a large, glossy radiography envelope. “They performed the scan on Amma.”

I tore the envelope open, pulled out the printed ultrasound images, and snapped the typed radiological report open against the light.

Liver: Normal in size and echotexture.

Gallbladder and Pancreas: Unremarkable.

Right Kidney: Normal.

Left Kidney: Normal cortical thickness, no focal lesion, no pelvicalyceal dilatation.

Urinary Bladder: Normal.

Impression: Normal study. No evidence of space-occupying lesion or retroperitoneal mass.

I stared at the pristine black-and-white printouts. The radiologist had found absolutely nothing.

Was it a waking mirage? Had I touched a phantom conjured by the dawn mist? Or had divine grace reached down into that rustic examination room and performed an inexplicable miracle under my very eyes? I had no answers. The clinical record was closed, but the mystery remained etched into my soul.

Months turned into years.

Eventually, my tenure at the rural health center came to an end. I packed my bags and moved on to enroll in a prestigious university to pursue my doctoral research (PhD) in public health and epidemiological sciences.

The fast-paced rhythm of academic life took over. I spent long, quiet hours buried under towering stacks of journals in the university library, surrounded by the musty smell of old paper.

One rainy afternoon, while scouring an international oncology journal to read a paper on doctor-patient relationships in oncology, I stumbled upon a rare case report describing the phenomenon of *Spontaneous Tumor Regression*—the sudden, medically unexplainable disappearance of malignant neoplasms. The authors hypothesized that hyper-activated immune cells, under extreme biological triggers, could occasionally launch a massive cytokine cascade and dismantle malignant tissue.

For a brief, electric moment, my mind flashed back to the village clinic, to the cool stone veranda and the woman with the vanishing tumour.

Could that have been what happened to her?

I read the paper deeper, only to discard the theory with a sigh. Spontaneous tumor regression is an agonizingly slow biological process that unfolds over weeks, months, or years as the immune system systematically starves the neoplastic tissue. It was physiologically impossible for an established tumor to achieve complete spontaneous regression within three hours. There was zero precedent for it in the history of medicine.

And yet, that lady's simple, raw description continued to peck at the back of my mind like an insatiable bird:

"A loud pop inside the belly... like a bubble bursting... followed by bucketfuls of urine."

I searched digital databases, indexed textbooks, and urological archives, yet the riddle yielded no clues.

Then came the breakthrough—not in the gilded halls of a clinical class, nor within the high-tech confines of a radiology console, but in an ordinary village living room.

As part of my doctoral field research exploring the social determinants of community trust in primary physicians, I traveled to a small town called Sholavaram, situated on the outskirts of Chennai. I was sitting on a woven coir mat inside a modest village home, conducting a focus-group interview with several local elders.

During the conversation, an elderly woman began narrating her personal medical history.

"Ayya, three years ago, I suffered from an unbearable pain in my flank," she explained, gesturing to her left side. "The local doctor said it was a kidney stone, stuck right where the urinary tube leaves the kidney. The pain was so agonizing that my whole side swelled up tight, until the stone finally passed into my bladder, and all that trapped water came rushing out."

In that single, unvarnished sentence, lightning struck my mind.

The entire puzzle crystallized with breathtaking, dazzling clarity. A heavy cobweb that had clouded my understanding for years was instantly swept away!

The Pelvi-Ureteric Junction!

It had never been a tumor. It had never been a cancer.

Murugan's mother had developed a tiny, sharp renal calculus that drifted down the kidney and became wedged precisely at the pelvi-ureteric junction (PUJ)—the narrow funnel where the renal pelvis tapers into the ureter.

The calculus had formed a perfect, airtight ball-valve obstruction. The kidney, continuously producing urine under relentless physiological pressure, had nowhere to drain its output. The renal pelvis began to distend, ballooning larger and larger under immense back-pressure. Over hours of excruciating agony, the kidney swelled into a colossal, tense, water-filled sac.

By the time she arrived at my clinic door at dawn, that water-filled sac was stretched so tight under hydraulic pressure that its walls became rigid, smooth, and as hard as granite to the touch—mimicking a rock-solid, mango-sized retroperitoneal neoplasm with terrifying precision.

And then, the magic of simple pharmacology took over.

When I administered the painkiller injection, the smooth muscles of her ureter and renal pelvis finally relaxed. The muscular spasm that had been gripping the tiny calculus suddenly unlocked. The stone dislodged and dropped freely into the wide ureter with a physical *'pop'* that reverberated through her retroperitoneum!

With the obstruction blown open, the massive reservoir of high-pressure urine that had been trapped inside the ballooned kidney rushed down into her bladder like an uncorked bottle, emptying out in bucketfuls and causing the phantom "tumor" to collapse completely, disappearing forever!

By the time I re-examined her at ten in the morning, the kidney had decompressed back to its normal, soft, impalpable dimensions. And by the time the radiologist scanned her three days later, the microscopic stone had long been washed out into the soil, leaving behind pristine, unblemished renal parenchyma.

A profound sense of awe washed over me as I sat there in Sholavaram.

Whether this mechanical miracle was the exact, absolute truth of what transpired on that misty morning, I can never say with one hundred percent certainty. The opportunity to clinically verify that fleeting moment in time has long passed into the annals of memory.

Yet, that encounter bestowed upon me the most transformative lesson of my entire medical career: true healing demands that a physician keep both intellect and humility wide open. The human body is a miraculous, endlessly mysterious vessel. For every symptom, there may exist a straightforward textbook explanation, but there are times when the living organism dances through pathways of wonder and biomechanical marvels that no standard algorithm could ever anticipate.

The greatest healers are not those who claim to possess all the answers, but those who stand before the marvels of life with an open mind, an empathetic heart, and an enduring sense of reverence.

Super sensory perception

Medicine was not merely a career choice for Sundar; it was an inheritance carried in his bloodstream. His father was an eminent nephrologist whose clinical intuition was revered across the state, a man who could detect the onset of renal failure with his strong sense of intuition. His mother was a distinguished psychiatrist whose private clinic in Chennai quietly tended to the unravelling minds of politicians, matinee idols, and literary figures. His elder sister had already flown across the ocean, pursuing advanced pediatric fellowship training in London. From his earliest boyhood, there was never any ambiguity about his trajectory. He studied with a methodical, unrelenting focus, cleared his competitive entrance examinations with distinction, collected his undergraduate medical degree, and, within six months, secured a fiercely contested postgraduate residency seat in Emergency Medicine at the state's premier tertiary teaching hospital.

A week before his official induction, Sundar had walked through the casualty corridors to seek orientation from his seniors. That was where he met Tamilarasan, a third-year resident whose eyes were permanently rimmed with sleepless shadows, though his grin remained disarmingly warm.

"Welcome to the madhouse, brother," Tamilarasan had chuckled, clapping him on the shoulder. "The Emergency Department never breathes. Your first year will strip you raw. Shifts run from seven in the morning until eleven at night, if you're lucky. You get one hour of break in total, but only after every single senior has finished theirs. Don't dream of heroic emergency thoracotomies or central lines right away; those are luxuries reserved for destiny and third-years. Your world will be nasogastric tubes, Foley catheters, arterial blood gases, endless urine dipsticks, drawing twenty vials of blood every hour, and writing relentless clinical notes. Welcome to hell, my friend. It's been eagerly waiting for you."

The memory echoed in Sundar's mind as dawn broke on his very first official day of duty. A knot of nervous dread tightened in his stomach. To soothe the flutter in his chest, he had stopped by the neighborhood temple at daybreak, offering a quiet prayer for competence and favor, letting the priest press a cooling smear of sacred vibhooti across his forehead. He checked his wristwatch as he crossed the hospital gates: precisely seven hours and two minutes.

Standing monolithic at the casualty threshold was Professor Fernandes, the head of the department, hands clasped behind his back, his gaze scanning every entering face like an imperial sentry. The moment his sharp eyes landed on

Sundar, his brow furrowed into a terrifying scowl.

Sundar felt his knees weaken. He slipped into the bay, instinctually trying to conceal himself behind the broad shoulders of Tamilarasan.

Fernandes snapped his fingers, the sound cutting through the ambient hum of suction machines and cardiac monitors. "You. Fresh blood. Step forward."

Sundar stepped out, the air caught in his throat.

"Go to the washbasin right now and scrub that forehead clean," Fernandes growled, his voice deep and gravelly. "Your religion, your deities, and your household rituals stay outside those double doors. Inside this trauma center, your only identity is that of a physician. We do not brandish sacred symbols when life and death are hanging on the monitors. And another thing: tomorrow morning, you are at this desk before my boots hit the floor. Rounds start at seven sharp. Not seven-oh-two. Do you hear me?"

"Yes, Professor," Sundar whispered, rushing to the sink.

As the cold tap water washed away the vibhooti, an unexpected sensation bloomed within him. It was not humiliation, but an electric surge of purpose. To be reprimanded with such clinical ferocity felt like an initiation into an ancient, uncompromising brotherhood. Drying his face, he picked up his clipboard and stepped into the trauma rounds with his pulse racing with fierce, newfound energy.

Casualty bays possessed an unpredictable rhythm. Early mornings could occasionally trick young doctors with eerie quietude before sudden catastrophe shattered the calm. By mid-morning, the ward had a brief lull. Tamilarasan had pulled up a rolling stool beside Sundar, patiently tracing the jagged complexes of an electrocardiogram strip.

"Look closely at the ST segments in Lead II, III, and aVF, Sundar," Tamilarasan murmured, tapping the paper with the tip of a ballpoint pen. "When you see elevation here, you aren't just reading ink; you are watching an inferior myocardial wall suffocating in real time."

Before Sundar could respond, the swinging doors of the casualty burst open with a resounding crash. A chaotic wave of people surged into the resuscitation bay, breaking the quiet with frantic shouts and wailing.

At the center of the commotion was a wheelchair pushed rapidly by a heavily built, muscular man whose eyes were wide with panic. Seated in the chair was a middle-aged woman draped in an unstitched length of bright yellow cotton. Her hair was extraordinary—an immense, heavy crown of matted, dreadlocked tresses coiled upon itself, thick with dust, camphor, and time. Her entire face was coated in a deep layer of golden turmeric, and between her brows rested a crimson vermilion dot the size of a five rupee coin. Her eyes were closed, her chin slack, suspended in what looked like an impenetrable trance. Behind her surged a retinue of fifteen village devotees, weeping, praying aloud, and clasping their hands in frantic supplication.

Tamilarasan instantly abandoned the ECG on the desk. "Resuscitation Bay Two! Move!" he barked, gesturing to Staff Nurse Bincymol.

The muscular man gently grasped the woman's shoulders as they transferred her onto the examination gurney. "Thayee! Mother! Open your eyes, we have reached the big hospital! Look at me, Thayee!" he pleaded, his voice breaking.

The woman's eyelids fluttered open, but her gaze was vacant, staring straight through the fluorescent ceiling lights into an unseen void. She showed no awareness of her surroundings.

"What happened to her?" Tamilarasan demanded, pulling his penlight from his coat pocket.

"Doctor," the muscular man stammered, trembling. "Our Thayee collapsed an hour ago during the sanctum puja in the temple courtyard. She falls into divine trances often when the Mother Goddess Aadhiparasakthi descends upon her. In those states, whatever prophecy she speaks comes true to the very letter. Usually, after the trance, she wakes within minutes, laughing and blessing everyone. But today... she fell backwards on the granite stones. She hasn't uttered a single syllable since. She won't recognize our voices. We were terrified and brought her straight here."

Tamilarasan immediately took control. "Sundar, manual blood pressure and pulse immediately. Sister Bincymol, glucometer check, stat."

Tamilarasan clicked the penlight, shining the beam into the woman's eyes. "Pupils are three millimeters, equal and briskly reactive to light. No obvious signs of brainstem herniation."

“Blood pressure is one-twenty over eighty, pulse seventy-two and regular,” Sundar reported, checking the cuff.

“Random blood sugar is one-ten milligrams per deciliter, normal,” Bincymol chimed in, discarding the lancet.

“Let’s get her positioned flat and examine the spine,” Tamilarasan instructed.

Sundar reached down, sliding his hands gently beneath the woman’s head to support her occiput as they lowered the backrest. The moment his fingers sank into the dense, matted hair at the base of her skull, he felt a warm, sticky thickness. He pulled his hands back. His latex gloves were drenched in dark, glistening crimson.

“Tamil Sir!” Sundar gasped, holding up his hands. “Blood! Active hemorrhage from the scalp!”

Tamilarasan lunged forward, his fingers parting the thick, heavy locks with urgency. Beneath the dense coil of hair lay a gaping, jagged laceration across the posterior scalp, dark venous blood welling out and trickling down her neck.

“Sister, get a suture tray, local anesthetic, and four-zero Ethilon ready!” Tamilarasan ordered. He glanced at Sundar. “Have you sutured scalp wounds before?”

Sundar hesitated for a split second, feeling the immense weight of the moment. “Under supervision, yes, Sir. I can do it.”

“Good. Sister, assist him. He’s our new first-year. Let’s close the bleeder fast, and then rush her to the CT suite to look for intracranial injury.”

Bincymol summoned the casualty attendant, an older orderly carrying a disposable shaving razor and sterile prep solution. “Hold her head steady,” the orderly muttered, stepping forward to shave the thick hair around the wound margin to allow clean sterilization and suturing.

The moment the steel razor flashed under the lights, the muscular devotee let out an enraged roar. He lunged forward, violently shoving the orderly away.

“Get that blade away from her!” the man shouted, his chest heaving, veins bulging along his neck. “Are you out of your minds? No knife or razor will touch our Mother’s head!”

The orderly stumbled against a crash cart, his face turning red with fury. “Who do you think you’re pushing? We are trying to save her life!”

Within seconds, the fifteen devotees swarmed the gurney, shouting, waving their arms, and creating an explosive uproar in the trauma bay. Sundar, standing with sterile gloves already on, froze in disbelief, his hands raised in the air.

“Tamil Sir!” Sundar shouted over the deafening noise.

At the far end of the bay, Tamilarasan was in the middle of delicately extracting a foreign body—a raw peanut—from the nostril of a screaming toddler. Hearing the commotion, he handed the forceps to an intern, yelled “Security! Casualty main bay now!” and sprinted back to Sundar’s side.

Two hospital security guards rushed in, placing themselves between the furious crowd and the medical team.

“Listen to me!” Tamilarasan yelled, stepping directly into the muscular man’s space. “What is wrong with you? This is a sterile resuscitation area!”

“Doctor, you do not understand!” the devotee pleaded, his anger collapsing into frantic desperation. “This matted crown belongs entirely to the Goddess! For six continuous years, our Thayee has fasted, prayed, and walked on embers to grow these sacred locks. Next month is the grand festival where she is ordained to shave her head as a holy offering at the sanctum. If a razor touches her scalp before that ritual, the wrath of the divine will destroy her and our village! We will never allow a blade on her head!”

“Are you insane?” Tamilarasan fired back, his voice cutting through the panic. “Your Thayee has a deep cut on the back of her head. If we don’t clean and suture the wound, she will bleed out, or develop a dangerous infection. Her life is in severe danger!”

“Do whatever stitching you want,” the man insisted stubbornly, folding his palms together. “Tie it, sew it, bandage it. But no razor touches her sacred hair. Not a single strand will be cut.”

Tamilarasan turned to Sundar, exasperated. “Sundar, can we suture the wound without shaving the margins?”

Sundar leaned over, parting the heavy, dusty dreadlocks with gloved fingers. The smell of old turmeric, sweat, hardened blood, and unwashed hair assaulted

his nostrils. “Sir, it’s impossible. The hair is heavily matted with grime and dirt. If I try to pass a needle through this without cleaning and shaving, we will drag massive contamination straight into the subgaleal space. It will form a massive abscess in no time.”

“Yes, Doctor, it is extremely contaminated,” Sister Bincymol affirmed, turning her head away in disgust.

Tamilarasan grimaced, stepped forward, and inspected the depth of the injury himself. The laceration was nearly ten centimeters long, deep enough to expose the pericranium, weeping blood continuously beneath the dense matted thicket. Realizing that further argument over the razor was futile while she remained in an altered mental state, he grabbed a stack of sterile gauze pads and a crepe bandage.

“We don’t have time for a theology debate right now,” Tamilarasan said curtly. “Hold direct pressure. Sundar, help me apply a tight pressure wrap to control the external bleeding. We are taking her for an immediate plain CT brain.”

Together, they wrapped the woman’s head in a thick, compressive bandage, unlocked the gurney wheels, and pushed her rapidly down the corridor toward the radiology suite, her entourage jogging behind until security halted them at the double doors.

Sundar’s temper flared as they sprinted alongside the rolling bed. “Sir, how can you surrender to such absolute backwardness and stupidity? How can these people be so stubbornly blind? What matters more—a human life or a bundle of filthy matted hair? It makes my blood boil!”

“Sundar, watch your language and lower your voice,” Tamilarasan commanded sharply, not breaking his stride. “This is a hospital, not a debate hall. You are a physician; you do not get the luxury of emotional outbursts or judging the cultural baggage of your patients. If you cannot maintain clinical composure under stress, go back to the duty room.”

Sundar clamped his jaw shut, swallowing his frustration as they wheeled the patient onto the CT scanner table.

Within five minutes, the radiological cross-sections appeared on the high-resolution diagnostic monitors in the control booth. Tamilarasan leaned forward, clicking the trackball. Sundar stared over his senior’s shoulder, his irritation instantly freezing into cold dread.

A massive, hyperdense, biconvex lens-shaped white lesion dominated the left temporoparietal region of the brain. The skull fracture beneath the matted hair had torn the blood vessel inside the head. The rapidly expanding hematoma was exerting tremendous mass effect, squashing the left lateral ventricle and shoving the delicate midline structures of the brain far over to the right side.

"Acute extradural hematoma with significant midline shift," Tamilarasan whispered, his voice tense. "The pressure is skyrocketing. If a neurosurgeon doesn't perform an emergency surgery and evacuate that clot within the hour, brain herniation is imminent. She will develop cardio-respiratory arrest and die."

"Sir," Sundar said urgently, his hands clenching into fists. "These people refused to even let us shave an inch of hair for a simple stitch. How are they going to permit a neurosurgeon to shave half her head and saw open her skull? Let's just wheel her straight to the emergency theater and intubate her. We don't even need to ask them; they don't understand science anyway!"

"Quiet, Sundar!" Tamilarasan warned, fixing him with a stern stare. "You cannot touch a patient without informed consent from the legal guardians, no matter how severe the pathology. That is the law, and that is medical ethics. We do not operate in secret. Let's call Professor Fernandes right now."

Tamilarasan dialled the department chief on the internal intercom. "Professor, we have a critical trauma case in the CT suite. Middle-aged female, massive acute left extradural hematoma with midline shift. Needs urgent craniotomy. But the attendants are religious fanatics and refuse to even shave her hair. I don't think they would allow brain surgery. What should we do?"

"Do not let them leave," Fernandes's voice boomed through the receiver. "Bring the patient back to Bay One. I am coming down right now."

They wheeled the unconscious woman back to the resuscitation area. Fernandes strode in moments later, his white coat billowing behind him. He gathered the muscular guardian and four elder relatives, ushering them into his private consultation room alongside Tamilarasan and Bincymol to begin a high-stakes counseling session.

Sundar was assigned to stay at the bedside, monitoring her vitals, pupil size, and neurological decline every five minutes.

Standing alone beside the gurney, Sundar gazed down at the silent woman. The sterile trauma bay, with its beeping monitors, stainless steel trays, and harsh white illumination, felt violently at odds with her turmeric-stained face and ancient, mystic aura. A deep, bitter ache rose in Sundar's chest. Throughout his grueling years of medical school, textbooks had taught him anatomy, cellular biochemistry, pharmacology, and surgical techniques. They had never prepared him for the insurmountable wall of human irrationality.

Here lay a woman whose life could be effortlessly saved. A state-of-the-art neurosurgical team was one elevator ride away. The government provided all surgical consumables and post-operative ICU care entirely free of charge. World-class surgeons stood ready to relieve the pressure in her skull and return her safely to her family. Yet, she was sliding toward brain death because of six years of matted hair and unyielding dogma. How can someone throw their life away in the modern era for such blind superstition? Sundar thought, his heart aching with heavy, helpless sorrow.

Suddenly, a violent spasm shook the gurney.

Sundar gasped, stepping back as the woman's eyes snapped wide open. They were no longer vacant; they burned with an intense, piercing clarity. With lightning speed and terrifying strength, her rough, calloused hand shot out, locking onto Sundar's right wrist with a grip like a vise.

Sundar's heart hammered against his ribs. A cold shiver tore down his spine. "Sister! Sir!" he tried to call out, but the air caught in his throat.

The woman fixed her fiery, unblinking gaze directly into Sundar's eyes. When she spoke, her voice was not the weak whisper of a dying trauma patient; it was a low, resonant, otherworldly rasp that seemed to vibrate straight into his bones.

"Listen to me, young man," she uttered, each syllable drenched in uncanny authority. "You walk through this world thinking your path was built by your own cleverness, that you rose to this high seat without suffering a single scratch. But you know nothing. You stand here alive and whole only because of the old man. Your grandfather. It was his ceaseless worship, his prayers, his boundless merit, and his ultimate sacrifice that bought your breath. Every drop of privilege you enjoy today was paid for with his blood. His memorial day arrives next month. Do not forget him, boy. Feed the hungry and honor his soul on that day."

As abruptly as she had seized him, her grip went limp. Her head fell sideways onto the pillow, her eyes rolling back into deep unconsciousness.

Sundar stood frozen, trembling violently from head to toe. The room seemed to lose its gravity. His skin prickled with goosebumps so severe that his arms ached. How could this woman an illiterate devotee from a remote rural temple, brought to the city in an unconscious state know anything about his lineage? How did she know about his grandfather? How could she know the precise month of his grandfather's death anniversary?

Before Sundar could gather his scattered thoughts, the door to the consultation room swung open. The heavy-set man strode out, flanked by the weeping devotees. His jaw was set like iron.

"We are taking our Mother home," the man declared fiercely. "The doctors say they must cut her head open and desecrate her vows to save her body. But our Mother would rather surrender her mortal breath than break her covenant with the Goddess. Prepare the transport."

"Wait! No! You can't!" Sundar cried out, rushing forward to block the gurney. "She will die! She has a massive bleed in her brain! Please, listen to me, you are killing her!"

Tamilarasan caught Sundar firmly by the arms, holding him back. "Stand down, Sundar. Let them go."

"Tamil Sir, are you crazy? Look at the CT scan!" Sundar pleaded, his voice breaking into a desperate crack.

"We sat with them for thirty minutes, Sundar," Tamilarasan said softly, his voice heavy with defeat. "Professor Fernandes explained every single surgical risk, every consequence, and the certainty of death without an operation. They understand the outcome, but they refuse. They have signed the legal Left Against Medical Advice forms. Legally and ethically, we cannot hold a patient by force. Let them go."

Sundar watched in stunned, heartbroken silence as the family gently lifted the woman back into the wheelchair and pushed her out into the bright, indifferent afternoon sun.

For the remainder of the shift, Sundar functioned like a ghost. He drew blood, charted vitals, and placed catheters with mechanical detachment, but his mind

was trapped in the piercing gaze of the woman's eyes and the chilling resonance of her words.

At seven in the evening, Tamilarasan approached him, placing a gentle hand on his shoulder. "Sundar, go to the canteen. Grab a hot cup of tea and sit outside for half an hour. You look completely shattered. Go."

Sundar walked out into the hospital courtyard, the humid night air washing over him. With trembling fingers, he pulled out his mobile phone and dialed his father.

"Sundar?" his father's calm, cultured voice answered on the third ring. "How was your first official day in the casualty ward?"

"Appa..." Sundar choked out, unable to hold back the tremor in his voice. "Something terrifying happened today."

"What is it, son? Did a patient pass away?"

"Yes... no... I don't know," Sundar stammered, leaning against a stone pillar. "A middle aged woman was brought in with a serious head trauma. Her family refused surgery and took her away to die because of their superstitious beliefs. But Appa... just before they took her, she woke from a deep coma, grabbed my wrist, and looked into my soul. She spoke about our family. She spoke about Thatha."

A sudden, absolute silence fell over the other end of the line.

"Appa? Are you there?" Sundar asked, unsettled by the quiet.

"What did she say, Sundar?" his father asked, his voice suddenly stripped of its usual clinical coolness.

"She said I achieved everything in life without knowing suffering because of Thatha's sacrifice," Sundar whispered. "She said he sacrificed himself for me, and she told me to perform charity on his memorial day next month. Appa, why would a total stranger say that? What sacrifice did Thatha make? Tell me."

His father took a slow, shuddering breath. "Sundar... there are truths we chose to bury when you were a boy. We wanted to protect your childhood. Take leave this Sunday and come home. This is not something I can tell you over a telephone."

The following Sunday, Sundar sat across from his father in the quiet study of their ancestral home. The room was lined with leather-bound nephrology textbooks and silver academic trophies, but the atmosphere felt hushed and solemn. A warm brass tumbler of filter coffee sat untouched on the wooden table between them.

His father rested his elbows on the desk, his fingers interlaced, his eyes reflecting deep, painful memories.

"You were four years old, Sundar," his father began, his voice dipping into a low, vulnerable cadence. "You were our bright, joyful little boy. Then, one week, you developed persistent low-grade fevers, unexplained bruising along your shins, and profound fatigue. We ran routine blood tests, suspecting a viral infection. The peripheral smear broke our hearts into pieces. It was Acute Lymphoblastic Leukemia. High-risk, aggressive."

Sundar's breath hitched. He touched his chest instinctively, staring at his father in disbelief. "Leukemia? I... I had blood cancer?"

"We exhausted every frontline chemotherapy protocol available in India," his father continued, a tear slipping down his cheek. "For nearly a year, your mother and I lived in a waking nightmare. You were wasting away. The oncologists told us chemotherapy had failed; your only chance of survival was an immediate, fully matched allogeneic bone marrow transplant. In those days, donor registries in India were virtually nonexistent. We tested your mother, your sister, and myself. None of us were a match. The HLA compatibility was hopeless."

His father paused, wiping his eyes with a handkerchief.

"The only person in our entire extended lineage whose human leukocyte antigens matched yours with absolute perfection was your grandfather. He was seventy-one years old at the time. But Thatha had severe ischemic heart disease; he had already survived a massive myocardial infarction two years prior. His cardiac ejection fraction was barely thirty percent. The oncology team refused to harvest his marrow. Bone marrow aspiration from the iliac crest in those days required prolonged, deep general anesthesia. The anesthesiologists were blunt: Thatha's damaged heart would never tolerate the hemodynamic stress of general anesthesia. They told him it was suicide."

Sundar listened, his entire world spinning on its axis.

"Your grandfather didn't care about their warnings," his father whispered, his voice breaking into tears. "He told the doctors, 'My life is already spent. My grandson's life has barely begun. Put me under and take whatever marrow the boy needs to live.' He signed the high-risk waivers himself. They wheeled him into the harvest theater, harvested the marrow, and infused it into your veins. The transplant took, Sundar. Your marrow regenerated; the leukemia was completely eradicated."

His father looked up, his eyes bloodshot and full of grief. "Thatha never woke up from the anesthetic recovery room. His heart went into refractory ventricular fibrillation just as they finished the procedure. He laid down his life so that your blood could be renewed. The very blood cells flowing through your veins today, the immune system that keeps you alive, the bone marrow inside your skeleton—it is your grandfather's living marrow. That woman was right, Sundar. You exist because of an old man's ultimate sacrifice."

Sundar sat motionless as tears streamed silently down his face. His hand rose unconsciously to his neck, feeling the rhythmic, vital pulse of the carotid artery. It was not his blood alone; it was an altar of ancestral devotion beating within his chest.

The following month, on the precise date of his grandfather's memorial anniversary, Sundar traveled to their ancestral village. He stood in the quiet, sunlit central courtyard of the old family house, looking up at the framed black-and-white portrait of his grandfather adorned with a fresh garland of white jasmine.

Sundar fell to his knees, pressing his forehead against the cool stone floor. He placed his hands over his heart, whispering a solemn vow into the silent room: every breath he took, every patient he examined, and every drop of skill he acquired as a physician would be offered as a living tribute to the man who died so that he might heal others. All this was too overwhelming to process that Sundar never asked the question, "How did the woman know about thatha?"

Six years passed in a blur of relentless discipline and growth.

Sundar completed his residency at the top of his class and rose to become a brilliant, deeply respected consultant in Emergency Medicine. He became known across the city not only for his razor-sharp diagnostic precision and cool head in mass casualties, but for an extraordinary, almost sacred empathy toward his patients. Whenever a destitute, frightened villager was wheeled through the

casualty doors, Sundar saw in them the faces of those who had carried him to his own life.

One evening, Dr. Sundar sat in the grand auditorium of an international medical conference in the city. The plenary session was dedicated to neurobiology, transplant epigenetics, and anomalous clinical phenomena.

At the podium stood an eminent neuroscientist and researcher who was presenting a lecture on organ transplant psychology and cellular memory.

"Consider this documented case from our multi-center study," the lecturer explained, projecting clinical data onto the vast projector screens. "A female patient with terminal renal failure received a deceased-donor kidney transplant. Following her recovery, she began experiencing persistent, vivid dreams about an unfamiliar countryside house, a blue bicycle, and an intense emotional bond with a golden retriever. Months later, during a chance visit to the donor's hometown, the donor's pet dog ran straight across a crowded marketplace and buried its head in the recipient's lap, weeping with recognition. How do we explain this?"

The auditorium hummed with quiet intrigue.

"Emerging research in bio-electromagnetism and cellular memory suggests that somatic cells, particularly highly regenerative tissue like bone marrow stem cells and specialized organs, store subtle epigenetic and energetic imprints of the donor's consciousness," the speaker continued. "Furthermore, when individuals experience intense neurological events—such as acute temporoparietal trauma, midline brain displacement, or temporal lobe epilepsy—the brain's normal sensory gating mechanisms collapse. In this altered, heightened state of neuro-electrical arousal, the mind can access an ultra-conscious state—a higher realm of perception. In this state, a subject can perceive subtle biometric, genetic, and cellular resonance that is completely invisible to everyday human consciousness."

Sitting in the dim auditorium, Sundar felt a profound, electrifying shock of realization course through every nerve in his body.

The grand puzzle of his life clicked together with breathtaking clarity.

The woman in the yellow saree was not a fraud, nor was she merely an unexplainable miracle. When her skull struck the granite temple stones, the traumatic midline compression and temporal lobe irritation had blown open the doors of her perception. In that brief, incandescent burst of a higher state

of consciousness, her mind had tapped into the bio-energetic signature of his body. She did not see a stranger in a white coat; she had felt the living, vibrating presence of his grandfather's marrow pulsing within his blood.

Sundar slowly raised his right hand and pressed his fingers against his wrist. He closed his eyes, feeling the steady, rhythmic beat of his pulse under his skin.

Science and spirit, intellect and mystery, clinical rigor and devotion were not enemies after all. They were merely different windows looking into the same vast, infinite architecture of human connection. He sat in the quiet auditorium, feeling the warmth of his grandfather's eternal gift flowing through his veins, smiling with deep, unshakable peace.

Spiritual Guidance

About 15 years ago when I first joined the rural clinic where I continue to work today, the area was poorly connected by public transport facilities. There was a private bus facility that used to ply from Chengalpet to the village at specific timings. If we missed that bus, we had to take a share autorickshaw. A share autorickshaw is an interesting contraption. Ideally it is supposed to seat about 5 persons in two tiers, maximum of 6 if the persons are thin built and squeeze in with each other. But on busy Monday mornings, when I used to come from Chennai to the clinic, it was not uncommon to find roughly 14-15 people in that share auto. If you miss that share auto, you cannot reach that village and start the clinic on time. On many days I have travelled sitting, sharing the driver's seat with two persons standing on my sides holding the top of the autorickshaw. Speed breakers were hell, because the entire share auto will shake and all the 14 of us precariously dangling in the auto will let out loud exclamations of discomfort and pain! Over time, I discovered a schedule by which I could avoid taking the crowded share autorickshaw. Slowly life started becoming easier. However, the long journey of 2 hours and 15 mins to the clinic in the morning and another 2 hours in the evening, was mentally challenging sometimes, and physically exhausting at others. This is the story of Velu, a young man who lay sprawled on the doorway of the clinic as I was walking in early one Monday morning after my huge ordeal of a journey from home.

I had to literally jump over Velu and open the door to the clinic. The door creaked open with a gush of musty smell of a room that had been closed for the whole day on Sunday. I flipped on the light switch, kept my bag on the table and came out to see what was happening to the young man on the floor. He was moaning and groaning in pain and lying absolutely still on the floor. A demure young woman was standing outside and did not establish eye contact with me. Nobody said anything. I knelt and held Velu's hand. I did not know his name at that time. So I asked, "Thambi, who are you? What is your name? look at me, open your eyes…."

The young man was thin built. He was wearing a white dhoti and was bare chested. He had well developed chest, shoulder, arm and upper limb muscles. I remember thinking he must be a manual laborer by occupation. He remained still on the floor and kept letting out groans of pain. I shook his shoulder gently and called out once again, "Thambi…open your eyes. Tell me. What is your name? what happened?"

He did not open his eyes. Then I turned to the young girl standing outside.

"What happened? What is his name? Will you be able to tell me?" I asked her.

She spoke in a straight emotionless voice. "He is Velu. He is my husband. From last night he is having pain in his stomach. Just now we took auto and reached here." Throughout this she did not look at my eyes even once. Her head was bowed down and her voice was low and lacked any emotion. All I could notice was that she was scared and worried.

I gently touched his abdomen. It was quite rigid. Was it the tone of his abdominal muscles, or was it some disease process happening inside his abdominal cavity? I was not sure. When some structure like the stomach or gall bladder inside the abdomen tears, the contents of this structure ooze into the abdominal cavity and it can lead to rigidity like this. I was starting to wonder if it was that but could not be too sure.

"He cannot be lying here on the floor like this. Come and give me a hand, let us take him to the bed inside." I called out to the young woman. She came running and caught hold of Velu's left side. I supported the right side of his body and we both gently lifted him up. I was doing most of the heavy lifting, the frail young girl was too scared to do anything. As we lifted him, Velu let out a shout of pain.

"Please tolerate the pain for a second. Let us get you to the cot." I said and gently helped Velu to the treatment room and made him lie down on the cot.

Meanwhile the housekeeping staff on duty that morning also entered the clinic.

"Sir, what is happening? Looks like you have a patient early in the morning. Is there anything I can do?" She asked.

"Akka, can you do me a favour? Open all the windows. And get me the BP apparatus from the clinic room and also my stethoscope from my bag"

The BP was very low and his pulse was thready and fast. He was sweating profusely and continued to groan with the pain. It all pointed towards a probable perforation of some organ within his abdomen. I could see that the young girl, Selvi, was restless and shifting here and there in the same place where she was standing. She must be extremely worried.

"Your husband must be immediately taken to the big hospital in Chengalpet. It looks like he has a problem in his abdomen. He will need scans. It also looks like

he may need surgery." I told the girl.

The girl kept staring at me and was speechless.

"Did you understand what I just said?"

Still no response.

"Can you call any elders in your family to come here? I will explain everything to them."

She ran out immediately and pulled out a small button mobile phone from the fold in her saree. She called someone and spoke in an animated manner. Then she came in.

"Have you informed someone?"

"I just told my parents. They are coming with Velu's parents. They are on the way" she said. She had the same blunt affect. She probably did not understand what I was saying. Both she and Velu were very young, must have been in their late teens. They came from the hamlet called Iruttupatti which was about 10 Km from the clinic. It was a tribal settlement and the people in the hamlet lived in abject poverty. Their main occupation was cutting trees and cleaning farm lands once the crop is harvested. They must have been married early as is the custom in their community.

"Don't give him any water or food to drink or eat. We will give him intravenous fluids. We will prepare him to transfer him to Chengalpet hospital" I said and moved on to see the other patients who had started coming into the clinic.

About half an hour later there was a lot of commotion just outside the clinic. Some ten to fifteen people, from the Iruttuppatti hamlet had all come in 6 different two wheelers and they were all ganged up inside the clinic campus. Selvi was standing in their midst and they were talking to her. I finished seeing two patients who were waiting in the clinic and then I got up and walked to the treatment room where Velu was lying on the cot with the intravenous fluid running. He was quite peaceful and his groaning sounds had reduced. Around the same time, a middle aged man, must be in his mid 40s walked up to me.

"Doctor, I am Velu's father. What is the problem?"

I explained the possibility that there could be either a ruptured appendix, gall bladder or stomach and he needs to be seen in a higher hospital like the Chengalpet hospital. Velu's father listened intently to the entire conversation.

"Doctor, can you treat him here?"

"Sir, I just now explained to you why I cannot treat him here. It looks like a complex problem. We must admit him in Chengalpet GH, get some scans and have expert surgeons see him."

"You are giving him intravenous fluids here. That is enough. Keep him here."

I was starting to lose my patience. I saw another man standing outside and gestured him to come and join us.

"Sir, who are you?"

"I am Selvi's father"

"Ok, I am trying to explain to Velu's father that his son's life is in danger. We cannot treat him here. We must take him to Chengalpet GH immediately. Otherwise anything can happen. Please explain to him"

"You have to treat him here…don't you understand?" said Selvi's father. His voice was raised, and he had an angry expression on his face. I had expected this man to talk sense to Velu's father but I had invited more trouble for myself. I did not know what to do. Meanwhile two more much younger men walked into the treatment room. The treatment room was fully crowded now. The men who entered were talking to the two older men, "What is he saying? Will he treat our Velu here or not? If he won't treat us here, we will see how he runs this hospital?" tempers were rising.

The two elder men calmed the younger ones and Velu's father once again addressed me, "The decision is your doctor. Will you treat him here and save him? Otherwise send him with us, we will take him home"

I was not sure what to do. I was angry because a bunch of goons were threatening to vandalize the hospital if I did not do what they wanted. On the other hand I was conflicted because if I kept him in the clinic, he would probably get more sick and probably even die. It was a potentially preventable death.

Velu was probably not even an adult yet and so not in a position to make his own decisions. Selvi looked much younger than him, also probably a teenager. I had to let the decision rest with the father. I quickly walked out of the treatment room, which was by now reeking with the smell of sweat, anger and frustration. I went into the clinic space where I see patients. Fortunately, no other patients had come. I sat down in my chair, closed my eyes. My first thought was what my teacher would do in such a situation. The flash of my teacher standing by Velu's side with one of his hands on Velu's shoulder and the other holding Velu's father's hand came in my mind. I realized my teacher never viewed patients or their families as adversaries to be managed, but as frightened human beings caught in a stormy situation. Protocols were made to protect life, but if enforcing the protocol meant driving this boy away to die in his hut, it was failing. I needed to stand with them, not against them. I got my answer. I got up and walked to the treatment room.

"I have explained to you repeatedly what to do. But you are not willing to go to Chengalpet GH. You probably have your own reasons. I will treat him here and closely monitor him. If there is any worsening, then we must take him to Chengalpet GH. You must not stop me from doing that to save your son's life." I said to Velu's father.

The angry expression on Velu's father softened and he came towards me. He held his palms together in a salutation and said, "Doctor, you are like God. Please treat him here. Nothing will happen to him. You just treat him here. We believe in you."

I started intravenous antibiotics and gave him some pain killer injections and continued the intravenous fluids.

The clinic was only an outpatient facility. There were no arrangements to admit patients and observe them. There were no staff nurses for night shift. I decided to stay back that night to observe this young man. By 6 pm, all the men who had come from Iruttuppatti had left to their home. Velu, his wife and father were the only ones remaining in the clinic. Everything was quiet. I went into the treatment room and sat with Velu. He was deep asleep now. There was no sign of any pain on his face. But the board like rigidity on his abdomen remained. Selvi was sitting on the floor beneath Velu.

"How old are you and Velu?" I asked.

Selvi did not reply. Velu's father who was sitting on a stool nearby said, "Velu must be 17 and she must be 15"

"What happened and how did Velu develop this pain?"

Selvi got a bit agitated by this question and she replied in a hurry, "Yesterday he went to cut a tree in Vittalapuram. There was an old well there. He peeped into the well. A ghost must have caught him from that old well. From then he has not been ok" Her voice was barely above a whisper, carrying the absolute sincerity of a fifteen-year-old child who had no other framework to make sense of her husband's sudden agony.

Velu's father shouted at Selvi, "Shut up girl. These people don't believe in ghosts. So we cannot tell them these stories. They don't understand. We must speak in a language they understand. Sorry Doctor, she is a young girl. She doesn't know how to speak to you"

"I will go and get some dinner. What about you? Where will you eat?"

"Selvi's brother will bring us food in some time"

I then walked to the small tea shop that stays open in the corner near the hospital and got myself some bun, biscuits and tea for dinner. Once, I finished eating, I came back and went to the treatment room to check on Velu. To my absolute surprise Velu was sitting up. I went near him and asked, "Velu, how is your pain?"

For the first time in this whole episode Velu spoke. His voice was high pitched. It had not even broken into a proper male voice yet. "Pain is lesser now. I am hungry. Can I eat something?"

I was surprised. Not only had Velu miraculously improved with minimal treatment, but he was also getting back his hunger. I put my hand on his abdomen and felt it. The board like rigidity was gone. His abdomen was soft and supple. As I felt his newly relaxed abdomen that night, I realized what I had taken for classic surgical peritonitis was likely severe, dehydrating muscle spasm and localized guarding. In my exhaustion and panic that morning, I had prepared for the worst, but simple rehydration and time had unclenched his body.

I encouraged him to eat his food. By 9.30 PM, they all finished dinner and all of us went to sleep, Velu on the cot, Selvi and Velu's father on either side floor and me on the cot next to them. The relief of Velu's miraculous recovery was so deep that I slept despite the onslaught of violent mosquitoes and other insects in the room.

I woke up early in the morning, only to find all three of them up and ready to leave. Velu's father came near me and stood with folded hands, "Sir, you saved my son's life. We are always grateful to you. Thank you so much." I did not know what to say. I watched from the clinic doorway as the three of them stepped out into the crisp morning air.

Just 24 hours earlier, this same yard had been a scene of chaotic fury six motorcycles roaring in, kicking up clouds of angry dust, filled with fifteen men shouting threats and desperate in their fear. The air back then had reeked of sweat, hostility, and impending violence. Now, the morning was enveloped in a profound, golden stillness.

Velu walked on his own two feet, his posture upright, flanked by his young wife and his father. There were no roaring engines, no raised voices, and no lingering anger only the soft, rhythmic sound of their footsteps against the earth. Velu's father turned back one last time, brought his palms together in a silent, deeply respectful namaste, and then turned to join his children. I stood quietly, thanking my teacher for the quiet wisdom that had stayed my hand the previous afternoon. As the small family walked away, their silhouettes slowly shrinking down the long, dusty village road into the soft light of the rising sun, it felt as though the storm of yesterday had never happened leaving behind only a quiet, enduring grace.

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Throughout all the 10+ years of medical education, I have been touched and nourished by brilliant teachers and mentors. Dr. K.P. Misra is my guru and mentor who has largely shaped the way I see the world, both medical and personal. Even today the first question that pops in my mind when faced by a conflicting situation is "What would Sir do?" Dr. Usha Sriram, Dr. Arjun Rajagopalan, Dr. Jayaprakash Muliyil, Dr. KR. John,

Dr. Jasmin Helan Prasad have hand-held me at various stages of my medical education and taught me the ropes. I would like to offer my most humble respects to all of them for shaping my thoughts and world-view.

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I discovered S. Ramakrishnan in 2023 and in the past three years I have devoured most of his books with the earnestness of a hungry child. S. Ra's writing style and his mastery over thoughts and words have left a deep impact on me. Six of the short stories in this collection were originally written in Tamil, influenced by S. Ra. But then I translated them to English to fit them into this collection. My dream is to write something in Tamil and show it to him at some point.

Lastly I am indebted to all the patients who trust me and submit their health and their wellbeing to me. It has been a joyous responsibility over the past three decades! They shared with me their joys, sorrows, pains, achievements, doubts, confusions, uncertainties, opened their hearts and poured everything on to me.

About the Author

Vijayaprasad Gopichandran obtained his Bachelor of Medicine and Bachelor of Surgery (MBBS, 2002) from the Madras Medical College, Chennai, India and Doctor of Medicine in Community Medicine (MD, 2010) from the Christian Medical College, Vellore, India. Following this he worked at the Rural Women's Social Education Center, a rights based women's organization in Chengalpet, providing community based health care services in rural areas. He pursued his doctoral research in Public Health at the SRM University, Kancheepuram District with an INSPIRE Fellowship from the Department of Science and Technology, Government of India. His area of work for the PhD was "trust in the doctor-patient relationship". After obtaining his PhD degree in Nov 2015, he worked as Assistant Professor at the Department of Community Medicine, ESIC Medical College and Hospital, KK Nagar, Chennai, India till Feb 2024. He currently works as a primary care physician at the RUWSEC clinic serving the villages around Tirukazhukundram in Chengalpet district and at the Omayal Achi Community Health Centre, Arakkampakkam, Thiruvallur District. His areas of interest include non-communicable diseases epidemiology, health communication, scale development and validation, doctor-patient relationships, medical and public health ethics. He serves as working editor for the Indian Journal of Medical Ethics since 2014. He has more than 25 years of primary care clinical work experience in outpatient and inpatient settings and 15 years of clinical work in community health. He has been actively involved in community based epidemiological and bioethics research, more than 100 of which have been published in national and international journals. He has published his doctoral work on trust in doctor-patient relationships as a book by Springer Publications.

He regularly blogs at https://www.blogger.com/blog/posts/4104185901104293799

Beyond protocols and pathology, clinical medicine is fundamentally an encounter between two vulnerable human beings.

Spanning three decades from the idealistic corridors of medical training to the grounding realities of rural primary care Hearing the Unspoken brings together 22 narratives that reveal what happens when a doctor stops looking only at diagnoses and begins listening to stories.

Across five thematic collections, these stories explore the moral distress of novice clinicians, the quiet dignity of marginalized voices, the loneliness of illness, everyday social complexities, and the humbling mysteries of healing. Honest, reflective, and deeply empathetic, this book deconstructs the illusion of the all-knowing healer. It serves as a reminder that the true art of medicine lies not in infallible science, but in the courage to listen to the unspoken.